



STATE OF UGANDA POPULATION REPORT 2025



MENTAL HEALTH: A SILENT EMERGENCY

Addressing the Psychological Dimensions of Uganda's Demographic Dividend



December 2025



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FOREWORD



It is with great pleasure that I present the State of Uganda Population Report (SUPRE) 2025, produced under the national theme “Mental Health: A Silent Emergency.” This year’s report comes at a pivotal moment in Uganda’s development journey. The country has committed to an ambitious Ten-Fold Growth Strategy, aimed at expanding the economy from USD 50 billion in 2023 to USD 500 billion by 2040. Achieving this aspiration will require sustained investment in a healthy, skilled, productive, and emotionally resilient population.

The findings presented in this year’s report are compelling. Mental health conditions are rising steadily, with facility-reported cases increasing by more than 70 percent between 2021 and 2024.

Depression, anxiety, trauma-related disorders, substance misuse, and psychosocial distress are affecting millions of Ugandans, including approximately 23 percent of children and 24 percent of adults, as well as large numbers of adolescents and young people navigating school pressures, unemployment, and social uncertainty. Yet fewer than one in ten people in need receive appropriate care, largely due to limited infrastructure, critical workforce shortages, persistent stigma, and constrained financing.

This report also brings to the fore the substantial economic cost of mental illness. The monetary value of disability-adjusted life years lost to mental disorders has increased more than tenfold between 2000 and 2023 and continues to rise. Without accelerated investment in prevention, early identification, and community-based care, Uganda risks long-term productivity losses that could undermine the Ten-Fold Growth Strategy and the aspirations of the Fourth National Development Plan (NDP IV).

In producing SUPRE 2025, the National Planning Authority has drawn on available national datasets, facility-based information systems, system readiness assessments, and published research to present as comprehensive a picture as possible of the mental health landscape in Uganda. However, the report also candidly highlights a major constraint: the scarcity, fragmentation, and under-reporting of mental health data across sectors. Many cases remain undocumented due to stigma, reliance on informal and faith-based care, and weak routine data systems. This limits the depth of analysis that can be undertaken and underscores the urgent need to strengthen mental health surveillance, research, and integrated information systems to better inform policy, planning, and investment going forward.

I commend the National Planning Authority for leading the development of this report and for elevating mental health as a strategic development issue. I encourage policymakers, local governments, development partners, civil society organisations, and all Ugandans to engage with the findings of SUPRE 2025 and to support the implementation of its recommended actions.

Together, let us confront this silent emergency and build a mentally healthy, resilient, and productive population to underpin Uganda’s socio-economic transformation

Hon. Amos Lugolobi

Minister of State for Finance, Planning and Economic Development (Planning)

PREFACE



The State of Uganda Population Report (SUPRE) is the National Planning Authority's flagship instrument for translating population evidence into development planning intelligence. Produced annually, the report provides a structured analytical lens through which demographic trends, social conditions, and development systems are examined in relation to Uganda's long-term transformation agenda.

The 2025 edition of SUPRE is dedicated to *Mental Health: A Silent Emergency*. This thematic focus reflects a strategic planning judgement: that mental health has become a cross-cutting constraint to human capital development, service delivery effectiveness, and socio-economic resilience, yet remains insufficiently integrated into population analysis and development

decision-making. By situating mental health within Uganda's demographic profile, labour dynamics, education systems, urbanisation patterns, and social protection landscape, this report deliberately moves beyond a sector-specific treatment and positions mental health as a structural development variable.

SUPRE 2025 adopts a population-based and systems-oriented approach. It examines how mental health risks and outcomes are shaped across the life course, how demographic pressures interact with socio-economic stressors, and how existing service delivery, workforce, financing, and governance arrangements respond to this emerging burden. In doing so, the report bridges evidence from population statistics, socio-economic analysis, cost-of-inaction modelling, and system readiness assessments to provide an integrated foundation for policy and planning.

A defining contribution of this edition is its emphasis on development systems rather than individual outcomes alone. The report interrogates the alignment between mental health needs and the organisation of primary health care, referral networks, human resources, education and labour market transitions, community systems, and data architectures. This orientation is intended to support planners and decision-makers in identifying leverage points where coordinated, multi-sectoral interventions can yield the greatest population-level impact.

The preparation of SUPRE 2025 was undertaken through extensive collaboration with Government institutions, specialised health facilities, academia, development partners, civil society organisations, and technical experts. At the same time, the process has reinforced an important planning reality: that mental health data remain fragmented across sectors, under-reported at community level, and weakly integrated into routine information systems. Addressing these gaps is essential if future population planning is to be responsive, preventive, and evidence-driven.

The National Planning Authority presents this report as a strategic planning resource. It is intended to inform national and sub-national development plans, sector strategies, investment decisions, and programme design under the Fourth National Development Plan, the Ten-Fold Growth Strategy, and Uganda Vision 2040. Beyond its immediate findings, SUPRE 2025 seeks to catalyse a shift in how mental health is understood—away from a marginal social issue and toward its rightful place as a determinant of productivity, inclusion, and long-term national resilience.

It is our expectation that this report will strengthen planning coherence, deepen cross-sector dialogue, and support more deliberate integration of mental health considerations into Uganda’s population and development architecture.



Prof. Pamela K. Mbabazi
Chairperson

ACKNOWLEDGEMENT



The State of Uganda Population Report (SUPRE) 2025, developed under the theme “*Mental Health: A Silent Emergency*,” represents the National Planning Authority’s continued commitment to generating and disseminating evidence to inform population and development policy in Uganda. By focusing on mental health, this year’s report addresses an area that has historically received limited attention in planning and investment, yet is increasingly recognized as fundamental to human capital development, productivity, and national resilience.

The preparation of SUPRE 2025 was informed by a wide range of data sources arising out of collaboration with our partners. Nevertheless, the process revealed significant handicaps in data availability, and quality on mental health across sectors. These constraints highlight the urgent need to strengthen mental health data ecosystem to support evidence-informed decision-making in the future.

On behalf of the Executive Board, Management, and staff of the National Planning Authority, I extend sincere appreciation to the Government of Uganda and all institutions and individuals who contributed to the development of this report. In particular, I acknowledge the Ministry of Health; Butabika National Mental Health Referral Hospital; the Ministry of Education and Sports; the Uganda Bureau of Statistics; Makerere University School of Public Health; Local Governments; Development Partners, especially UNFPA; Civil Society Organisations; academia; and all stakeholders whose technical input and collaboration made this report possible. I also commend the Management and staff of the National Planning Authority for their dedication and professionalism throughout the preparation of SUPRE 2025.

It is my hope that this report will stimulate informed dialogue, strengthen collaboration, and catalyse action to improve mental health outcomes as part of Uganda’s broader development agenda.

A handwritten signature in blue ink, appearing to read 'Joseph Muvawala', with a stylized flourish extending from the bottom left.

Joseph Muvawala, PhD

Executive Director



Message from UNFPA Country Representative



Mental health does not exist in a vacuum. This was the key message of this year's State of Uganda Population Report themed "Mental Health: A Silent Emergency." With Uganda's high fertility rate and large youth population, mental health risks are amplified by strained resources and increased psychosocial stress in large, resource-constrained households. Yet, one of the most profound impacts is found at the intersection of sexual and reproductive health (SRH) and gender-based violence (GBV), creating a vicious, bidirectional cycle of vulnerability, disproportionately affecting women and adolescent girls.

The report crucially highlights the damaging effects of GBV, HIV-related stigma, and of the early family responsibilities and reduced autonomy associated with teenage childbearing. Early marriage and early childbearing heighten the risk of girls dropping out of school, thereby lowering their educational attainment and employment prospects, and putting them at greater risk of social and economic isolation, and mental stress.

GBV is significantly correlated with a heightened and pervasive risk of severe psychological distress among survivors. The traumatic experience of GBV, which encompasses physical, sexual, emotional, and economic abuse, often leads to a complex array of mental health consequences. The distress is compounded by societal stigma, lack of adequate support systems, and the systemic impunity that frequently surrounds perpetrators, creating a climate of chronic stress and insecurity that severely impacts a survivor's overall well-being and ability to function.

While national policy acknowledges the link between SRH, GBV and mental health, service integration remains limited. Addressing this silent crisis demands a holistic, multisectoral response. Strengthening SRH services, including through voluntary family planning and adolescent-friendly services, is crucial for improving mental health outcomes as well as safeguarding rights and choices for adolescents and youth. This will enable us to achieve UNFPA's mission, delivering a world where every pregnancy is wanted, every childbirth is safe and every young person's potential is fulfilled.

UNFPA continues to support the Government of Uganda in advancing inclusive development through increased household incomes, full monetization of the economy, and employment as

the cornerstone of sustainable socio-economic transformation. Addressing the growing emergency of mental health with a particular focus on its interrelation with SRH and GBV is key in ensuring that Uganda's young population can thrive and attain their full potential.

Kristine Blokhuis

Kristine Blokhuis

UNFPA Representative



LIST OF ACRONYMS

AIDS	Acquired Immune Deficiency Syndrome
ART	Antiretroviral Therapy
AU	African Union
CAMHS	Child and Adolescent Mental Health Services
CHEWs	Community Health Extension Workers
CPD	Country Programme Document
CPD9	9th Government of Uganda–UNFPA Country Programme
CPD10	10th Government of Uganda–UNFPA Country Programme
CRPD	Convention on the Rights of Persons with Disabilities
CSOs	Civil Society Organizations
DALYs	Disability-Adjusted Life Years
DD	Demographic Dividend
DSM-5	Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition
EAC	East African Community
GBV	Gender-Based Violence
GKMA	Greater Kampala Metropolitan Area
GoU	Government of Uganda
HC	Health Centre
HC IV	Health Centre Level IV
HCD	Human Capital Development
HIV	Human Immunodeficiency Virus
HMIS	Health Management Information System
ICESCR	International Covenant on Economic, Social and Cultural Rights
LGs	Local GovernmentS
MDAs	Ministries, Departments and Agencies
MEC	Ministry of Education and Sports
MHPSS	Mental Health and Psychosocial Support Services
MoES	Ministry of Education and Sports
MoFPED	Ministry of Finance, Planning and Economic Development
MoGLSD	Ministry of Gender, Labour and Social Development
MoH	Ministry of Health
MNS	Mental, Neurological and Substance Use Disorders
NCDs	Non-Communicable Diseases
NDP	National Development Plan
NDP IV	Fourth National Development Plan
NEET	Not in Education, Employment or Training
NHP III	National Health Policy
NPA	National Planning Authority
PHC	Primary Health Care
PTSD	Post-Traumatic Stress Disorder
SDGs	Sustainable Development Goals
SRH	Sexual and Reproductive Health

SRHR	Sexual and Reproductive Health and Rights
SUPRE	State of Uganda Population Report
TFR	Total Fertility Rate
UBOS	Uganda Bureau of Statistics
UDHS	Uganda Demographic and Health Survey
UHC	Universal Health Coverage
UHRC	Uganda Human Rights Commission
UNFPA	United Nations Population Fund
VHTs	Village Health Teams
WHO	World Health Organization
YLDs	Years Lived with Disability



GLOSSARY

Adolescent Mental Health

The emotional, psychological, and social well-being of individuals aged 10–19 years, influencing how they think, feel, cope with stress, relate to others, and make decisions during a critical developmental stage.

Age Structure

The distribution of a population across different age groups, often used to assess dependency levels, labour force potential, and demand for services such as education, health, and mental health care.

Anxiety Disorders

A group of mental health conditions characterised by excessive fear, worry, or nervousness that interferes with daily functioning, including generalized anxiety disorder and panic disorder.

Cost of Inaction

The estimated human, social, and economic losses incurred when mental health conditions are not prevented, diagnosed, or treated, including productivity losses, increased health care costs, and long-term disability.

Coverage Radius

The geographic distance between a population and the nearest health facility providing mental health services, used as an indicator of physical access to care.

Demographic Dividend

The potential economic growth that can result from changes in a country's age structure, particularly when the working-age population grows larger relative to dependents, provided there are adequate investments in health, education, and employment.

Demographic Transition

The shift from high fertility and mortality rates to lower fertility and mortality rates as a country develops, with implications for population growth, age structure, and service demand.

Depression

A common mental health disorder marked by persistent sadness, loss of interest or pleasure, low energy, and impaired daily functioning.

Disability-Adjusted Life Years (DALYs)

A summary measure of population health that combines years of life lost due to premature mortality and years lived with disability, used to quantify the burden of disease including mental disorders.

Health System Readiness

The capacity of the health system to deliver effective mental health services, including infrastructure, workforce, financing, medicines, referral systems, and data management.

Human Capital Development (HCD)

Investments in health, education, skills, and well-being that enhance the productivity and economic potential of individuals across the life course.

Life-Course Approach

A framework that recognises that mental health risks and protective factors accumulate from early childhood through old age, requiring age-sensitive and continuous interventions.

Mental Health

A state of well-being in which an individual realises their abilities, can cope with normal life stresses, work productively, and contribute to their community.

Mental Health Disorder

A clinically significant disturbance in thinking, emotional regulation, or behaviour that reflects dysfunction in psychological or biological processes and causes distress or impairment in daily life.

Mental Illness

A subset of mental health disorders characterised by more severe and persistent symptoms that significantly interfere with social, occupational, or family functioning.

Mental, Neurological and Substance Use Disorders (MNS)

A group of conditions that include depression, anxiety, epilepsy, psychosis, and substance use disorders, collectively contributing substantially to disability and productivity loss.

Multisectoral Approach

Coordinated action across multiple sectors such as health, education, labour, social protection, justice, and local government to address the social and structural determinants of mental health.

NEET (Not in Education, Employment, or Training)

Young people who are not engaged in formal education, work, or skills training, a group at heightened risk of psychological distress and social exclusion.

Non-Communicable Diseases (NCDs)

Chronic diseases that are not transmitted from person to person, such as cardiovascular diseases, diabetes, cancers, and mental health disorders.

Primary Health Care (PHC)



Essential health services provided at community and first-level health facilities, intended to be the first point of contact for individuals, including for basic mental health care.

Psychosocial Distress

A broad range of emotional and social difficulties, including symptoms of anxiety, depression, behavioural problems, and functional impairment, often linked to life stressors.

Referral Pathway

A structured system for transferring patients between levels of care, from community and primary facilities to higher-level or specialised mental health services.

Social Determinants of Mental Health

The social and economic conditions that influence mental well-being, including income, education, employment, housing, gender norms, and exposure to violence.

State of Uganda Population Report (SUPRE)

An annual flagship publication of the National Planning Authority that provides evidence-based analysis on population dynamics and development issues to inform policy and planning.

Stigma

Negative attitudes, beliefs, and discrimination directed at individuals with mental health conditions, which often discourage help-seeking and treatment.

System Integration

The incorporation of mental health services into general health, education, and social protection systems rather than treating mental health as a standalone issue.

Youth Bulge

A demographic situation where a large proportion of the population consists of young people, creating both opportunities and risks depending on investments in human capital and employment.

TABLE OF CONTENTS

FOREWORD	i
ACKNOWLEDGEMENT	iv
LIST OF ACRONYMS	vii
GLOSSARY	ix
TABLE OF CONTENTS	xii
LIST OF TABLES	xvi
LIST OF FIGURES	xvii
EXECUTIVE SUMMARY	xviii
CHAPTER 1: OVERVIEW OF THE STATE OF UGANDA POPULATION REPORT (SUPRE) 2025	1
1.0 Introduction.....	1
1.1 Snapshot of Uganda’s Population 2025.....	1
1.1.1 Population Size, Growth, and Distribution.....	1
1.1.2 Population Age Structure and Dependency Patterns	2
1.1.3 Total Fertility Rate.....	4
1.1.4 Mortality (Infant and Maternal)	4
1.1.5 Education and Literacy.....	5
1.1.6 Labour force participation	5
1.1.7 Urbanization.....	6
1.1.8 Teenage Pregnancy and Child Marriage.....	6
1.1.9 Gender-Based Violence.....	7
1.1.10 Poverty	8
1.2 Background.....	8
1.3. Rationale	10
1.4 Objectives	11
1.4.1 Specific objectives.....	11
1.5 Approach and Methodology.....	12
1.6 Conceptual Framework underpinning the report	13
1.7 Limitations in data availability.....	14



1.8 Structure of the Report	14
CHAPTER 2 SITUATION ANALYSIS	15
SITUATION ANALYSIS	15
2.1 Introduction.....	15
2.2 Population Dynamics and Demographic Context.....	15
2.2.1 Age Structure and Mental Health in Uganda	16
2.3 Population Health Profile and Epidemiological Context.....	19
2.3.1 Mortality and Morbidity Trends	19
2.3.2 Fertility Trends and Reproductive Health Context.....	21
2.3.3 Life Expectancy, Disability and Chronic Conditions.	23
2.3.4 Non- Communicable Diseases and Comorbidity with Mental Health	25
2.3.5 Sexual Reproductive Health and Mental Health	26
2.4 Overview of Mental Health in Uganda	27
2.4.1 Current status of Mental Health in Uganda	27
2.4.2 Regional Mental Health Disparities within Uganda	28
2.4.3 Key Mental Health Conditions in Uganda	31
2.5 Migration, Urbanization, Mobility, and Mental Health in Uganda.....	33
2.6 Expenditure on mental health in Uganda	34
2.7 Policy and Legal Framework for Mental Health	34
2.7.1 Introduction	34
2.7.2 International and Regional Legal Commitments.....	35
2.7.3 National Legal Framework: Progress and Persistent Gaps	35
2.8 Overall Emerging Issues from the National Situation Analysis.....	39
CHAPTER 3: SOCIO-ECONOMIC DRIVERS OF MENTAL HEALTH	42
3.1 Introduction.....	42
3.2 Background and Overview	42
3.3 Methodology	44
3.3.1 Study Design.....	44
3.3.2 Data Sources.....	44
3.3.3 Inclusion Criteria	44
3.3.4 Exclusion Criteria	44
3.3.5 Theoretical Framework.....	45
3.4 Discussion of Findings	45
3.4.1 Findings from Theoretical and Empirical Literature.....	45

3.4.2 Findings from Empirical Estimations – Ugandan Perspective	49
3.5 Conclusions	54
CHAPTER 4: COST OF MENTAL HEALTH AND INACTION (HUMAN AND ECONOMIC BURDEN) IN UGANDA	55
4.1. Introduction	55
4.2. Analytical Framework and Methodology	56
4.2.1. Conceptual Framework	56
4.2.2. Theoretical Framework	58
4.2.3. Empirical/Estimation Model	58
4.2.4. Study Population and Data Sources	59
4.3. Human Burden of Mental Health	59
4.3.1. Disability-Adjusted Life Year	59
4.3.2. Monetary value of DALYs	62
4.4 Economic Burden of Mental Health	63
4.5 Cost of Inaction	64
4.5.1. Human Cost	65
4.5.2. Economic Cost	66
4.5.3. Total Cost of Inaction and potential Savings from scale-up	69
4.6. Mental Health Intervention Investment Scenarios	70
4.7. Conclusions	73
CHAPTER 5: SYSTEM READINESS AND HUMAN RESOURCE ANALYSIS	75
5.1 Introduction	75
5.2 Overview of Uganda’s health infrastructure and workforce context	76
5.2.1 Structure of Health System	76
5.2.2. Standards for Mental Health management	78
5.2.3 Workforce for mental health	78
5.2.4 Challenges and Opportunities in Service Delivery	82
5.3 Objectives	83
5.4 Study Design and Methodology	83
5.4.1 Study Design	83
5.4.2 Sampling Framework, Sample Size, and Data Collection	84
5.4.3 Conceptual Framework	84
5.5 Results and Findings	85
5.5.1 Participating Facilities	85

5.5.2 Existence of a functional Mental Health Unit	86
5.5.3 Coverage Radius	86
5.5.4 Services Provided	87
5.5.5 Medicines Procurement Plans and Drug Stockouts	88
5.5.6 Infrastructure and Utilities	88
5.5.7 Community and Patient Referral Pathways	88
5.5.8 Functional Emergency Units	88
5.6 Discussion of results	90
5.6.1 Health Facility Landscape and Coverage	90
5.6.2 Functionality of Mental Health Units	90
5.6.3 Service Availability and Integration	91
5.6.4 Infrastructure Utilities and Human Resources for Mental Health	91
5.6.5 Mental Health Medicines, Commodities, referrals and emergency services	92
5.6.6 Financing and Budgeting	92
5.6.7 Service Utilization and Data Systems	92
5.7 Conclusion	93
CHAPTER 6: CONCLUSIONS AND RECOMMENDATIONS	94
References	98
Annex	102

LIST OF TABLES

Table 1: Mental health cases according to sub-regions	29
Table 2: Mental health conditions	32
Table 3: Correlation Analysis Results.....	50
Table 4: Multiple Linear Regression Results.....	50
Table 5: Monetary value of DALYs in Uganda 2000-2023 (UGX)	63
Table 6: Economic burden of mental health conditions in Uganda (2021 UGX, millions)	64
Table 7: Projected Monetary Value of DALYs under Business-as-Usual and Scaled-Up Scenarios, 2024–2040	65
Table 8: Projected Direct Costs and Potential Savings from Scaled-Up Mental Health Interventions in Uganda, 2024-2040	66
Table 9: Projected Absenteeism Costs and Potential Savings from Scaled-Up Mental Health Interventions, 2024–2040	67
Table 10: Projected Presenteeism Costs and Potential Savings from Scaled-Up Mental Health Interventions, 2024–2040	68
Table 11: Projected Indirect Costs (Absenteeism and Presenteeism) and Potential Savings from Scaled-Up Mental Health Interventions, 2024–2040	69
Table 12: Projected Total Cost of Inaction and Potential Savings from Scaled-Up Mental Health Interventions in Uganda, 2024–2040	70
Table 13: Treatment-Based Interventions considered for reduction of the Total Cost of Mental Health	71
Table 14: Approved Mental Health Establishments for the Combined Platform (Government, PNFP, and PFP).....	79
Table 15: Indicative Filled Posts and Gaps for the Combined Platform (Planning Estimate)	80
Table 16: National Training Capacity for Selected Mid-level Mental Health Programmes	80
Table 17: Annual Graduation rates in Mental health Programs	81
Table 18: Estimated Mental Health Workforce Gaps by Cadre	81
Table 19: Projected Mental Health Workforce Gap Reduction by Cadre (2026–2030)	82
Table 20: Facilities Participating in the Study by Type.....	85
Table 21: Core Functional aspects of Mental health Units	86
Table 22: Percent of Mental health disorders by sex and age group in 2024	89

LIST OF FIGURES

Figure 1: Uganda's population trends (1948-2025).....	1
Figure 2: Population Pyramid	3
Figure 3: Total Fertility Rates 1991-2024.....	4
Figure 4: Mortality (Infant, Maternal, and Under Five) trends 1995-2022.....	5
Figure 5: Trends of Teenage Pregnancy 2000-2022	7
Figure 6: Conceptual Framework informing the State of Population Report of Uganda 2025	13
Figure 7: Percentage of Disability-Adjusted Life Years	24
Figure 8: Mental health cases in Uganda (2021-2024).....	28
Figure 9: Mental Health Cases by 2024 according to Subregions	30
Figure 10: Percentage change in mental health cases by 2021-2024	31
Figure 11: Drivers of Mental Health in Uganda	46
Figure 12: Conceptual Framework for Cost-of-Illness of Mental Health in Uganda	57
Figure 13: Disability-adjusted life years (DALYs) from Mental Health in Uganda -2000-2023	60
Figure 14: Disability-adjusted life years (DALYs) from Mental health by age groups in Uganda in 2023	61
Figure 15: The DALYs in Uganda by mental health disorders in 2023	62
Figure 16: Costs of absenteeism and presenteeism for mental health conditions (2021 UGX, millions)	64
Figure 17: Conceptual Framework.....	85
Figure 18:Service utilization by facility and age group	89

EXECUTIVE SUMMARY

While the 2025 State of World Population Report focuses on fertility decline below the replacement level of 2.1 births per woman, highlighting challenges such as labour shortages, population ageing, and pressure on pension and care systems in many developed countries; Uganda faces a markedly different demographic reality. With a total fertility rate of about 4.5 births per woman, the country continues to experience rapid population growth and a high child and youth dependency burden. Uganda's central challenge is therefore not population decline, but how to effectively support, nurture, and transition its large youthful population into a healthy, skilled, and productive workforce capable of driving socio-economic transformation under the Fourth National Development Plan (NDP IV) and the Ten-Fold Growth Strategy.

Against this backdrop, the State of Uganda's Population Report (SUPRE) 2025 focuses on *"Mental Health: A Silent Emergency,"* recognizing mental health as a critical yet historically under-addressed determinant of Uganda's demographic and development outcomes. High fertility, large household sizes, and the socio-economic pressures associated with rapid population growth place significant psychological strain on families, caregivers, and young people, while poor mental health undermines educational attainment, labour productivity, and social cohesion. Addressing mental health is therefore not separate from the fertility and population agenda; it is central to converting Uganda's youthful population into a resilient human capital base that can sustain inclusive growth and long-term development.

As the National Planning Authority's flagship annual publication, the State of Uganda Population Report 2025 provides an evidence-based assessment of the prevalence, drivers, economic costs, and system readiness for mental health, and sets out priority actions for strengthening mental health as a foundation for Uganda's socio-economic transformation.

Mental Health as a Development Imperative

The findings of SUPRE 2025 show that mental health in Uganda has reached a critical inflection point. Facility-reported data indicate that mental health cases increased by 71 percent between 2021 and 2024, rising from 494,326 to 843,295 cases. Population-based estimates suggest that mental health conditions affect approximately 24.2 percent of adults and 22.9 percent of children, accounting for about 13 percent of total disability-adjusted life years (DALYs). High-risk conditions such as depression, substance-use disorders, and self-harm are increasing rapidly, particularly among adolescents, youth, and working-age adults.

These trends are unfolding within a demographic context characterised by a large youth population, with over 73 percent of Ugandans below the age of 30, and persistently high levels of youth exclusion, as more than half of young people aged 18–30 are not in employment, education, or training. Rather than facing labour shortages associated with population ageing, Uganda's core challenge is a high dependency burden and the need to transform a large youthful population into a healthy, skilled, and productive workforce. Rising mental ill-health among children, adolescents, and young adults therefore poses a direct threat to human capital formation, labour productivity, and the country's ability to realise the demographic dividend under NDP IV.



Socio-Economic Drivers and Life-Course Risk

Mental health outcomes in Uganda are strongly shaped by socio-economic conditions across the life course. Poverty, unemployment, education-related stress, rapid urbanisation, gender-based violence, substance abuse, and household insecurity interact to heighten vulnerability to psychological distress, depression, anxiety, and harmful coping behaviours. While income and education are traditionally protective, the analysis shows that higher educational attainment and income are also associated with increased reported mental illness, reflecting aspirational stress, labour market frustration, and improved detection rather than reduced risk. These patterns underscore that mental health is not only a health-sector issue, but a structural development challenge rooted in social and economic systems.

The Economic Cost of Inaction

Mental illness imposes a substantial and growing economic burden on Uganda, largely through productivity losses rather than direct health spending. In 2021, direct expenditure on mental health care amounted to UGX 137.3 billion, while indirect losses from absenteeism and presenteeism reached UGX 696.6 billion, bringing the total annual cost to UGX 833.9 billion. The monetary value of DALYs lost to mental disorders increased from UGX 283 billion in 2000 to UGX 2.96 trillion in 2023. Under a business-as-usual scenario, total economic losses are projected to rise from approximately US\$1.1 billion in 2024 to US\$4.5 billion by 2040. The evidence clearly demonstrates that the cost of inaction far exceeds the cost of strategic investment in prevention, early identification, and integrated care.

System Readiness, Workforce Constraints, and Governance Gaps

Despite policy progress, Uganda's mental health response remains constrained by significant system readiness gaps. Services are heavily concentrated at the referral level, community-based care is weak, and primary health facilities lack trained personnel, functional emergency response, and reliable access to essential psychotropic medicines. Severe workforce shortages persist, with specialists concentrated at national and regional facilities, while lower-level facilities operate with limited capacity. Data systems remain fragmented, with mental health indicators poorly captured across health, education, and social protection platforms, undermining evidence-based planning and accountability. Policy and legal frameworks are largely sound in design, but implementation, regulation, and enforcement remain weak.

Our analysis of the mental health manpower demand and supply confirms that these service delivery gaps are fundamentally driven by deep structural imbalances in the mental health workforce. To operationalize minimum service standards across the national service delivery platform, Uganda requires an estimated 9,661 mental health posts across Health Centre IIIs, Health Centre IVs, General Hospitals, Regional Referral Hospitals, and National Referral Hospitals. Yet only about 4,290 posts are currently filled, leaving a system-wide gap of more than 5,300 community-facility positions, with the most acute shortages at community-facing and district levels where early detection, follow-up, and continuity of care should occur. The analysis shows that over 96 percent of required mental health posts are located at Health Centre IIIs, Health Centre IVs, and General Hospitals, underscoring that access to care depends overwhelmingly on the availability of mid- and lower-level cadres rather than specialists alone. However, staffing deficits at these levels severely undermine task-sharing, referral efficiency,

and service integration, resulting in delayed care, unnecessary referrals, and congestion at referral hospitals.

Workforce supply analysis further reveals that training output remains insufficient and misaligned with demand. While nursing programmes generate the bulk of mental health graduates, output for critical clinical cadres, particularly psychiatric clinical officers, clinical psychologists, and psychiatrists, remains extremely limited, with some programmes producing only one to three graduates annually. Even under optimistic scale-up scenarios, specialist shortages are projected to persist beyond 2030, reinforcing the need for a deliberate, phased workforce strategy that rapidly expands deployable frontline cadres while gradually strengthening advanced training capacity and retention.

Conclusions and Priority Actions

Taken together, the findings of SUPRE 2025 point to an urgent need for a paradigm shift from a facility-centred, specialist-dependent model to an integrated, community-anchored, and system-wide response that embeds mental health across primary health care, education, labour, social protection, and community systems. Mental health must be treated as a foundational investment in Uganda's human capital and long-term development trajectory.

Accordingly, the report proposes a focused set of high-impact, feasible actions to strengthen prevention, care, and system resilience:

1. Address the socio-economic drivers of mental illness through integrated social and economic interventions, while strengthening the evidence base to inform design and implementation.

Preliminary analysis in SUPRE 2025 indicates that a range of socio-economic factors, including poverty, unemployment, education-related stress, food insecurity, substance abuse, and household instability, are significantly associated with mental health outcomes. However, the depth of analysis was constrained by limited facility-based data and the absence of detailed case-level information capturing a wider set of socio-economic risk factors. There is therefore a need for follow-up studies and implementation-focused research, using more comprehensive facility data and qualitative case studies, to better understand pathways of risk and inform the targeting, sequencing, and effectiveness of interventions.

Notwithstanding these limitations, Government, through the Ministry of Gender, Labour and Social Development working with the Ministry of Finance, the Ministry of Education and Sports, the Ministry of Local Government, and Local Governments, should embed psychosocial support, life-skills development, and clear referral pathways within livelihood, skills development, and youth employment programmes, particularly for unemployed youth, NEET populations, and households experiencing chronic poverty and food insecurity. The education system should strengthen school-based mental health promotion, guidance and counselling, and early identification of psychosocial distress linked to academic pressure, family instability, and adolescent development. At community level, substance-use prevention and harm-reduction initiatives should be scaled up through Local



Governments, Village Health Teams, and civil society actors, supported by regulation of alcohol and drug availability and sustained community awareness efforts.

2. Develop nationwide community based mental health care systems to improve access to and availability of mental health services country wide

Government, through the Ministry of Health in collaboration with Local Governments, should position Village Health Teams (VHTs) and Community Health Extension Workers (CHEWs) as the backbone of community-based mental health care through targeted financing, skills development, and integration into the formal health system. This will enable early identification of individuals at risk of mental disorders, referral to appropriate health facilities, and guided follow-up at household level. Community mental health screening, referral tracking, and follow-up should be formally embedded in CHEW/VHT guidelines, reporting tools, and the routine health information system, allowing parish- and sub-county-level monitoring of emerging mental health trends and timely response. This task-sharing approach should be linked to and supported by health facilities through mentorship, referral feedback, technical supervision and integration with primary health care services to ensure continuity of care.

3. Strengthen integration of mental health in health care at all levels by institutionalizing a minimum, integrated mental health service package across both public and private health facilities and ensuring facility readiness for such care.

In order to ensure health facility readiness, the Ministry of Health, working with District Local Governments, referral hospitals, and private health sector regulators, should ensure availability of a) integrated mental health screening, b) uninterrupted availability of essential mental health medicines and supplies, c) basic treatment, and referral, as well as d) standard operating procedures for care and treatment; emergency and crisis-response; and number and cadre of trained staff. Capacitating private facilities will expand access points for early identification, care and referral, particularly in urban and peri-urban areas where they are often the first point of contact. Integration of mental health into routine care will reduce stigma by normalizing help-seeking and avoid isolation of services. Compliance should be reinforced through routine supervision, licensing and accreditation processes, performance reviews, and quality improvement mechanisms, supported by targeted capacity-building for facilities that fall below minimum standards. Health training institutions should ensure mental health is embedded in health training curricular to ensure preservice training for health cadre

4. Strengthen mental health human resources through deliberate and targeted training and recruitment and decentralized competency building

To address critical mental health workforce shortages, Referral Hospitals and districts should be mandated to recruit mental health professional including counsellors, psychologists and social workers into the health system as per the existing revised staffing structures by health facility level. A specialist-led model to support a tiered, competency-based task-sharing approach that strengthens the capacity of non-specialist health workers to provide mental health care while redefining the role(s) of mental health specialists needs

to be implemented. Psychiatrists should be repositioned as regional level mentors and supervisors, providing outreach clinics, tele-mentoring, and rotational support rather than remaining concentrated at central facilities.

As per the mental Health act (2018) Ministry of public service should create positions for and Ministry of Health and Local Governments should recruit district mental health focal persons to coordinate mental health programs and services in all districts.

The Ministry of Health, working with the ministry of Education and Sports, training institutions, Local Governments, and professional councils, should adopt a tiered mental health workforce strategy that prioritizes; a) training in relevant knowledge and skills in mental health care for all health worker training institutions, b) in-service training courses that target non specialist health workforce aimed at task-sharing and decentralized service delivery. Given the severe shortage of specialists, mental health screening, basic treatment, and follow-up should be systematically integrated into the roles of all health workers including general medical officers, clinical officers, nurses, midwives, and community health workers as guided by the minimum integrated mental health service package. To reverse low uptake in mental health specialist training programmes, Government should introduce targeted incentives such as bonded scholarships.

5. Reposition Butabika Hospital as a national centre of excellence, training, and referral support rather than a congested long-stay facility

Government, through the Ministry of Health and the Health Service Commission, should prioritise targeted investment and processes to reposition Butabika National Referral Mental Hospital to become a National Institute of Mental Health whose role will include: providing highly specialized mental health care; training of specialist and non-specialist health workers; designing and implementation of mental health related research and technical support and mentoring to regional and district health facilities.

Government, through the Ministry of Health and the Health Service Commission should prioritize targeted investment to address critical staffing gaps, infrastructure deficits, and service quality constraints, while deliberately reducing avoidable excessive patient admissions through strengthening the referral systems.

Immediate actions include filling approved specialist and nursing positions to enhance the level of specialist care and improve unsafe staff-to-patient ratios, stabilizing funding for essential medicines and equipment maintenance, and completing critical infrastructure projects such as secure perimeter works and modern records systems.

6. Close policy, legal, and regulatory gaps to strengthen mental health governance, patient safety, and accountability across all service providers

Government should urgently operationalise the Mental Health (Amendment) Act through clear subsidiary regulations, enforceable standards, and inspection mechanisms that apply to all mental health service providers, including public facilities, private medical centres, faith-based institutions, and traditional and informal care providers. Regulatory frameworks should explicitly prohibit the operation of unlicensed detention, remand,



correction, or isolation centres masquerading as treatment facilities, and strengthen protections against abuse, unlawful confinement, inhumane treatment, and violations of patient rights.

The Ministry of Health, working with Local Governments, regulatory councils, and law enforcement agencies, should establish mandatory licensing, registration, routine inspection, and sanctions for non-compliance, alongside clear referral and reporting obligations for private and informal providers. Mental health responsibilities should be embedded within binding sectoral regulatory frameworks

7. Establish and regulate a national digital mental health backbone as complementary entry points for early assessment, guidance, and referral

Government, through the Ministry of Health and relevant regulatory bodies, should integrate accredited digital mental health platforms into the formal mental health system as complementary access points for early screening, self-referral, guidance, and crisis support. Well-governed digital platforms can reduce stigma-related barriers to help-seeking, particularly among adolescents, youth, and urban populations, while extending reach beyond facility-based services. Clear standards should be established for governance, clinical oversight, data protection, interoperability with national health information systems, and referral linkages to public and private health facilities, ensuring quality, safety, and accountability.

8. Leverage private-sector and diaspora-led initiatives to expand specialised mental health and addiction care

Government should harness accredited private-sector and diaspora-led mental health initiatives to expand specialised capacity in areas such as substance-use treatment, rehabilitation, counselling, and stepped care, through structured public-private partnerships. Regulatory facilitation, contractual arrangements, and targeted fiscal incentives should be aligned with national mental health priorities to complement public services, reduce pressure on referral hospitals, and improve access to specialised care where public capacity remains limited.

9. Strengthen mental health data systems to support evidence-based policy, planning, and investment

Mental health indicators should be systematically integrated into routine health, education, and social protection information systems to enable timely surveillance, service planning, and accountability. Government should prioritise standardised reporting, facility and community-level data capture, and interoperability across systems, while investing in periodic surveys and research to address critical data gaps on prevalence, socio-economic determinants, service utilisation, and outcomes.

CHAPTER 1: OVERVIEW OF THE STATE OF UGANDA POPULATION REPORT (SUPRE) 2025

1.0 Introduction

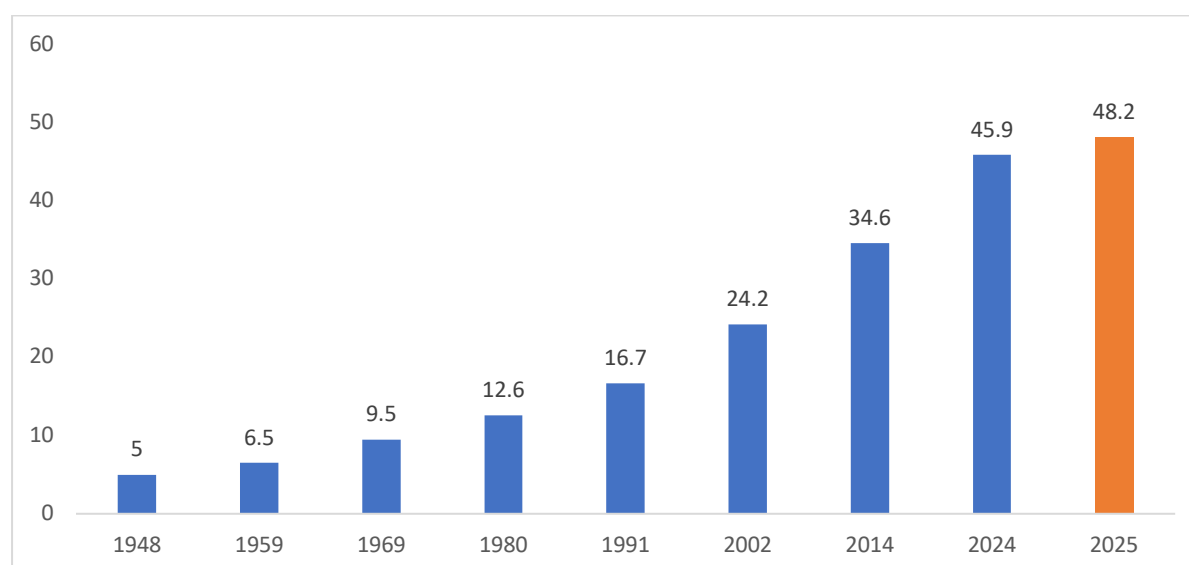
The State of Uganda Population Report (SUPRE) is an annual flagship publication of the National Planning Authority (NPA), produced in collaboration with various partners. It provides evidence-based insights on some critical population and development concerns that shape Uganda's progress towards sustainable socio-economic transformation.

1.1 Snapshot of Uganda's Population 2025

1.1.1 Population Size, Growth, and Distribution

Uganda's population has expanded rapidly and consistently over the past seven decades, underscoring the scale of demographic change confronting the country's development systems. The population increased from 4.9 million people in 1948 to 45.9 million in 2024, comprising 22.3 million males and 23.6 million females, according to the National Population and Housing Census (NPHC) 2024 (Figure 1). Projections indicate that the population will reach approximately 48.2 million people, comprising 23.4 million males and 24.8 million females in 2025, reflecting sustained high fertility alongside gradual improvements in survival and life expectancy.

Figure 1: Uganda's population trends (1948-2025)



Source: UBOS 2024

Historical trends show a steady acceleration in population growth. The annual growth rate rose from 2.0 percent in 1948 to 2.5 percent in 1959, and further to 2.7 percent in 1969. Although it declined slightly to 2.5 percent in 1991, the growth rate surged to 3.2 percent in 2002, marking the highest recorded level during this period. Since then, growth has moderated but remains high, declining to 3.0 percent in 2014 and 2.9 percent in 2024. Even at this reduced

pace, Uganda's growth rate remains among the highest globally, implying that more than one million people are added to the population each year

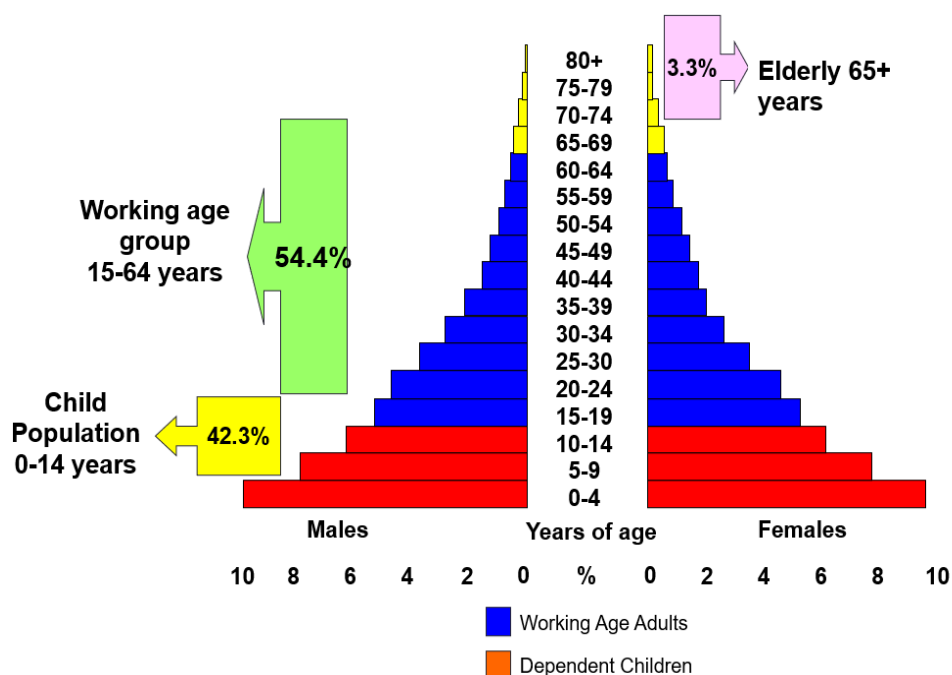
Population growth has been accompanied by increasing spatial concentration and regional variation. While the population remains predominantly rural, urbanisation is accelerating, particularly within the Greater Kampala Metropolitan Area (GKMA) and other emerging secondary cities. These areas attract large numbers of young people in search of education, employment, and services, often resulting in overcrowding, informal settlements, housing insecurity, and heightened exposure to psychosocial stressors. Conversely, many rural areas continue to experience high dependency ratios, subsistence livelihoods, and limited access to health and social services, conditions that can exacerbate psychological distress and constrain help-seeking behaviours.

The scale, pace, and spatial distribution of Uganda's population growth have direct implications for population wellbeing, including mental health. Rapid growth places sustained pressure on education systems, labour markets, health services, and social protection mechanisms, increasing the risk of unmet psychosocial needs, particularly among children, adolescents, and young adults. As the population continues to expand and redistribute, addressing mental health challenges within the broader demographic context becomes essential for safeguarding human capital, enhancing resilience, and advancing Uganda's long-term socio-economic transformation agenda

1.1.2 Population Age Structure and Dependency Patterns

Uganda's population age structure is predominantly youthful, reflecting a long history of persistently high fertility combined with gradual improvements in child survival and overall life expectancy. According to the 2024 National Population and Housing Census, approximately 33.6 million people (73.2 percent) of Uganda's population are below the age of 30 years, making the country one of the youngest populations globally. Children aged 0-14 years account for 19.4 million people (42.3 percent) of the total population, while those aged 15-64 years, considered the economically productive age group, comprise 24.98 million people (54.4 percent). Older persons aged 65 years and above constitute a relatively small but growing share of the population at 1.5 million people (3.3 percent) (Figure 2).

This youthful age structure results in a broad-based population pyramid, characterized by a large child population relative to the working-age population and a correspondingly high dependency burden. High fertility continues to add large cohorts of children annually (1 million per annum), while declining mortality sustains population momentum. Uganda's median age of approximately 15 years underscores the extent to which children and adolescents dominate the population profile. Such an age structure places sustained pressure on households, education systems, health services, and social support networks, particularly in contexts of limited resources and high poverty incidence.

Figure 2: Population Pyramid

Source: UBOS 2024

From a development perspective, this child-heavy age structure presents both challenges and opportunities. In the short to medium term, high dependency ratios (83.8) constrain savings, investment, and per capita service provision, potentially amplifying household stress and psychosocial vulnerability. Over time, however, deliberate investments that accelerate fertility decline, improve survival, and strengthen human capital development can gradually transform the age structure, allowing a growing cohort of young people to enter the working-age population. This demographic transition from a child bulge to a youth bulge creates the conditions for a demographic dividend, provided that education, skills development, employment creation, and health systems, including mental health services, expand commensurately.

At the upper end of the age spectrum, the proportion of older persons, though still modest, is gradually increasing as life expectancy improves. This emerging trend raises important considerations for long-term care systems, social protection, and community support mechanisms. Older persons are more likely to experience social isolation, chronic illness, disability, and loss of income, factors that increase vulnerability to mental health conditions such as depression and cognitive decline. As Uganda's population continues to evolve, the mental health needs of older persons will require increasing policy attention to prevent widening disparities in wellbeing and to ensure inclusive population ageing.

Overall, Uganda's age structure and dependency patterns have profound implications for population wellbeing and mental health across the life course. High child dependency, a rapidly

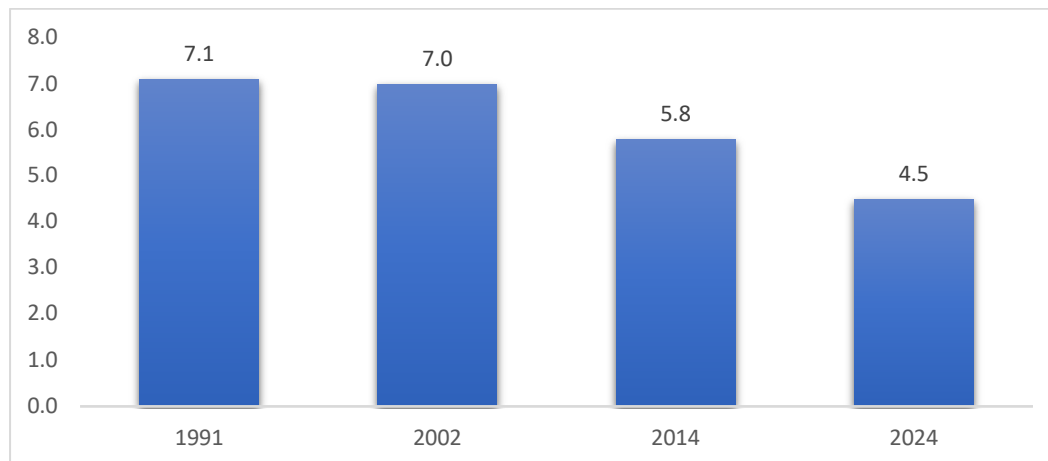
expanding youth population, and a slowly growing older population together shape the scale and nature of mental health needs, reinforcing the importance of age-sensitive, life-course approaches to mental health policy, planning, and service delivery.

1.1.3 Total Fertility Rate

Uganda has experienced a decline in its Total Fertility Rate (TFR) over the past few decades, though at a slow pace. TFR reduced from 7.1 children in 1991 to 4.5 children in 2024 (Figure 3). These changes are attributed to initiatives promoting family planning, improving reproductive health awareness, and enhancing overall access to contraception. The shift reflects evolving societal attitudes toward family size and increased opportunities for women.

The slow pace is, however, a result of a low contraceptive prevalence rate (CPR), early sexual debut, early marriages, and near-universal marital union. In addition, the teenage pregnancy rate is high and has stagnated between 25% and 24% over the past ten years. The situation is worsened by the low status of women and the cultural practices that encourage early marriages, preference for the male child, and large family sizes.

Figure 3: Total Fertility Rates 1991-2024

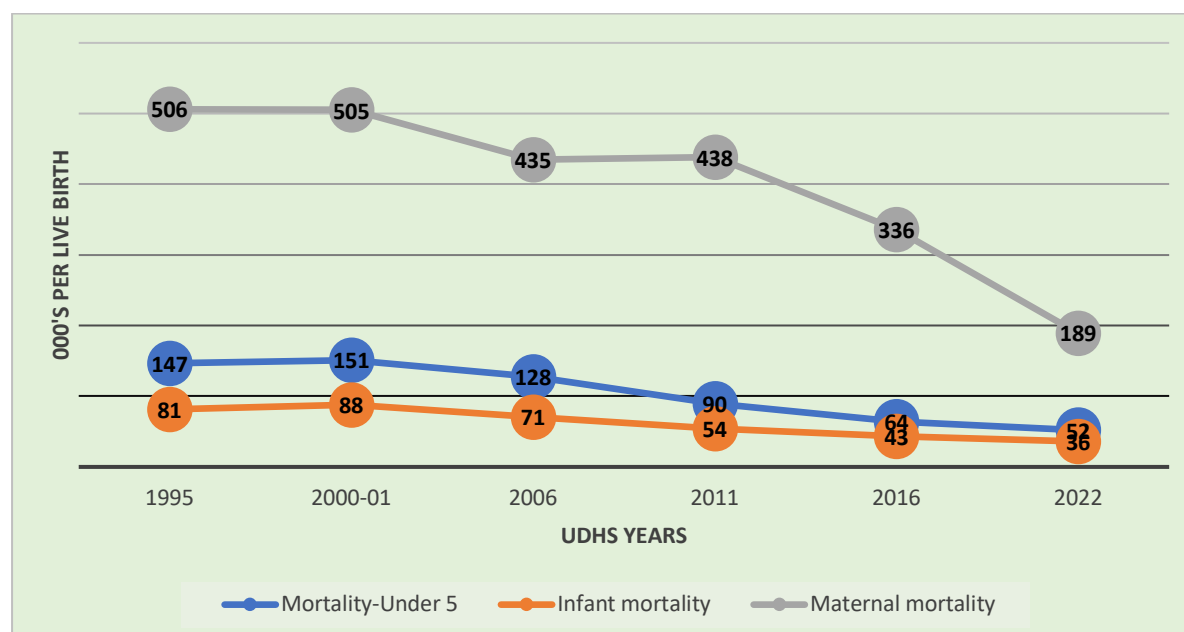


Source: UBOS 2025

1.1.4 Mortality (Infant and Maternal)

Infant and child mortality rates are primary indicators of a country's socio-economic situation and quality of life. Generally, there has been a decline in mortality levels. The Country's Infant Mortality Rate (IMR) has declined from 81 deaths per 1,000 live births in 1995 to 43 deaths to 36 live births in 2022. The under-five mortality rate declined from 147 deaths per 1,000 live births in 1995 to 52 in 2022.

In addition, Maternal Mortality Rate (MMR) has declined from 506 deaths per 100,000 live births in 1988 to 336 deaths per 100,000 live births in 2016 to 189 deaths per 100,000 live births in 2023. (Figure 4).

Figure 4: Mortality (Infant, Maternal, and Under Five) trends 1995-2022

Source: UDHS 2023

1.1.5 Education and Literacy

Uganda's education and literacy profile shows steady progress alongside persistent gaps across the education cycle. Early Childhood Education attendance was limited, with 41.6% of children aged 3-5 years enrolled in 2024, indicating that a majority still enter primary school without early learning exposure. At the primary level, access was relatively high, with 75.2% of children aged 6-12 years attending school. In total, 8.9 million learners were enrolled in primary education in 2024, of whom 5.8 million were of official primary school age.

Transition and retention challenges become more pronounced at the secondary level, where 62% of adolescents aged 13-18 years were attending school. In terms of educational attainment, 39% of the population aged 10 years and above had attained some primary education, while 20.2% had no formal education, reflecting historical and socio-economic inequalities.

Literacy outcomes mirror these patterns, with 74% of the population aged 10 years and above being literate, of which 76.6% were males, pointing to a persistent but narrowing gender gap in basic literacy skills (UBOS, 2024).

1.1.6 Labour force participation

According to Census 2024, Uganda's population of working age (14-64) was 25.2 million persons (56.7% of the population) and projected to reach 28.5 million persons (57.5% of the population) in 2025. However, only 37.5 percent of this working-age population (9.4 million people) was employed in 2024, pointing to limited absorption of labour by the economy. The labour force participation rate was 42.9 percent (10.8 million people), while 1.36 million individuals (12.6%) of the labour force, were unemployed. Among youth aged 15-24, the unemployment rate stands at 16.7%, with young women disproportionately affected: 18.7% of

female youth are unemployed compared to 14.7% of their male counterparts. These barriers must be addressed to fully harness the Demographic Dividend.

Of particular concern is the high prevalence of youth exclusion from economic and human capital development systems, with 5.25 million young people aged 18-30 years (50.9%) not in employment, education, or training (NEET). This combination of low employment absorption, high unemployment, and widespread youth disengagement highlights a critical mismatch between population dynamics and economic opportunity, with far-reaching implications for income security, social stability and mental health outcomes.

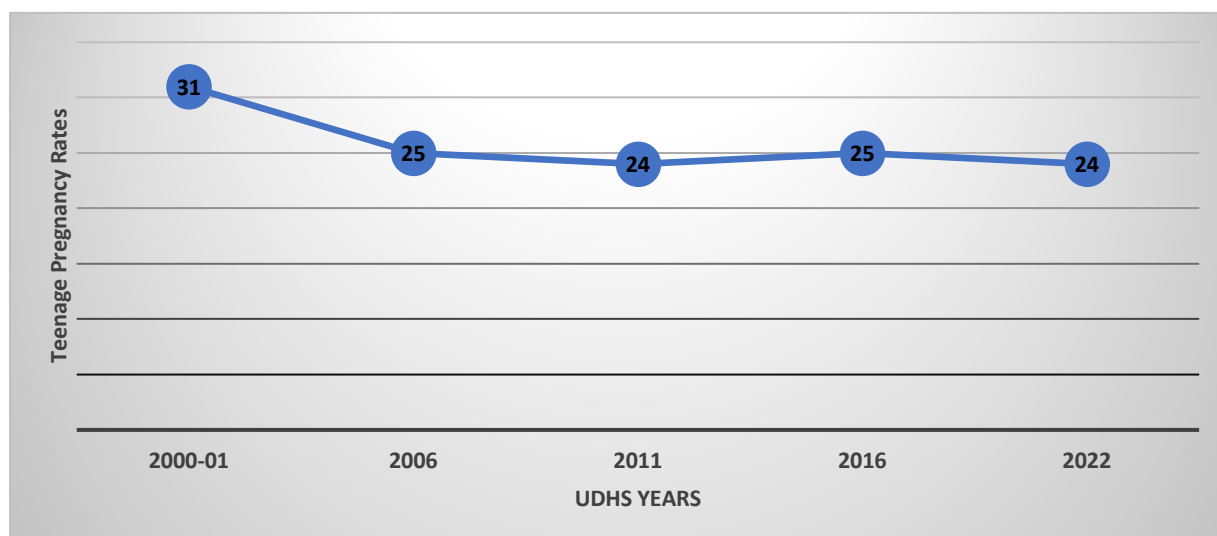
1.1.7 Urbanization

Uganda is undergoing rapid urbanization, with an urban population of about 17.5 million people (38%). The combined daytime population across the eleven cities was 5,547,645 people in 2024, underscoring the intensity of daily urban mobility and the functional importance of cities beyond their resident populations. Kampala Capital City dominates the urban system with a daytime population of 2,503,174, accounting for nearly half of the total across all cities, reflecting its role as the country's primary economic, administrative, and service hub. Secondary cities such as Arua (440,540), Mbale (371,626), Jinja (363,134), Masaka (328,485), Mbarara (324,974), Gulu (323,888), and Lira (304,057) each attract daytime populations exceeding 300,000 or close to it, indicating strong regional pull factors linked to trade, services, education, and administration. Smaller but rapidly growing cities including Soroti (220,698), Hoima (190,075), and Fort Portal (176,994) also register substantial daytime inflows, reflecting emerging urban economies and sectoral activities such as oil and gas, tourism, and regional services.

Overall, the daytime population pattern illustrates the growing functional role of Uganda's cities in absorbing labour, facilitating economic activity, and delivering services, while simultaneously placing significant pressure on urban infrastructure, transport systems, housing, sanitation, and social services, with important implications for urban planning and service provision (UBOS, 2024).

1.1.8 Teenage Pregnancy and Child Marriage

Uganda's teenage pregnancy rate decreased from 31% in 2000-01 but remained relatively stable between 25% and 24% from 2006 to 2022 (Figure 5). This is comparably low with the 28% teenage pregnancy rate in Sub-Saharan, West and Central African countries. These pregnancies have severe health impacts. Young mothers in Uganda risk poor maternal and child health, being isolated, attempting unsafe abortions, failing to continue with school and poverty.

Figure 5: Trends of Teenage Pregnancy 2000-2022

Source: UDHS 2022

According to the 2024 NPHC, adolescents aged 10-19 constitute 24.9% (11.4 million) of the total population (45.9 million), making adolescent health and well-being a critical priority for national planning. With a population growth rate of 2.9%, the adolescent population is projected to expand rapidly, further underscoring the urgency of targeted investments.

A significant proportion of adolescents engage in sexual activity at a young age. Among women aged 20-49, 20% had their first sexual experience before age 15, and 66% before age 18. For men in the same age group, 10% initiated sex before age 15, and 47% before age 18. These figures underscore the urgent need to provide adolescents with comprehensive sexual and reproductive health (SRH) information and services.

Approximately one in four (24%) girls in Uganda aged 15-19 years have had a child or are pregnant and 42% of all the pregnancies among adolescents are unintended. Child marriage remains a significant concern in Uganda. Among adolescents aged 15-19, 17% of women and 1% of men were in a marital union in 2022, with about 34% of Ugandan girls married by 18 years, which contributes to school dropout, maternal health risks (as 28% of maternal deaths are by teenage mothers), and intergenerational poverty.

1.1.9 Gender-Based Violence

Gender-based violence (GBV) is an everyday threat for Ugandan women and girls. 56% of married women aged 15-49 suffer physical or sexual violence by a husband (UBOS, 2021). More than one in three women (36%) experience sexual violence, most often from a partner. Child sexual abuse is also pervasive, with 59% of women reporting sexual abuse in childhood. Also, 33% of girls below the age of 15 years were forced to have their first sex. Among women who experience GBV, only a minority report it to the Police.

1.1.10 Poverty

Uganda's national poverty rate has shown a notable decline in recent years. According to the Uganda National Household Survey (UNHS) 2023/24, the national poverty rate fell from 20.3% in 2019/20 to 16.1% in 2023/24, reflecting a reduction of roughly four percentage points and a drop in the number of people living below the national poverty line to around 7 million individuals.

Rural areas continue to bear the larger share of poverty, with the rural poverty rate decreasing from 23.4% to 19.4% over the same period, while urban poverty declined more modestly from 11.7% to 10.3%.

Despite this overall progress, significant regional disparities remain; for example, Karamoja's poverty rate remains very high at over 74%, illustrating pockets of entrenched deprivation that require targeted interventions. These trends underscore long-term progress in poverty reduction, but they also highlight ongoing challenges in lifting vulnerable populations and preventing non-poor households from falling below the poverty line, particularly in underperforming regions.

1.2 Background

While the 2025 State of World Population Report focuses on fertility decline below the replacement level of 2.1 births per woman, highlighting challenges such as labour shortages, population ageing, and pressure on pension and care systems in many developed countries, Uganda faces a different demographic reality. With a total fertility rate of about 4.5 births per woman, the country continues to experience rapid population growth and a high child and youth dependency burden. Uganda's central challenge is therefore not population decline, but how to effectively support, nurture, and transition its large youthful population into a healthy, skilled, and productive workforce capable of driving socio-economic transformation.

Against this backdrop, SUPRE 2025 focuses on **"Mental Health: A Silent Emergency,"** recognizing mental health as a critical yet under-addressed determinant of Uganda's demographic and development outcomes. High fertility, large household sizes, and socio-economic stressors associated with rapid population growth place significant psychological strain on families, caregivers, and young people. At the same time, poor mental health undermines educational attainment, labour productivity, and social cohesion, weakening the country's ability to harness its demographic potential. Addressing mental health is therefore not separate from the fertility and population agenda; it is central to converting Uganda's youthful population into a resilient human capital base that can sustain inclusive growth and long-term development.

The report was developed against the background of Uganda's ambitious tenfold growth strategy to expand the economy from USD 50 billion in 2023 to USD 500 billion in 2040 where addressing mental health issues is key to achieving this strategy. The strategy recognises that sustained economic growth is not achievable without deliberate investment in a healthy, skilled, productive, and ethical labour force. As the first of three five-year development plans under Uganda's Ten-Fold Growth Strategy, NDP IV places Human Capital Development across the life cycle at the centre of socio-economic transformation. However, evidence indicating

that nearly 23 percent of children under the age of 18 experience mental health conditions signals a significant risk to the future workforce on which this growth agenda depends. Without timely prevention, early identification, and effective management of mental health challenges, Uganda risks undermining educational attainment, labour productivity, and long-term economic competitiveness. Addressing mental health is therefore not only a health priority but a strategic investment in building a resilient and productive workforce capable of driving the country's development aspirations.

NDP IV represents the final national development plan under the 2030 Agenda for Sustainable Development, placing heightened responsibility on Uganda to consolidate progress across the Sustainable Development Goals (SDGs). Mental health has emerged as a critical, yet under-addressed, enabler of sustainable development, with direct linkages to poverty reduction (SDG 1), food security (SDG 2), health and wellbeing (SDG 3), education outcomes (SDG 4), gender equality (SDG 5), decent work and economic growth (SDG 8), and the reduction of inequalities (SDG 10). By dedicating SUPRE 2025 to mental health, the National Planning Authority and its partners aim to elevate national awareness of mental health as a development priority, strengthen the evidence base, and inform more responsive policy, planning, and investment decisions. The report issues a clear call to action for government, civil society, the private sector, and communities to collectively prioritize mental health as a foundation for building a healthy, skilled, productive, and resilient population capable of driving Uganda's socio-economic transformation.

Globally, more than 1 billion people representing about 13.6% of the world's population, are living with mental health disorders (WHO, 2024). The prevalence is slightly higher among females (14.8%) compared to males (13%). Anxiety disorders affect an estimated 284 million people, depression impacts 264 million, and alcohol and drug-related disorders affect over 178 million people combined (WHO, 2023). Despite this burden, mental health remains underfunded, accounting for less than 2% of health budgets worldwide.

Across the African Region, more than 116 million people were already estimated to be living with mental health conditions, including 100 million people with clinical depression, of which 66 million people are women (Africa CDC, 2023). As of 2023, 15.3% of youth under 19 years suffered from depression, 11.8% from anxiety disorders, and 12.5% from PTSD disorders.

The situation has been aggravated by poverty, unemployment, conflict (post-war trauma) substance abuse, gender-based violence, and communicable diseases such as HIV/AIDS, COVID 19, and Ebola. Stigma is widespread, with many communities attributing mental illness to supernatural causes, and service delivery is extremely limited. Africa averages only 1.4 mental health workers per 100,000 people, compared to the global average of 9.

Uganda mirrors these global and regional challenges. The 2024 National Population and Housing Census revealed that 12% of persons aged 10 years and above experienced probable psychological distress, with 7.8% showing symptoms of depression and 0.9% reporting suicidality. Applied to Uganda's population of 45.9 million, this translates into millions of



individuals struggling with untreated conditions. Earlier studies estimated that as many as 16 million Ugandans (32% of the population)¹ live with some form of mental illness, a sharp rise from 24% a decade ago.

In Northeastern Uganda (Karamoja and Teso), recent surveys indicate that 26.6% of children exhibit symptoms of anxiety and 8.6% depression, driven by food insecurity, poverty, and cycles of conflict. In urban centers such as Kampala and Wakiso, stress-related conditions are rising, linked to unemployment, substance abuse, and cyber-related anxieties. Although service availability is somewhat better than in rural regions, stigma and underfunding still limit timely access to care.

Broader socio-economic conditions continue to intensify the burden of mental illness in Uganda. Findings from the 2024 National Population and Housing Census indicate that one in three households (33.1%) remains trapped in subsistence livelihoods, while more than half of young people aged 18-30 years (50.9%) are not in employment, education, or training (NEET). Existing evidence consistently shows that such conditions, characterized by poverty, unemployment, gender-based violence, stigma, and substance abuse significantly heighten the risk of psychological distress and mental health disorders.

Against this backdrop, there is an urgent need for a comprehensive, evidence-driven assessment that situates mental health within Uganda's population dynamics and development trajectory under NDP IV. The State of Uganda's Population Report 2025 (SUPRE 2025) responds to this need by elevating mental health as a national development concern and providing a robust analytical foundation to guide policy, planning, and investment decisions.

1.3. Rationale

Mental health has increasingly emerged as a significant, yet insufficiently visible, public health and development concern in Uganda. Available evidence suggests that a substantial proportion of the population experiences mental health conditions, with depression and anxiety among the most prevalent. Recent studies estimate that approximately 22.9% of children and 24.2% of adults live with some form of mental illness, including anxiety disorders, depression, post-traumatic stress disorder (PTSD), and other related conditions (Asiimwe, 2023). However, these figures likely understate the true magnitude of the challenge, as stigma, limited awareness, cultural perceptions, and barriers to access continue to suppress reporting and help-seeking, particularly in rural, peri-urban, humanitarian settings and populations in war-affected regions. In addition, Uganda's large refugee population and population in war-affected regions, many of whom have experienced conflict, displacement, and trauma, places additional demand on mental health and psychosocial support systems.

¹ MarMohan Kaggwa, 2022.

While Uganda has made efforts to integrate mental health into the broader health system, service coverage remains constrained by longstanding resource, workforce, and infrastructure limitations. Public financing for mental health remains modest relative to overall health sector demands, and the availability of specialized mental health professionals is limited², with services heavily concentrated in urban centers. As a result, a large proportion of individuals with mental health conditions do not receive timely or appropriate care, relying instead on informal coping mechanisms or delayed interventions. These systemic constraints reflect broader health system pressures, underscoring the need for strengthened prioritization, coordination, and investment.

The implications of unmet mental health needs extend well beyond the health sector, with significant social and economic consequences. Mental health conditions continue to affect educational participation, labour productivity, and income security, while also placing emotional and financial strain on households and communities. Uganda's predominantly young population, that is, over 73% of whom are below the age of 30, is particularly vulnerable, as untreated mental health challenges among children and young people can result in long-term disengagement from education, skills development, and employment. In addition, Uganda's large refugee population, many of whom have experienced conflict, displacement, and trauma, places additional demand on mental health and psychosocial support systems.

Against this backdrop, there is a clear need for a comprehensive, population-based assessment that situates mental health within Uganda's broader demographic, social, and economic context. By examining the prevalence, distribution, drivers, and impacts of mental health conditions, SUPRE 2025 seeks to strengthen the national evidence base and support more informed, integrated, and forward-looking policy and planning responses that recognise mental health as a foundational component of human capital development and sustainable transformation.

1.4 Objectives

The overarching objective of the State of Uganda's Population Report 2025 is to generate and disseminate evidence on the prevalence, distribution, determinants, and impacts of mental health in Uganda to inform policy, planning, and investment.

1.4.1 Specific objectives.

The specific objectives of the SUPRE 2025 are to:

- a) Assess the prevalence and distribution of mental health in Uganda at national and regional levels.
- b) Examine the demographic and socioeconomic determinants of mental health.

² Uganda has only about 70 psychiatrists for a population of nearly 46 million, most of whom are based in Kampala (World Bank, 2023)



- c) Assess the cost of mental health and inaction (human and economic burden in Uganda).
- d) Assess the system readiness and human resource analysis (infrastructure and workforce) that supports mental health.
- e) Propose policy implications and recommendations to address mental health.

1.5 Approach and Methodology

This report follows a structured but integrated approach. Chapters One and Two set the foundation by introducing and describing the mental health situation in Uganda, including the policy context and key challenges. The remaining chapters, Three, Four, and Five, are written as self-contained analytical papers, each focusing on a specific aspect of mental health. While each chapter can stand on its own, they are closely linked in terms of concepts, data, and policy relevance. Detailed methods are explained within each chapter; this section provides an overall summary of how the study was designed and carried out.

Chapter Three examines the socio-economic factors associated with mental health outcomes in Uganda. Ideally, a more comprehensive analysis incorporating a wider range of variables and longitudinal data would have been undertaken to fully capture the complexity of mental health dynamics across the life course. However, the analysis was constrained by significant data limitations, as mental health indicators remain sparsely and inconsistently captured in national datasets. Within these constraints, the chapter adopts a life-course perspective, informed by Human Capital Theory and the Social Determinants of Health framework, to examine how factors such as education, income, employment, and living environment influence mental health over time. Quantitative methods, including correlation analysis and multivariate regression techniques, are applied to assess the direction and relative strength of these relationships using available data. To address key evidence gaps, the analysis is complemented by a review of published studies and grey literature, which helps to contextualize the findings and strengthen interpretation in a data-scarce environment.

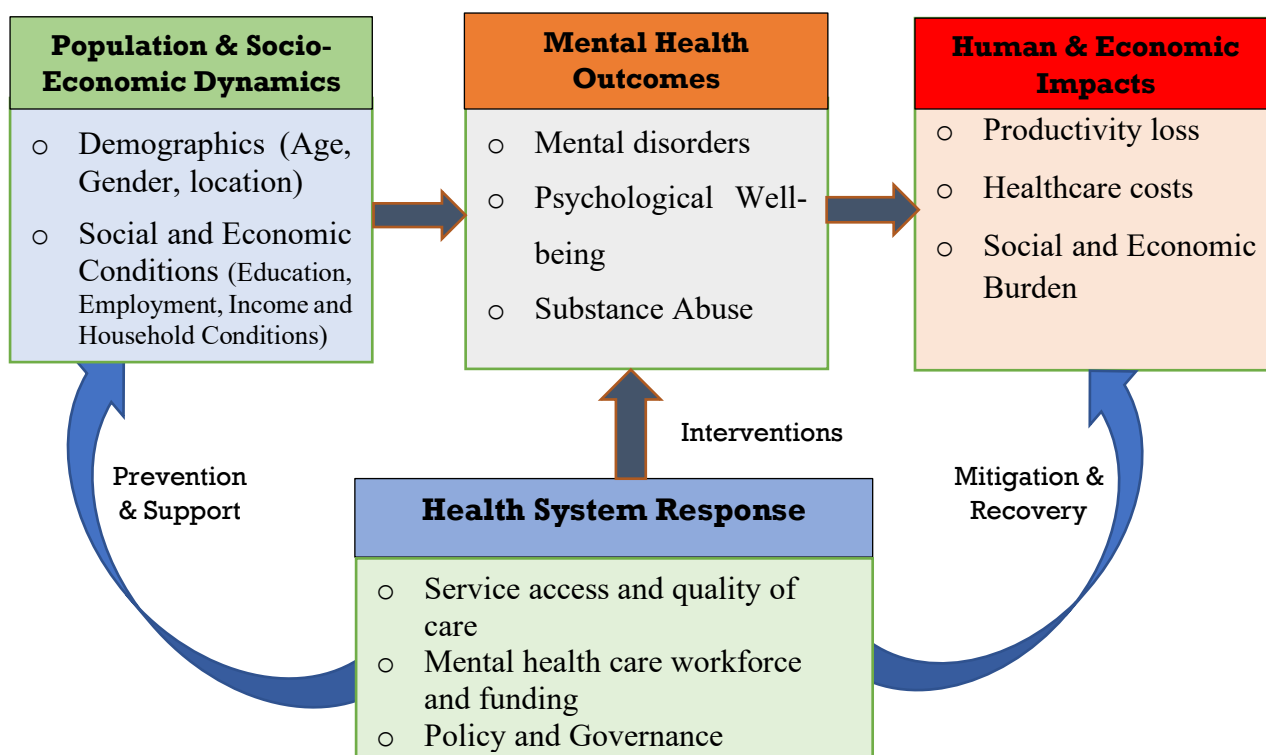
Chapter Four examines the human and economic costs of mental illness, with particular attention to the cost of inaction. The chapter uses a scenario-based approach within a cost-of-illness framework to connect mental health conditions with economic outcomes. Both direct costs, such as health care spending, and indirect costs, including lost productivity and reduced earnings, are considered. By linking disease prevalence with economic losses, the analysis provides a clear picture of how mental illness affects individuals, households, the wider economy and highlights the long-term consequences of underinvestment in mental health services.

Chapter Five assesses the readiness of Uganda's mental health system to respond to current and future needs. The chapter uses a mixed-methods approach that combines quantitative surveys with qualitative interviews. Facility surveys were conducted at different levels of care, including Health Centre IIIs and IVs, General Hospitals, and Regional and National Referral Hospitals, across all regions of the country. These surveys examined staffing levels, service availability, infrastructure, financing, and service use (Aber et al., 2023; Katende et al., 2015). In addition, interviews and focus group discussions with health managers, community health workers, and specialists provided practical insights into everyday challenges and opportunities for strengthening service delivery.

1.6 Conceptual Framework underpinning the report

The overarching conceptual framework that guides this report was designed to reflect mental health as a population-wide issue that is shaped by social and economic conditions, affects human and economic outcomes, and is influenced by the strength of the health system. It provides a common structure for understanding how the determinants analysed in Chapter Three, the costs examined in Chapter Four, and the system readiness and response assessed in Chapter Five are interlinked. Bringing these elements together in a single framework, Figure 6 provides a holistic interpretation of the evidence and reinforces the report's focus on mental health as a core component of Uganda's population dynamics and development trajectory.

Figure 6: Conceptual Framework informing the State of Population Report of Uganda 2025



Source: Author

Figure 6 shows that mental health outcomes are shaped by broader population and socio-economic conditions such as age structure, education, employment, income, and life transitions, and that these outcomes have direct consequences for individual well-being, productivity, and national economic performance. At the same time, the figure emphasizes the central role of the health system in shaping these relationships. The availability and quality of mental health services influence whether mental health challenges translate into long-term human and economic losses or are effectively prevented, managed, and reduced. Taken together, the framework highlights mental health as both a social outcome and a driver of development, highlighting the importance of early action and sustained system investment for Uganda's population's future.

1.7 Limitations in data availability

The development of SUPRE 2025 utilized different data sources including; Health Management Information System (HMIS), World Development Indicators (WDI), Institute for Health Metrics and Evaluation (IHME), World Health Organization (WHO) and primary data collected from selected health facilities.

The situation analysis of this report was based on HMIS data reviews due to limited mental health data in the Censuses and surveys conducted in Uganda. HMIS data has limitations in the coverage of key mental health indicators and data quality. HMIS data is fragmented and underreported, lacking detailed disaggregation by age, sex, disability, and socio-economic factors. This hinders understanding of mental health inequities as it only provides partial insights, often relying on reported cases in health facilities.

Variables used to determine socio-economic drivers of mental health are at the macro-level and might not bring out some of the micro-level details of mental illness among individuals and or households. Besides, using data on Disability-Adjusted Life Years (DALYs) as a proxy for mental illness has several weaknesses which includes tracking a clear transmission mechanism through which the independent variables affect the dependent variable.

Primary data used in the assessment of system readiness was limited by data availability, as many health facilities did not have comprehensive or consistent records on mental health, and data on the socio-economic determinants of mental illness were largely unavailable, constraining regression analysis. Notwithstanding these limitations, the findings of this report are intended to stimulate greater attention among duty bearers to the need for systematic, routine, and integrated data collection on mental health as a foundation for evidence-informed policy, planning, and investment.

1.8 Structure of the Report

This report is organised into five substantive chapters. Chapter One introduces the State of Uganda's Population Report 2025, outlining the background, rationale, objectives, conceptual framing, and policy context underpinning the focus on mental health as a population and development concern. Chapter Two presents a comprehensive situation analysis of Uganda's demographic profile and population dynamics, examining how trends in population growth, age structure, urbanisation, migration, and health transitions shape mental health risks and vulnerabilities across the life course. Chapter Three analyses the key socio-economic and environmental drivers of mental health, including poverty, education, employment, social conditions, and structural inequalities. Chapter Four quantifies the human and economic costs of mental illness and the consequences of inaction, highlighting the implications for productivity, human capital development, and national transformation. Chapter Five assesses system readiness and response, with a focus on mental health infrastructure, workforce capacity, service availability, financing, and governance, and concludes with policy implications and recommendations to inform integrated, evidence-based planning and investment.

CHAPTER 2

SITUATION ANALYSIS

2.1 Introduction

This Chapter presents a comprehensive situation analysis of Uganda's population dynamics and their implications for mental health and development outcomes. It situates mental health within the broader demographic, epidemiological, and socio-economic context, recognising that population size, growth, age structure, mobility, and health transitions shape exposure to psychosocial stressors and influence patterns of mental wellbeing across the life course. Rather than treating mental health as an isolated health condition, the chapter adopts a population and development lens to examine how demographic pressures interact with social and economic conditions to generate mental health risks and vulnerabilities at individual, household, and community levels.

Drawing on the latest population, health, and socio-economic evidence, the chapter analyses how Uganda's youthful population structure, rapid population growth, urbanisation, and mobility trends intersect with broader health and development challenges to affect mental health outcomes. The situation analysis also highlights spatial, gender, and age-related disparities, as well as gaps in financing and data systems, to inform a nuanced understanding of the mental health landscape. Together, the chapter provides an empirical foundation for subsequent analysis of drivers, costs, and system readiness, and underscores why mental health constitutes a critical population and development concern under NDP IV.

2.2 Population Dynamics and Demographic Context

Uganda's population dynamics provide the structural context within which mental health risks and resilience are shaped. The country has experienced sustained population growth, resulting in a rapidly expanding population characterized by a pronounced youth bulge, high dependency ratios, increasing urbanization, and evolving household structures. These demographic features create both opportunities and pressures for development, while simultaneously influencing exposure to psychosocial stressors across different population groups.

This section examines age structure, and settlement patterns, with particular attention to how these dynamics affect mental health vulnerability across the life course. Rapid population growth places strain on education, health, employment, and social support systems, while a youthful age structure concentrates mental health risks among children, adolescents, and young adults navigating critical developmental and transitional stages. Urbanization and changing household arrangements further reshape social support networks, living conditions, and access to services, with implications for stress, social isolation, and coping capacity. By situating mental health within these demographic realities, this section establishes a foundation for understanding subsequent patterns of mental illness and wellbeing discussed in the chapter.



2.2.1 Age Structure and Mental Health in Uganda

The analysis of mental health across Uganda's age structure aligns with the outpatient HMIS 105 and inpatient HMIS 108 reporting formats, ensuring consistency with national health reporting standards. Understanding mental health patterns across age groups is critical for targeted interventions, as risk factors, prevalence, and service access vary across the life cycle.

A. Early Childhood (0-9 years)

Mental health in early childhood remains poorly documented in Uganda, but global estimates suggest that one in seven children experiences a mental disorder, often related to developmental, behavioral, or emotional issues. In Uganda, risk factors include poverty, malnutrition, HIV/AIDS, domestic violence, and displacement, which increase children's vulnerability to emotional distress. Common conditions among children include developmental delays, behavioral disorders, trauma from abuse or neglect, and anxiety. The scarcity of child-specific mental health services means early warning signs often go undetected. Strengthening early childhood psychosocial support, diagnostics, reporting and enhancing parental awareness are critical to preventing long-term consequences.

B. Adolescents (10-19 years)

Adolescents in Uganda face a growing burden of mental health challenges, both within and outside the school system. Emerging evidence from practitioners, educators, and service providers suggests that in-school adolescents are increasingly experiencing psychological distress linked to domestic violence, family instability, biological and hormonal changes associated with adolescence, academic performance pressures, and peer influence. While these concerns are widely reported in practice, they are not yet systematically captured in official education or health statistics, pointing to a significant gap in routine data and surveillance within schools and training institutions. The absence of structured, school-based mental health reporting limits the timely identification of at-risk learners and constrains evidence-informed responses within the education system.

Available empirical studies nevertheless indicate that the scale of adolescent mental illness is substantial. Research conducted in Central and Eastern Uganda found that 65% of adolescents exhibited mental health difficulties, with anxiety affecting 44% and disruptive behavioural disorders 31% (Nakigudde et al., 2024). In Kampala, 66% of adolescents reported poor mental well-being, and 41% experienced multiple forms of childhood trauma (Ndyanabangi et al., 2023). Out-of-school adolescents and youth face heightened vulnerability due to poverty, social isolation, and limited access to psychosocial support, further compounding mental health risks. Together, these findings highlight the urgent need to strengthen school and institutional readiness through functional mental health reporting and tracking systems, trained personnel, guidance and counselling structures, clear referral pathways, and strong linkages to health and social services. Establishing inclusive, school- and community-based mental health

programmes is critical to enable early prevention and response, safeguard adolescent wellbeing and learning outcomes, and protect Uganda's long-term human capital development.

C. Youth and Young Adults (20-34 years)

Youth and young adults in Uganda face heightened vulnerability to mental illness due to the convergence of demographic pressure, labour market exclusion, and socio-economic insecurity that characterise this life stage. This age group constitutes the largest share of Uganda's working-age population and absorbs the bulk of the 600,000 -700,000 new labour market entrants each year, over 95 percent of whom are young people.

However, employment creation has not kept pace with population growth, resulting in widespread unemployment, underemployment, and prolonged school-to-work transitions. The National Labour Force Survey (2021) indicates that approximately 51 percent of Ugandan youth equivalent to about 4.2 million individuals, are neither in employment, education, nor training (NEETs), with higher concentrations among those in their twenties, females, rural residents, and individuals from poor households.

Although nationally representative data on mental health outcomes among young adults remain limited, these structural conditions including persistent joblessness, income insecurity, early family responsibilities, gender-based violence, HIV-related stigma, and substance abuse, are well-established risk factors for psychological distress, depression, anxiety, and harmful coping behaviours. Evidence from the NEETs analysis further shows that prolonged disengagement from work and training is associated with social isolation, discouragement, and increased vulnerability to substance use, particularly alcohol.

Given that young adults are central to Uganda's labour force, productivity, and demographic dividend aspirations under NDP IV, compromised mental wellbeing within this cohort poses significant risks to employability, human capital formation, and social stability. Strengthening routine data systems to capture mental health indicators alongside employment, education, and social protection metrics for young adults is therefore critical to informing integrated prevention, labour market, and psychosocial support interventions.

D. Adults (35–59 years)

Adults experience a high prevalence of mental health disorders due to multiple social and economic stressors, including workplace pressure, caregiving responsibilities, poverty, and chronic illness. Depression, anxiety, trauma-related disorders, and substance use are common. Evidence indicates that 24.2% of adults suffer from a mental disorder, with depression affecting 21.2% and anxiety 20.2% (Kinyanda et al., 2021). Limited integration of mental health into primary care and workplace systems means many conditions remain untreated, reducing productivity and quality of life.

E. Elderly (60 years and above)

Although older adults constitute a small proportion of the population (~3.3%), mental health challenges are increasingly significant. Common conditions include depression, anxiety, and dementia, exacerbated by social isolation, poverty, chronic illness, and lack of elderly care services. Among elderly individuals living with HIV, 16.8% report probable anxiety disorders (Kuteesa et al., 2025). As Uganda's population gradually ages, the mental health needs of older adults will grow, requiring targeted psychosocial and medical interventions.

2.2.4 Urbanization, Settlement Patterns, and Household Structure

Uganda is undergoing a gradual but accelerating process of urbanisation, accompanied by changing settlement patterns and household structures that have important implications for population wellbeing and mental health. Although the country remains predominantly rural, the 2024 National Population and Housing Census indicates a continued shift of the population towards urban areas, particularly within the Greater Kampala Metropolitan Area (GKMA) and emerging secondary towns. Urban centres increasingly attract young people seeking education, employment, and services, reshaping population distribution and intensifying demographic pressure in cities and peri-urban settlements.

Rapid urban growth has not always been matched by commensurate expansion in housing, infrastructure, and basic services. As a result, many urban residents, especially youth and low-income households live in informal settlements characterised by overcrowding, insecure tenure, inadequate sanitation, and limited access to social services. These conditions heighten exposure to environmental stressors, social fragmentation, and economic precarity, which are closely associated with psychological distress, anxiety, and harmful coping behaviours. Urban living may offer greater proximity to health and education services, but persistent inequality and high living costs often offset these advantages, particularly for recent migrants and informal sector workers.

Settlement patterns in rural areas also shape mental health risks and resilience. Rural households continue to be widely dispersed, with livelihoods largely dependent on subsistence agriculture (33.1%) and natural resources. While rural communities may benefit from stronger social cohesion and kinship networks, they often face limited access to health care, education, and psychosocial support services. Long distances to service points, poverty, and stigma can delay help-seeking for mental health conditions, increasing the likelihood of untreated distress and prolonged disability.

Household structure in Uganda is evolving alongside urbanisation and socio-economic change. Extended family arrangements remain common, but there is a growing prevalence of nuclear households, single-parent families, and child- or youth-headed households, particularly in urban and peri-urban settings. These shifts are influenced by labour migration, economic pressures, family breakdown, and mortality. Changes in household composition can weaken traditional support systems, increase caregiving burdens, especially for women and heighten

vulnerability to stress, domestic violence, and social isolation. For children, adolescents, and older persons, household instability may exacerbate emotional distress and undermine protective environments essential for mental wellbeing.

Overall, urbanisation, settlement patterns, and household structures interact with population growth and age composition to shape the spatial and social distribution of mental health risks in Uganda. As cities expand and family arrangements continue to evolve, there is a growing need for more prioritization of urban planning, housing policy, and social services that are responsive to psychosocial wellbeing. Integrating mental health considerations into urban development, community planning, and household-level interventions will be essential to address emerging vulnerabilities and promote resilience across diverse population settings.

2.3 Population Health Profile and Epidemiological Context

Uganda's population health profile provides critical context for understanding the scale, patterns, and determinants of mental health outcomes within the broader epidemiological transition. As the country experiences sustained population growth, a youthful age structure, and increasing urbanisation, it simultaneously faces a dual burden of communicable and non-communicable diseases, alongside persistent reproductive health challenges and emerging chronic conditions. These health dynamics interact closely with mental health, both as shared risk factors and as co-morbid conditions that compound vulnerability, disability, and health system strain. This section situates mental health within Uganda's evolving morbidity and mortality landscape, examining how demographic change, disease patterns, and longevity influence mental wellbeing across the life course, and setting the foundation for a focused analysis of mental health status.

2.3.1 Mortality and Morbidity Trends

Uganda has historically experienced high levels of mortality arising from a combination of preventable disease burden, delayed health-seeking behaviour, financial barriers to care, and uneven access to quality health services. Over the past three decades, however, the country has recorded sustained declines in major forms of mortality including neonatal, infant, child, and maternal, reflecting gradual improvements in health system capacity, service coverage, and socio-economic conditions. The Maternal Mortality Ratio (MMR) declined from 506 deaths per 100,000 live births in 1995 to 189 deaths per 100,000 live births in 2022, attributable in part to increased uptake of family planning, expanded skilled birth attendance, improved emergency obstetric care, and rising levels of female education.

Child survival has also improved markedly. Under-five mortality fell from 151 deaths per 1,000 live births in 2001 to 52 per 1,000 in 2022, driven by gains in immunisation coverage, malaria prevention and treatment, nutrition interventions, and strengthened maternal and newborn health services. Infant mortality declined from 122 deaths per 1,000 live births in 1988/89 to 36 per 1,000 in 2022. In contrast, neonatal mortality has stagnated at approximately 22 deaths per 1,000 live births, indicating persistent challenges in perinatal and immediate postpartum



care. These patterns highlight uneven progress across the continuum of care and underscore the need for integrated interventions that address both biomedical and psychosocial determinants of survival.

Despite improvements in mortality outcomes, declining death rates continue to interact with fertility behaviours and household decision-making. In some communities, perceptions of child survival risk encourage families to have more children as a protective strategy, contributing to sustained high fertility and large household sizes. These demographic responses can intensify financial and emotional strain within households and dilute caregiving resources, creating environments where children may receive limited psychosocial support, with potential consequences for emotional development and mental wellbeing.

Uganda has registered substantial improvements in survival outcomes over the past two decades, with life expectancy at birth increasing from 48.9 years in 2001 to about 66 years in 2022 and 68 years in 2024, reflecting sustained reductions in infectious disease mortality and notable gains in maternal and child health. Over the same period, infant mortality declined from 88 deaths per 1,000 live births in 2001 to 36 in 2022, while neonatal mortality reduced from 33 to 22 deaths per 1,000 live births, albeit at a slower pace. Maternal mortality has also fallen significantly, from 505 deaths per 100,000 live births in 2001 to 189 in 2022, underscoring progress in reproductive health services, skilled birth attendance, and emergency obstetric care. However, these survival gains are occurring alongside a marked epidemiological transition. The burden of non-communicable diseases has risen steadily, with the proportion of NCDs in the overall disease burden increasing from 16.8 percent in 2001 to 41 percent in 2022, signaling a rapid shift towards chronic and long-term health conditions. Within this evolving morbidity profile, mental, neurological, and substance-use disorders are increasingly prominent but remain under-diagnosed and under-reported. The coexistence of persistent communicable diseases, rising NCDs, and mental health conditions places growing pressure on households and health systems, increasing long-term care needs, psychosocial stress, and health-related financial risk. These trends underscore the need to integrate mental health promotion, screening, and care within primary health care, NCD prevention programmes, and maternal and child health platforms in order to sustain life-expectancy gains and protect population wellbeing.

Marked regional disparities further complicate Uganda's mortality and morbidity landscape. While the Teso sub-region records relatively low under-five mortality (about 31 deaths per 1,000 live births), West Nile experiences the highest levels (approximately 80 deaths per 1,000). Regions such as West Nile, Karamoja, Busoga, and Bunyoro, which bear higher mortality burdens, are also characterised by elevated poverty levels, weaker health systems, and limited access to mental health services. These structural conditions heighten exposure to psychosocial stress, family instability, trauma, and adolescent vulnerability. The overlap between survival disadvantages and mental health vulnerability illustrates a reinforcing cycle: regions with poorer health outcomes often face greater mental health burdens, while untreated mental illness can undermine treatment adherence, caregiving capacity, and long-term life outcomes.

Overall, the interplay between declining mortality, shifting morbidity patterns, and mental health vulnerability demonstrates that improving population wellbeing cannot rely solely on biomedical survival interventions. As Uganda transitions towards a health profile increasingly shaped by chronic disease and psychosocial stressors, integrating mental health promotion, screening, counselling, and community-based psychosocial support into primary health care, maternal and child health platforms, and school systems becomes essential. Strengthening mental health services is therefore a critical component of sustaining gains in life expectancy, reducing regional inequities, and safeguarding human capital development.

Text box 1: Why Mortality and Morbidity Trends Matter for Mental Health in Uganda

Mortality and Morbidity Trends Matter for Mental Health

- Uganda has made major gains in reducing maternal, infant, and under-five mortality; however, neonatal deaths remain stubbornly high, indicating persistent gaps in perinatal and early newborn care.
- Declining mortality interacts with fertility choices: where child survival is uncertain, families tend to have more children, which can increase household stress and reduce psychosocial resources per child.
- The disease burden is shifting. Non-communicable diseases and chronic conditions including mental, neurological, and substance-use disorders now represent a growing share of illness and disability.
- Mental health disorders are widespread: approximately one in four Ugandans (adults and children) experience common mental disorders such as depression and anxiety, yet most remain undiagnosed and untreated.
- Regional inequalities in mortality are mirrored in mental health vulnerability. Regions with higher infant and child mortality (e.g., West Nile, Karamoja) also tend to have fewer mental health services, higher poverty, and greater psychosocial stress.
- Sustaining life expectancy gains and improving population wellbeing requires integrating mental health services across primary health care, maternal and child health, community health workers, and schools.
- Addressing mental health is therefore not an optional addition to health programming but a central requirement for human capital development and equitable health outcomes.

2.3.2 Fertility Trends and Reproductive Health Context

Uganda continues to experience relatively high fertility levels compared to regional and global averages, despite a gradual decline over recent decades. The Total Fertility Rate (TFR) decreased from 7.3 children per woman in 1988 to 5.2 in 2022, reflecting modest progress in reproductive health outcomes. However, fertility remains well above replacement level and continues to be a major driver of rapid population growth, high dependency ratios, and sustained pressure on households and social services. The slow pace of fertility decline is closely

linked to persistently low contraceptive use, early marriage, limited access to comprehensive sexual and reproductive health information and services, and entrenched socio-cultural norms that favour large families.

Adolescent fertility remains a particularly persistent challenge. Teenage pregnancy has stagnated at approximately 24 percent for more than a decade, signaling limited progress in addressing early childbearing. Early sexual debut estimated at about 16.8 years for females combined with inadequate sexuality education, poverty, and gender inequality contributes to high rates of unintended pregnancy among adolescents and young women. Girls who enter marriage or motherhood early often experience reduced autonomy over reproductive decision-making, lower educational attainment, and prolonged exposure to high fertility across their reproductive lifespan. These circumstances heighten vulnerability to psychological distress, anxiety, and depression, particularly where social and economic support systems are weak.

Unmet need for family planning remains substantial, especially among young and unmarried women. Available evidence indicates that unmet need is highest among sexually active unmarried women, many of whom face barriers related to stigma, cost, distance to services, and fear of social repercussions. Unintended and closely spaced pregnancies can intensify maternal stress, contribute to poor mental health outcomes, and increase the risk of adverse maternal and child health outcomes, reinforcing cycles of vulnerability at household level.

National fertility averages mask pronounced regional, socio-economic, and spatial disparities. Fertility is highest in Karamoja (about 6.7 children per woman), followed by Bukedi (6.5), Busoga (5.7), and Bunyoro (5.5), while the Kampala region records the lowest fertility at approximately 3.7 children per woman. Rural fertility (about 5.6) remains significantly higher than urban fertility (about 4.3), reflecting differences in education, service access, and livelihood patterns. Education is a strong differentiating factor: women with no formal education have an average of 5.8 children, compared to 3.6 children among women with post-secondary education. These disparities underscore the role of structural inequality in shaping reproductive behaviour and associated well-being outcomes.

Although national surveys do not routinely capture direct links between fertility patterns and mental health outcomes, emerging Ugandan studies point to plausible and concerning pathways. High fertility is often associated with large household sizes, elevated dependency ratios, and chronic economic stress, which can strain caregiving capacity and household relationships. Evidence from climate-vulnerable and high-fertility settings shows elevated levels of anxiety, depression, and behavioural challenges among adolescents from larger families, suggesting that household crowding, limited parental attention, and economic insecurity may negatively affect psychosocial wellbeing.

From a population and development perspective, high fertility has important implications for mental health across the life course. For women, repeated and closely spaced pregnancies increase physical exhaustion, emotional strain, and vulnerability to common mental disorders. For children and adolescents, growing up in large, resource-constrained households can heighten exposure to stress, neglect, and adverse childhood experiences. At the societal level, sustained high fertility slows the pace of demographic transition, constrains investments in education and health, and delays the conditions necessary to harness the demographic dividend.

In line with NDP IV priorities, strengthening sexual and reproductive health and rights -including access to voluntary family planning, adolescent-friendly services, and gender-responsive interventions - is therefore critical not only for managing population growth but also for improving mental health outcomes. Integrating psychosocial support within reproductive, maternal, newborn, child, and adolescent health services offers an opportunity to address the interlinked challenges of fertility, well-being, and human capital development holistically and sustainably.

2.3.3 Life Expectancy, Disability and Chronic Conditions.

Improvements in survival and life expectancy in Uganda have altered the population health profile, shifting attention from premature mortality to the growing burden of disability and chronic conditions. As noted earlier, life expectancy has increased steadily to about 68.2 years, reflecting sustained progress in reducing deaths from communicable diseases and improvements in maternal and child health. While these gains represent an important development achievement, they have also expanded the number of years lived with illness and disability, particularly in the context of chronic physical and mental health conditions.

Uganda's disease burden is increasingly characterised by conditions that do not necessarily result in immediate death but contribute substantially to long-term disability and reduced quality of life. Disability-adjusted life years (DALYs), which combine years of life lost due to premature mortality with years lived with disability, provide a useful lens for understanding this transition. Evidence indicates that Uganda accounts for approximately 17 percent of the East African Community's DALY burden from neglected tropical diseases and mental health-related conditions, underscoring the scale of the challenge at both national and regional levels (Kirigia & Kubai, 2023).

Mental, neurological, and substance-use disorders constitute a significant and growing share of Uganda's overall disease burden. Available estimates suggest that mental and substance-use disorders account for about 13 percent of total DALYs, reflecting their substantial contribution to morbidity rather than mortality. Within this category, depression alone accounts for approximately 5.2 percent of years lived with disability (YLDs), making it one of the leading causes of non-fatal health loss in the country (Alitubeera et al., 2020). Anxiety disorders and



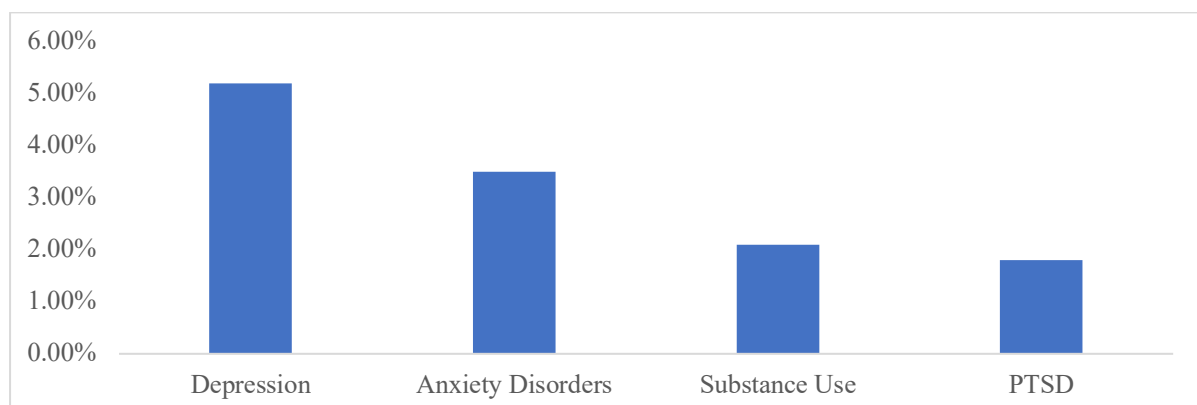
substance use disorders further compound this burden, particularly among young people, women, and socio-economically vulnerable populations.

The increasing prominence of mental health conditions within the DALY profile highlights a critical dimension of Uganda's epidemiological transition. As deaths from infectious diseases decline and life expectancy rises, a larger share of the population lives longer with chronic conditions, including mental health disorders that often begin early in life and persist if untreated. These conditions reduce functional capacity, limit educational attainment and productivity, and place sustained demands on households and caregivers. When combined with rising non-communicable diseases such as hypertension, diabetes, and cardiovascular conditions, mental health disorders contribute to complex patterns of co-morbidity that strain health systems and social support mechanisms.

Disability arising from mental health conditions is often less visible than physical impairment, yet its social and economic consequences are profound. Individuals living with untreated depression, anxiety, or substance-use disorders may experience long-term exclusion from education, employment, and social participation. Figure 7 shows the contribution of the different mental health conditions to the DALYs. At the population level, this translates into lost productivity, increased dependency, and weakened human capital accumulation, with implications for Uganda's ability to achieve its development objectives under NDP IV.

Overall, the relationship between rising life expectancy, disability, and chronic conditions underscores the need to reorient population health responses beyond survival alone. Addressing mental health as a major contributor to disability and DALYs is essential for improving quality of life, reducing long-term care needs, and ensuring that gains in longevity translate into healthier and more productive lives. Integrating mental health prevention, early detection, and long-term care within primary health care and chronic disease management platforms will be critical to sustaining population wellbeing as Uganda continues its demographic and epidemiological transition.

Figure 7: Percentage of Disability-Adjusted Life Years



Source: Alitubeera, et al., (2021)

2.3.4 Non- Communicable Diseases and Comorbidity with Mental Health

Non-communicable diseases (NCDs) have become an increasingly significant component of Uganda's disease burden, reflecting demographic change, rapid urbanisation, and shifting lifestyle patterns. Historically, Uganda's health system prioritised the control of communicable diseases such as HIV/AIDS, malaria, and tuberculosis. While these conditions remain important, recent epidemiological evidence indicates a steady rise in NCDs, including cardiovascular diseases, cancers, diabetes, chronic respiratory conditions, and mental, neurological, and substance-use disorders. NCDs are estimated to account for approximately 36 percent of total deaths in Uganda, and their relative contribution is projected to increase further as communicable disease mortality continues to decline.

Uganda therefore faces a dual disease burden, where persistent infectious diseases coexist with a growing prevalence of chronic NCDs, including mental health disorders. These conditions are closely interlinked and often co-occur. Chronic physical illnesses such as hypertension, diabetes, and cancer are associated with persistent pain, fatigue, functional limitations, and uncertainty about disease progression, all of which can erode psychological wellbeing. The long-term nature of NCD management, often requiring lifelong medication, regular monitoring, and repeated health-facility visits, creates sustained financial pressure on households, particularly where out-of-pocket expenditures remain high and health insurance coverage is limited. These pressures frequently contribute to anxiety, depression, emotional distress, and social withdrawal.

Mental health conditions, in turn, exacerbate the burden of NCDs. Untreated depression, anxiety, and substance-use disorders can reduce treatment adherence, impair self-care behaviours, and negatively affect health-seeking practices, leading to poorer disease control and increased risk of complications. Stigma associated with both mental illness and certain chronic conditions further compounds isolation and delays care. This bidirectional relationship creates a reinforcing cycle in which physical illness and mental illness interact to worsen health outcomes, reduce quality of life, and increase disability.

System-level constraints intensify the impact of NCD-mental health comorbidity. Limited diagnostic capacity, delayed care-seeking, weak referral pathways, and shortages of trained health workers, particularly at primary health care and community levels, constrain effective management of both physical and mental health conditions. These challenges are more pronounced in rural and underserved regions, where access to specialised services is limited and continuity of care is difficult to maintain.

Addressing the rising burden of NCDs in Uganda therefore requires an integrated, person-centred approach that recognises the interdependence of physical and mental health. Holistic NCD management encompassing early detection, lifestyle modification, psychosocial support, and affordable access to long-term treatment, can improve quality of life, enhance adherence to care, and reduce the social and economic impact of chronic illness on households.



Integrating mental health promotion, screening, and basic care within primary health care, chronic disease clinics, and community-based programmes is essential for effective NCD control and for sustaining population wellbeing as Uganda advances through its epidemiological transition.

2.3.5 Sexual Reproductive Health and Mental Health

Mental health and sexual reproductive health (SRH) are integral components of an individual's overall well-being. The different phases of life, from adolescence through adulthood to menopause, are marked by changes in sexual and reproductive behavior that significantly shape mental health status. Likewise, mental health influences SRH outcomes, making the relationship between the two deeply interdependent. This intersection is particularly critical for adolescents, women, and survivors of gender-based violence, who often face compounded vulnerabilities. In Uganda, the linkage between SRH and mental health is increasingly acknowledged in national policy dialogues. However, integration of services remains limited, and many individuals continue to navigate these challenges without adequate support.

Throughout the life course, SRH experiences such as puberty, menstruation, pregnancy, childbirth, infertility, and menopause can trigger or exacerbate mental health conditions. Poor mental health, in turn, can reduce health-seeking behavior, increase susceptibility to risky sexual practices, and undermine adherence to treatment. Addressing these interconnections holistically is essential for advancing people-centered health systems and ensuring that no one is left behind. Women's mental health during their reproductive years is shaped by a range of sexual and reproductive health (SRH) experiences, many of which carry significant emotional and psychological consequences.

Unintended pregnancies and unsafe abortions are among the most emotionally taxing SRH experiences. Women who face unplanned pregnancies may experience fear, shame, and uncertainty, particularly in contexts where reproductive autonomy is limited. Unsafe abortions, often pursued due to lack of access to safe services, can result in physical complications and psychological trauma. Feelings of guilt, isolation, and societal judgment can increase the risk of depression and suicidal ideation, especially when post-abortion care and counseling are unavailable.

Perinatal depression-which includes depression during pregnancy and after childbirth-is one of the most common and severe complications of maternal health. It affects not only the mother's emotional state but also her ability to bond with the newborn, provide care, and engage in daily activities. Perinatal depression is often underdiagnosed due to limited screening and stigma around mental illness. Left untreated, it can impair child development and strain family relationships.

Miscarriage and infant deaths remain a deeply personal and often traumatic experience. The emotional impact includes grief, sadness, and a sense of loss, which can persist long after the physical recovery. In many cases, miscarriage and infant deaths are under-discussed and poorly supported, leaving women to cope in silence. The absence of culturally sensitive counseling and acknowledgment of the emotional toll contributes to long-term depression and anxiety.

HIV and sexually transmitted infections (STIs) present a dual burden of physical illness and mental distress. Women living with HIV or STIs often face stigma, fear of disclosure, and social rejection, which can lead to chronic anxiety and depression. These mental health challenges may reduce treatment adherence, increase feelings of isolation, and undermine overall quality of life. For young women and adolescents, the psychological impact is especially severe, as they navigate identity, relationships, and societal expectations under the weight of a stigmatized diagnosis.

Infertility and Menopause: Infertility is a major source of psychological distress, especially in cultures where childbearing is central to social identity. Feelings of inadequacy, guilt, and fear of judgment can lead to chronic anxiety and depression. In many settings, infertility is stigmatized, and both men and women may internalize blame or face social exclusion.

During menopause, hormonal changes and societal perceptions of aging contribute to emotional instability. Women may experience mood swings, irritability, and depressive symptoms, which are often dismissed or misunderstood. Addressing the intersection of sexual and reproductive health (SRH) and mental health requires multisectoral interventions in a coordinated response. The Ministry of Health (MoH), the Ministry of Gender, Labour and Social Development (MoGLSD), the Ministry of Education and Sports (MoES), and Local Governments each play a critical role in shaping the environments where SRH and mental health outcomes are determined.

Addressing the intersection of SRH and mental health requires coordinated, multisectoral interventions. The Ministry of Health (MoH), the Ministry of Gender, Labour and Social Development (MoGLSD), the Ministry of Education and Sports (MoES), and Local Governments each have critical roles in creating supportive environments, strengthening service delivery, and promoting mental health literacy alongside SRH care. Holistic programming must integrate psychosocial support, counseling, and community engagement to ensure that individuals can access both reproductive health services and mental health care seamlessly.

2.4 Overview of Mental Health in Uganda

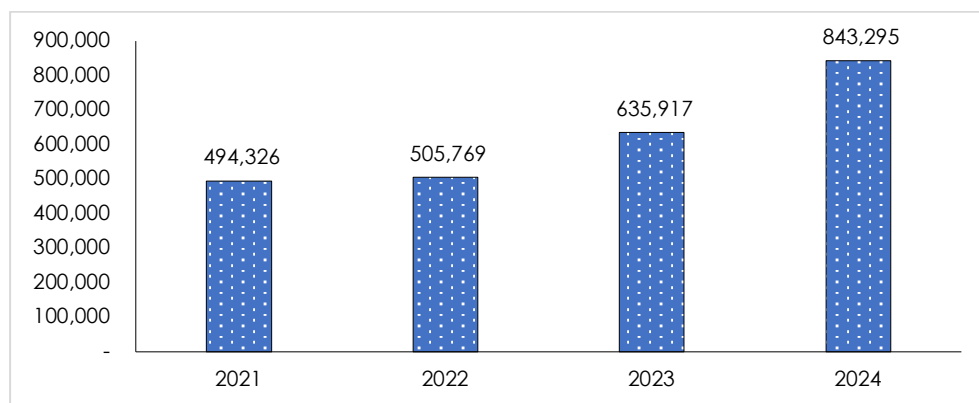
2.4.1 Current status of Mental Health in Uganda

Mental health is increasingly recognised as a critical public health concern in Uganda, yet the scale of need continues to outpace the availability of services and system capacity. Health Management Information System (HMIS) data from the Ministry of Health capture only mental health cases that present at health facilities, and therefore reflect facility-reported morbidity rather than the full population burden. As shown in Figure 8, reported cases rose steadily from 494,326 in 2021 to 505,769 in 2022, increased further to 635,917 in 2023, and reached 843,295 in 2024, representing an overall increase of approximately 71 percent over four years. While this upward trend partly signals improved case detection, reporting, and health-seeking behaviour within the formal health system, it also suggests a growing demand for mental health services. Importantly, these figures likely underestimate the true burden of mental illness, as many individuals do not seek care in health facilities (medical) and instead rely on alternative pathways, including spiritual (faith-based institutions), cultural (traditional and herbal

practitioners), community support mechanisms, or receive no care at all due to stigma, cost, or limited access.

The rising number of facility-reported cases must therefore be interpreted alongside persistent structural and socio-economic stressors that shape mental health vulnerability and care-seeking behaviour. Mental health services in Uganda remain heavily concentrated in urban areas and higher-level referral facilities, leaving rural and underserved populations with minimal access to timely and appropriate care. At the same time, widespread poverty, unemployment, gender-based violence, and prolonged exposure to social and economic shocks, including the lingering effects of the COVID-19 pandemic, continue to heighten vulnerability to anxiety, depression, and substance-use disorders. Young people and women are particularly affected, reflecting their disproportionate exposure to labour-market exclusion, caregiving burdens, and violence.

Figure 8: Mental health cases in Uganda (2021-2024)



Source: HMIS, MoH 2025

The trend shown in Figure 8, therefore, underscores both rising demand within the formal health system and substantial unmet mental health needs beyond it, highlighting the urgency of expanding community-based services, strengthening referral and reporting systems, engaging non-formal providers, and integrating mental health care within primary health and social service platforms.

2.4.2 Regional Mental Health Disparities within Uganda

The burden of mental health conditions is unevenly distributed across Uganda's regions. Central Uganda, which includes Kampala, accounts for the largest share of mental health admissions due to both higher prevalence and concentration of referral facilities. Northern and West Nile sub-regions report disproportionately high levels of post-conflict trauma and substance abuse, while Karamoja faces increasing cases of alcohol-related disorders but very limited service coverage.

Table 1 and Figure 9 show regional variation in mental health cases across Uganda's sub-regions between 2021 and 2024, with all sub-regions recording sharp increases except Bukedi, which experienced a decline. South Buganda recorded an extraordinary rise in mental cases (495.9%), increasing sharply from 32,216 in 2021 to 191,961 in 2024. This suggests a significant surge in reported mental health cases, possibly due to data expansion, improved reporting systems, or major program scale-up in the region. Karamoja (270%) and Busoga (124.0%) also saw a drastic increase in mental health cases from 2021 to 2024. Kampala (53.1%), Kigezi (45.7%), and North Buganda (47.3%) similarly indicate increasing trends of mental health cases.

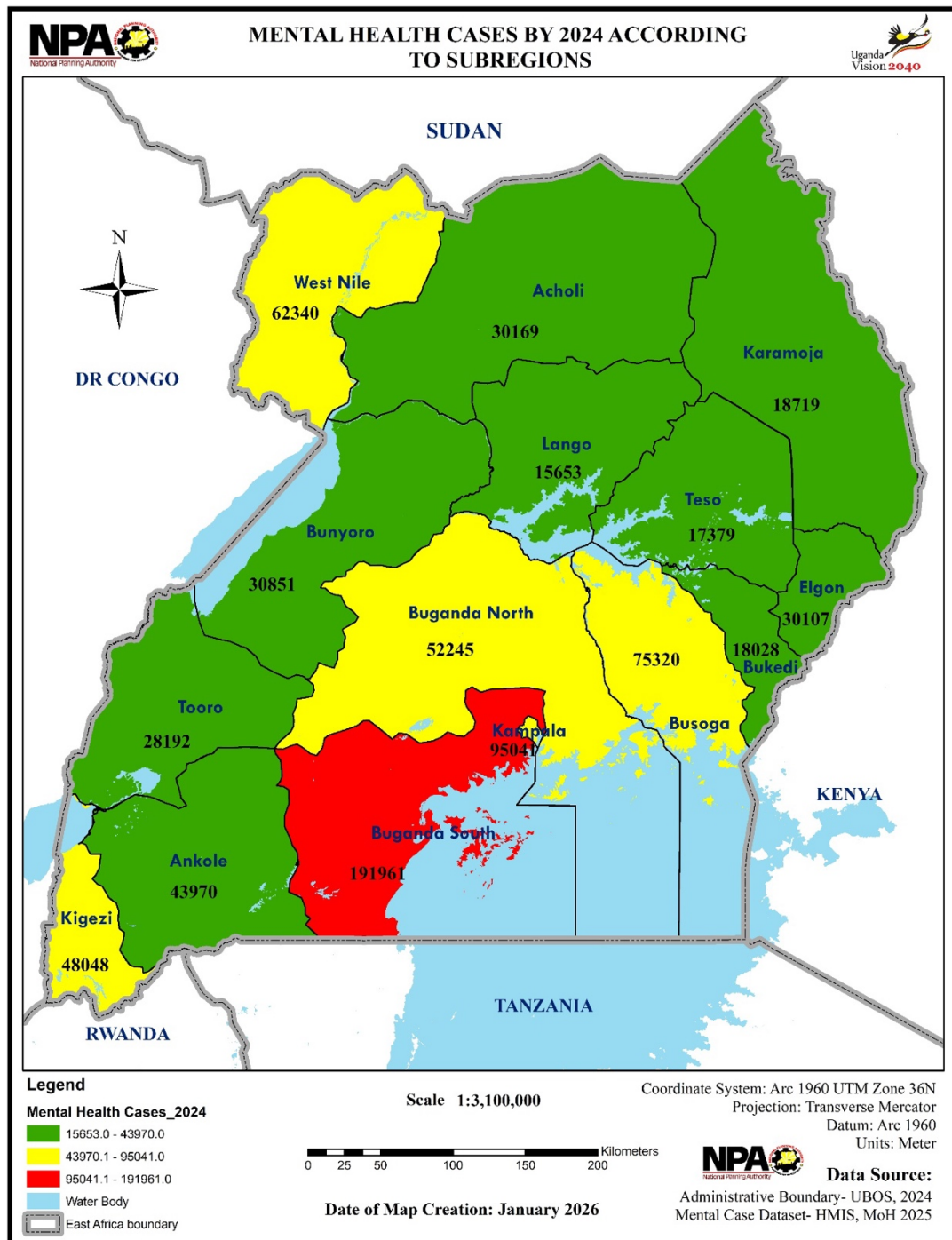
Table 1: Mental health cases according to sub-regions

Sub regions	YEAR				% Change (2021-2024)
	2021	2022	2023	2024	
Acholi	28,290	27,148	25,676	30,169	6.6
Ankole	32,884	30,826	35,818	43,970	33.7
Busoga	33,632	32,091	56,959	75,320	124.0
Bunyoro	27,529	26,977	28,533	30,851	12.1
Bugisu	19,892	20,708	24,835	30,107	51.4
Bukedi	20,647	19,078	26,450	18,028	-12.7
Kampala	62,085	74,949	87,571	95,041	53.1
Karamoja	5,059	4,915	12,411	18,719	270
Kigezi	32,970	35,147	37,170	48,048	45.7
Lango	12,281	11,505	12,030	15,653	27.5
North Buganda	35,459	35,077	54,451	52,245	47.3
South Buganda	32,216	34,802	44,010	191,961	495.9
Teso	12,602	12,683	14,263	17,379	37.9
Tooro	24,803	36,211	28,686	28,192	13.7
West Nile	46,177	39,601	63,774	62,340	35.0

Source: HMIS, MoH 2025



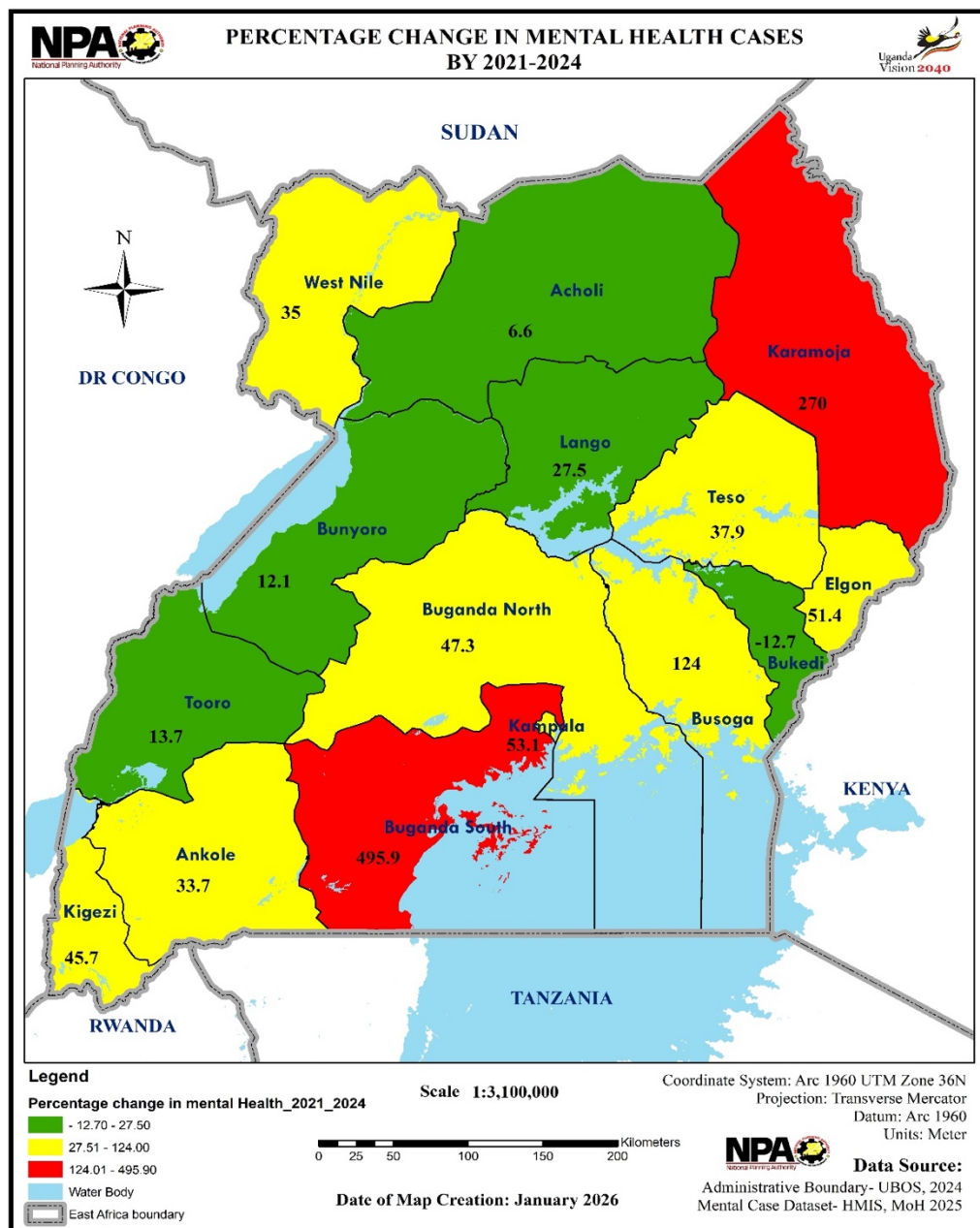
Figure 9: Mental Health Cases by 2024 according to Subregions



Beyond differences in absolute case numbers (Figure 9), the magnitude and pace of change across sub-regions (Figure 10) point to important structural and system-level dynamics. Large percentage increases in regions such as South Buganda, Karamoja, and Busoga are likely to reflect a combination of factors, including rapid population growth, rising psychosocial stress linked to poverty and unemployment, and improvements in case detection and reporting within the formal health system. In contrast, more modest increases in regions such as Kampala and Kigezi, which already had relatively higher baseline caseloads and better service availability, may reflect more stable reporting systems rather than lower mental health need.

The observed decline in Bukedi underscores the likelihood of under-detection, reporting gaps, or shifts in care-seeking behaviour rather than an actual reduction in mental health conditions. Overall, the regional patterns highlight persistent inequities in access to mental health services and surveillance capacity, underscoring the need for region-specific strategies that strengthen community-based services, expand reporting beyond referral facilities, and address underlying socio-economic and post-conflict stressors driving mental illness.

Figure 10: Percentage change in mental health cases by 2021-2024



2.4.3 Key Mental Health Conditions in Uganda

Uganda experienced a general rise in reported mental health cases between 2021 and 2024, although the degree of change differed noticeably among specific disorders. There were sharp increases in unipolar depressive disorder, which recorded the most dramatic rise (450%) from

20,005 in 2021 to 110,353 in 2024 (Table 2). This suggests growing recognition and reporting of depression, possibly linked to post-pandemic stress, economic hardship, or improved diagnosis. Anxiety disorder due to Gender-Based Violence (GBV) also surged by 145.3%, reflecting heightened awareness and reporting of GBV-related trauma and its mental health impacts, while bipolar disorder, other adult mental disorders, and HIV-related psychosis showed moderate-to-high increases (ranging from 47% to 58%). Anxiety disorders rose by 23.3% while schizophrenia and dementia increased by 35.2% and 29.6%, respectively. Epilepsy, though the most reported condition, rose modestly by 7.8% indicating consistent management and stable prevalence. Post-Traumatic Stress Disorder (PTSD) declined by 18%, possibly due to improved trauma response services or reporting shifts to other diagnostic categories, whereas alcohol-related Dementia decreased by 10.2%, which may suggest better management of alcohol misuse or underreporting.

Table 2: Mental health conditions

Condition	2021	2022	2023	2024	% Change
Anxiety Disorders	43,527	40,113	50,694	53,662	23.3
Anxiety Disorder due to GBV	7,514	8,669	11,434	18,429	145.3
Unipolar Depressive Disorder	20,005	21,363	87,588	110,353	451.6
Bipolar disorder	46,865	54,273	61,792	70,691	50.8
Post-traumatic stress disorder	14,473	12,733	13,602	11,864	-18.0
Schizophrenia	27,991	31,621	36,069	37,839	35.2
Dementia	3,682	4,058	4,216	4,771	29.6
Other Adult Mental Disorders	19,034	23,740	29,887	30,018	57.7
Internet addiction	1,313	1,874	1,749	1,633	24.4
Substance (Drug) Use Disorder	8,946	11,303	19,530	16,245	81.6
Alcohol Use Disorders	13,362	14,647	17,657	16,241	21.5
HIV related psychosis	9,718	10,133	10,581	14,291	47.1
Epilepsy	268,837	261,514	279,849	289,774	7.8
Attempted Suicide/Self-harm	938	960	1,167	1,186	26.4
Child Abuse and Neglect	24	29	20	31	29.2
Alcohol related Dementia	1,007	921	878	904	-10.2

Source: HMIS, MoH 2025

Data reveal escalating high-risk mental health trends in Uganda that require urgent, multisectoral policy responses and targeted resource allocation. Substance (drug) use disorders increased by 81.6% highlighting the growing challenge of substance abuse across the population, particularly among youth and urban communities. Similarly, cases of attempted suicide and self-harm rose by 26.4% signaling worsening mental distress and emphasizing the urgent need for strengthened suicide prevention and psychosocial support interventions. Although child abuse and neglect cases remain relatively few, they increased by 29.2% underscoring the rising vulnerability of children to adverse social and mental health outcomes. Together, these trends point to deepening psychosocial pressures within households and communities and the need for integrated, multi-sectoral responses to address these emerging risks.

While the condition-specific trends presented in Table 2 point to a growing mental health burden, they should be interpreted within the context of facility-based reporting and evolving diagnostic practices. Sharp increases in conditions such as unipolar depressive disorder and GBV-related anxiety may partly reflect improved screening, greater clinician awareness, and reclassification of cases previously captured under broader diagnostic categories such as general anxiety or psychosocial distress. Conversely, declines in conditions such as post-traumatic stress disorder and alcohol-related dementia may signal diagnostic substitution, shifts in service utilisation, or under-detection rather than true reductions in incidence. The pattern of rising substance-use disorders, suicide and self-harm, and GBV-related conditions also highlights gendered and life-course vulnerabilities, with young people, women, and urban populations likely bearing a disproportionate share of emerging mental health risks. Together, these trends underscore the need to strengthen mental health surveillance, standardise diagnostic practices, and expand community-based detection beyond health facilities to ensure a more accurate and equitable understanding of Uganda's mental health profile.

2.5 Migration, Urbanization, Mobility, and Mental Health in Uganda

Migration and rapid urbanization in Uganda are increasingly linked to adverse mental health outcomes. Both internal and international migration expose individuals to stressors such as trauma, family separation, exploitation, precarious livelihoods, and social isolation. Among returning migrant domestic workers from the Middle East, 133 cases of mental illness were documented between 2019 and 2023, with contributing factors including abuse and overwork. Refugee populations experience even higher mental health burdens: a survey in Nakivale Refugee Settlement reported generalized anxiety in 73%, PTSD in 67%, and major depressive disorder in 58% of participants. Urban refugee youth in Kampala further experience depression linked to food insecurity, unemployment, and social marginalization.

Uganda's urban population has expanded from 6.6% in 1990 to over 25% in 2024, with projections exceeding 30% by 2030. Rapid urbanization has fueled slum growth, with approximately 60% of urban dwellers residing in informal settlements. Poor housing, overcrowding, inadequate sanitation, exposure to crime, and weak social cohesion create environments that exacerbate psychological distress. Young women in Kampala's slums report sadness, low self-esteem, and anxiety associated with these conditions. Urban growth without proportional investment in housing, employment, and accessible mental health services threatens to escalate substance misuse, interpersonal violence, and social disengagement.

Structural factors such as unemployment, housing instability, and social marginalization are now key drivers of mental illness among migrants and urban populations. Evidence from the UPLIFT urban livelihoods project shows that improving socio-economic conditions can enhance mental well-being, self-esteem, and life purpose, underscoring the potential of integrated interventions to mitigate mental health risks. Failure to address these determinants risks widening mental health inequities, undermining human capital, and constraining economic and social development.

If urban growth continues without parallel investment in decent housing, employment opportunities, and accessible mental health care, the burden of psychological distress will rise sharply. The effects will include increased substance misuse, interpersonal violence, and social disengagement, straining an already overburdened healthcare system. However, evidence from the UPLIFT urban livelihoods project in Kampala demonstrates that improving socio-economic conditions can enhance psychological well-being, self-esteem, and life purpose. Conversely, ongoing qualitative research and local reporting highlight how persistent unemployment, poverty, and overcrowding in informal settlements drive escalating mental health concerns, particularly among youth and refugees.

2.6 Expenditure on mental health in Uganda

Uganda allocated approximately 6.1% of its national budget, equivalent to Shs 5.7 trillion, to healthcare annually (Health Alternative Policy Statement, FY 2025/2026). Despite this overall investment, mental health consistently receives disproportionately low funding, accounting for less than 1% of the total health budget. This allocation falls far short of the World Health Organization's (WHO) recommended minimum of 5% for mental health expenditure. Consequently, 75% of Ugandans with mental health disorders do not receive any form of treatment (Health Alternative Policy Statement, FY 2025/2026).

The bulk of available mental health funding is concentrated at Butabika National Mental Health Referral Hospital, leaving peripheral, community-level, and primary care mental health services severely under-resourced. This centralization of resources restricts access for rural populations, adolescents, refugees, and other vulnerable groups, perpetuating inequities in mental health care. Limited financial investment also constrains the recruitment and retention of specialized mental health personnel, the availability of psychotropic medications, and the development of community-based psychosocial support programs.

The underfunding of mental health services has broader socio-economic consequences. Untreated mental illness contributes to decreased productivity, increased caregiving burdens, heightened poverty, and exacerbated social exclusion. Without increased and strategically allocated funding, the country risks failing to address the growing mental health burden, which is amplified by population growth, urbanization, migration, and the rising prevalence of non-communicable diseases.

2.7 Policy and Legal Framework for Mental Health

2.7.1 Introduction

Mental health in Uganda is increasingly recognized to reflect a critical intersection of public health, human rights and socio-economic development. The findings of SUPRE 2025 demonstrate that mental illness affects a significant proportion of the population, imposes substantial economic costs, and undermines human capital development. Addressing these challenges requires more than policy intent and programmatic interventions; it requires a clear, enforceable, and adequately resourced legal and regulatory framework that translates rights into services, obligations into action, and commitments into measurable outcomes.

Uganda has made notable progress in aligning its mental health laws and policies with international and regional human rights standards. However, the evidence presented in this report indicates that implementation gaps, regulatory weaknesses, and institutional fragmentation continue to limit the impact of these legal advances. This section examines the existing legal and policy framework, identifies key regulatory and enforcement gaps and outlines priority legal actions required to strengthen mental health governance and delivery in Uganda.

2.7.2 International and Regional Legal Commitments

Uganda's mental health legal framework is underpinned by a strong set of international and regional human rights instruments that recognize mental health as an integral component of the right to health, human dignity, and non-discrimination. International frameworks such as the Universal Declaration of Human Rights (UDHR), the International Covenant on Economic, Social and Cultural Rights (ICESCR), and the Convention on the Rights of Persons with Disabilities (CRPD) establish binding obligations on the State to ensure the availability, accessibility, acceptability, and quality of mental health services; protect autonomy and informed consent; and prevent discrimination against persons with psychosocial disabilities.

At the regional level, the African Charter on Human and Peoples' Rights, the African Charter on the Rights and Welfare of the Child, the Maputo Protocol, and the African Youth Charter further reinforce these obligations, with particular emphasis on protecting women, children, youth, and other vulnerable groups from violence, exploitation, neglect, and harmful practices that directly affect mental wellbeing.

Collectively, these instruments provide a robust normative foundation for a rights-based, community-oriented, and life-course approach to mental health. They clearly shift mental health away from a purely custodial or biomedical model toward one centered on dignity, participation, and social inclusion. However, as SUPRE 2025 demonstrates, ratification has not consistently translated into enforceable domestic systems, particularly at community and local government levels where most mental health needs arise.

2.7.3 National Legal Framework: Progress and Persistent Gaps **Evolution of Mental Health Legislation**

At the national level, Uganda has undertaken significant legal reform in recent years. The Mental Treatment Act of 1938, even after its 1964 revision, reflected a colonial-era institutional model that prioritized detention, control, and confinement over treatment, rehabilitation, and rights protection. This approach was increasingly incompatible with contemporary human rights standards and Uganda's decentralized health system.

The enactment of the Mental Health (Amendment) Act, CAP 308 represents a major legal milestone. The Act aligns national law more closely with international human rights obligations by emphasizing preventive and community-based care, safeguarding informed consent, regulating restraint and seclusion, mandating integration of mental health into primary health care, and establishing institutional structures such as the Uganda Mental Health Advisory Board.

These reforms provide a sound legal basis for the shift toward decentralized, rights-based mental health services highlighted in SUPRE 2025.

Implementation and Regulatory Deficits

Despite this strengthened legal foundation, implementation remains the critical constraint. Many provisions of the Mental Health (Amendment) Act, CAP 308 have not been fully operationalized through regulations, guidelines, or enforceable standards. In practice, mental health services remain heavily concentrated at referral-level facilities, lower-level health centers lack functional mental health units, community-based services are underdeveloped, and local governments face unclear mandates and limited fiscal space.

The result is a gap between legal entitlement and lived reality, where rights exist in statute but are not consistently realized in service delivery. This disconnect undermines the credibility of the legal framework and weakens accountability across sectors and levels of government.

Policy Framework and Its Regulatory Limitations

Uganda's policy landscape reflects long-standing recognition of mental health as a public health priority. The National Mental Health Policy (2000, revised 2014) and the National Mental Health Policy Framework and Strategic Plan (2013–2020) articulated clear objectives, including integration of mental health into general health services, strengthening district-level capacity, promotion of community-based care, and reduction of stigma.

While these policies contributed to increased awareness and some service integration, implementation was uneven and often donor-dependent. Several priority actions including nationwide suicide prevention, sustained community outreach, and comprehensive workforce development were only partially realized. The expiration of the 2013–2020 Strategic Plan without a fully resourced successor has further weakened policy coherence and continuity.

Critically, many policy instruments remain programmatic rather than regulatory. They lack binding force, clear enforcement mechanisms, and systematic linkage to budgeting and performance frameworks, limiting their ability to drive sustained change.

Sectoral and Population-Specific Legal Gaps

SUPRE 2025 highlights growing mental health challenges among children, adolescents, youth, and economically vulnerable populations. While sector-specific guidelines represent important progress, there are still glaring challenges such as fragmentation that undermine multisectoral response required to address the socioeconomic drivers of mental illness identified in the report.

Mental Health in Education-related frameworks:

The Teachers' Handbook on Mental Health and Psychosocial Support (MHPSS) (2025), developed by the Ministry of Education and Sports with support from UNESCO and partners, provides a valuable framework for integrating MHPSS into Ugandan schools through guidance on teacher resilience, learner support, facilitation methods, and addressing key issues like anxiety, depression, and special populations. Despite its strengths in standardization and participatory approaches, implementation faces significant challenges that limit its reach and effectiveness. Resource and funding constraints remain a primary barrier, as the Handbook's rollout relies heavily on donor support, resulting in limited distribution and training opportunities, particularly in rural and underserved districts. Training and capacity-building shortfalls further hinder progress, with only a fraction of teachers receiving adequate orientation on the Handbook's methods, such as role play, community dialogues, and basic

counseling, leading to uneven application and teacher burnout due to high workloads and large class sizes without sufficient ongoing supervision or integration into pre-service curricula. Monitoring and evaluation deficiencies exacerbate these issues, as there are no robust, systematic mechanisms to track the Handbook's impact on student mental health outcomes, retention rates, or referral effectiveness, making it difficult to identify and address gaps in scalability. Integration and structural barriers, including poor linkages between schools and health facilities, persistent stigma surrounding mental health discussions, and the absence of enforceable mandates, result in fragmented adoption, where schools often prioritize academic activities over MHPSS initiatives. Finally, sustainability concerns threaten long-term viability, as the early-stage implementation (post-2025 launch) faces threats from vulnerable health facilities and insufficient long-term planning due to lack of dedicated domestic budgeting, potentially leading to reliance on short-term donor-driven efforts rather than institutionalized, nationwide integration. Without binding regulations, schools often prioritize academics over mental health activities.

The Handbook reflects positive implications by empowering schools as entry points for early detection and referral. However, to address the identified gaps in its implementation, the Handbook provisions should be embedded into statutory instruments under the Education Act, mandating MHPSS in curricula and teacher training.

Mental Health in Health-related frameworks: The Third National Health Policy (NHP III) of 2021, titled "Towards Universal Health Coverage," embeds mental health within a life-course approach, recognizing its contribution to the preventable disease burden and calling for integration into primary health care, community systems and the Uganda National Minimum Healthcare Package. However, implementation gaps continue to undermine its ability to address mental health challenges across populations. Severe funding and resource shortages represent a core challenge, with mental health receiving about 1% of the national health budget – far below WHO recommendations, resulting in chronic stock-outs of psychotropic medications, overcrowded facilities like Butabika Hospital Mental National Referral Hospital and limited expansion of community-based services, while donor dependency creates volatility and hinders domestic sustainability. Integration and multisectoral coordination failures persist, as mental health is often sidelined in favor of physical health priorities, with fragmented linkages to sectors like education and social services. Stigma and sociocultural barriers significantly deter help-seeking, particularly among adolescents, older persons and vulnerable groups, where cultural beliefs of distress and reliance on traditional healers prevail over formal care, perpetuating underutilization of available services. Workforce and capacity deficits compound these issues, with only critical health worker positions filled, acute shortages of specialists and inadequate training in mental, neurological, and substance use disorders, leading to burnout and uneven delivery, especially in rural areas. Monitoring and accountability weaknesses further constrain progress, as health information systems lack interoperability, resulting in poor tracking of outcomes and limited evidence for policy adjustments toward UHC targets.

The Child and Adolescent Mental Health Policy Guidelines (2017) represent a dedicated framework to promote mental health and prevent mental, neurological and substance use disorders among children and adolescents. The guidelines outline key objectives including



raising awareness and stakeholder involvement, building capacity to improve access to comprehensive child and adolescent mental health services (CAMHS) such as care, psychological treatment and rehabilitation, strengthening research, monitoring, evaluation and fostering multi-sectoral collaboration for quality service delivery. Intended for a 10-year implementation period across government sectors and civil society, they address underutilization of services due to factors like limited knowledge, parental oversight of early symptoms and inadequate specialized CAMHS availability, while emphasizing promotion in various settings like schools and communities

Despite these progressive aims, implementation of the 2017 guidelines faces substantial challenges, including chronic underfunding, limiting resources for child-specific services amid broader health priorities. Weak multi-sectoral coordination and intra-sectoral mechanisms hinder effective collaboration, while persistent stigma against mental illness and shortages of trained human resources, facilities and data for monitoring and evaluation contribute to fragmented and uneven rollout.

Mental Health in Social sector-related frameworks: The frameworks under the Ministry of Gender, Labour and Social Development, such as the Operational Framework for Social Care and Support System (2025) and the National Comprehensive Action Plan on the Rights of Persons with Disabilities 2020-2025, provide essential standards for integrating mental health into social protection and equity initiatives, targeting vulnerable groups like women, children, and PWDs through community-based and rights-based approaches. Despite their comprehensive design, implementation faces substantial hurdles that undermine their effectiveness in addressing mental health challenges. Resource constraints and inadequate funding allocation pose a major barrier, as MOGLSD relies heavily on donor support for programs like community mental health partnerships, leading to uneven coverage, particularly in rural areas where social care infrastructure is limited. Multisectoral coordination deficiencies further complicate rollout, with fragmented linkages between MOGLSD, MoH, and local governments resulting in duplicated efforts or gaps in service delivery, such as inconsistent psychosocial support for GBV survivors or refugees, exacerbated by bureaucratic silos. Stigma and sociocultural barriers persist, deterring uptake of services, especially in conservative communities where mental health is stigmatized or attributed to supernatural causes, resulting in underutilization of frameworks' provisions for inclusive care. Monitoring and evaluation weaknesses limit accountability, with inadequate data systems and indicators for mental health outcomes in social protection programs, making it difficult to track progress, adjust strategies, or demonstrate impact on vulnerable populations.

These gaps underscore the need for stronger regulatory enforcement and increased domestic investment to transform the frameworks into effective tools for mental health equity.

Constitutional and Oversight Mechanisms

The 1995 Constitution of Uganda provides a strong overarching legal basis for mental health protection through guarantees of human dignity, equality, and access to basic health services. Oversight bodies such as the Uganda Human Rights Commission play a vital role in monitoring abuses and advocating for humane treatment.

However, enforcement remains largely reactive. Many violations, particularly those occurring in communities, informal care settings, workplaces, and schools go undocumented and unaddressed. Preventive oversight, routine inspections, and community-level accountability mechanisms remain weak, limiting the protective potential of constitutional guarantees.

Key Legal and Regulatory Priorities Going Forward

In light of SUPRE 2025 findings, strengthening mental health outcomes in Uganda requires a shift from legal adequacy in principle to effectiveness in practice. Priority actions include:

1. Operationalizing the Mental Health (Amendment) Act, CAP 308 through detailed regulations, standards, and guidelines that clearly define roles, service packages, and accountability mechanisms at national and local levels.
2. Strengthening regulatory mandates for community-based and primary health care mental health services, including task-sharing, supervision, and referral systems.
3. Embedding mental health obligations within sectoral regulatory frameworks, particularly education, labor, social protection, and justice.
4. Establishing clearer legal and budgetary linkages to ensure predictable financing, expenditure tracking, and accountability for mental health services.
5. Enhancing preventive oversight and enforcement mechanisms, including routine inspections and accessible grievance redress systems.
6. Formalizing multisectoral coordination mechanisms to address the social and economic determinants of mental health in a legally binding manner. Government should establish a legally binding multisectoral coordination framework for mental health, through legislation or subsidiary regulation, that clearly defines the roles, obligations, and accountability of health, education, labor, social protection, justice, and local government sectors in addressing the social and economic determinants of mental health.

Conclusion

Uganda's legal and policy framework for mental health is largely sound in design and increasingly aligned with international human rights standards. However, as SUPRE 2025 clearly demonstrates, implementation gaps, regulatory weaknesses, and limited accountability continue to constrain impact. Bridging this gap will require sustained political commitment, stronger regulatory instruments, clearer institutional mandates, and increased domestic financing.

Strengthening the legal and regulatory foundations of mental health is essential not only for protecting individual rights, but also for safeguarding Uganda's human capital, productivity, and long-term development aspirations under NDP IV.

2.8 Overall Emerging Issues from the National Situation Analysis

The situation analysis reveals that mental health challenges in Uganda are not driven by a single factor, but by the intersection of rapid demographic change, socio-economic stressors, evolving



disease patterns, and system-level constraints. The following emerging issues capture the most critical cross-cutting dynamics shaping mental health outcomes and population wellbeing.

1. Rapid Population Growth and a Youthful Age Structure Are Amplifying Mental Health Risks

Uganda's sustained population growth and pronounced youth bulge are intensifying demand for education, employment, housing, and social services at a pace that outstrips system capacity. Children, adolescents, and young adults, who constitute the majority of the population, are exposed to heightened psychosocial stress during critical developmental and transitional stages. High dependency ratios, large household sizes, and prolonged school-to-work transitions are associated with increased household pressure, emotional strain, and vulnerability to anxiety, depression, substance use, and harmful coping behaviours, particularly among young people and women

2. Economic Insecurity, Unemployment, and NEET Status Are Closely Linked to Psychological Distress

The available literature highlights a strong intersection between mental health and labour-market exclusion. High levels of youth unemployment and NEET status, combined with poverty and informal livelihoods, are associated with social isolation, loss of purpose, substance misuse, and rising cases of depression and self-harm. Mental illness, in turn, undermines employability and productivity, reinforcing cycles of exclusion and constraining Uganda's ability to harness its demographic dividend under NDP IV.

3. Urbanisation and Migration Are Reshaping Mental Health Vulnerability

Rapid urbanisation, internal migration, and refugee inflows are creating new mental health risks, particularly in informal settlements and among displaced populations. Overcrowding, insecure housing, unemployment, weak social cohesion, and exposure to violence in urban and peri-urban areas exacerbate psychological distress, especially among youth, women, migrants, and refugees. At the same time, rural areas face persistent barriers to care, including distance, stigma, and limited-service availability, resulting in untreated distress and prolonged disability. These spatial inequalities point to the need for place-sensitive mental health responses embedded within urban planning, housing, and local government systems

4. The Epidemiological Transition Is Increasing the Burden of Chronic Mental Illness and Disability

As mortality declines and life expectancy rises, Uganda is experiencing a shift toward non-communicable diseases and long-term conditions, including mental, neurological, and substance-use disorders. Mental health conditions now account for a substantial share of disability and years lived with illness, often co-occurring with chronic physical diseases. This dual burden places sustained demands on households and health systems, increases care costs,

and reduces quality of life and productivity, particularly where psychosocial support is limited or absent

5. Gender-Based Violence and Reproductive Health Challenges Are reflected as Major Drivers of Mental Illness

The analysis shows a strong intersection between mental health, gender inequality, and sexual and reproductive health outcomes. Rising cases of GBV-related anxiety, perinatal depression, unintended pregnancy, and trauma linked to HIV, infertility, and reproductive loss disproportionately affect women and adolescent girls. These experiences are often compounded by stigma, limited autonomy, and inadequate access to integrated SRH and mental health services, resulting in long-term psychological harm across the life course

6. Facility-Based Data Mask the True Burden of Mental Illness

Reported increases in mental health cases largely reflect facility-based data, which capture only those who seek care within the formal health system. Many individuals continue to rely on faith-based institutions, traditional healers, informal providers, or receive no care at all due to stigma, cost, or distance. This results in significant underestimation of the true population burden and limits the ability to plan effectively. Weak surveillance, inconsistent diagnostics, and limited community-based reporting constrain evidence-informed policy and resource allocation

7. Financing and Service Delivery Gaps Undermine Policy Intent

While Uganda has made notable progress in strengthening its policy and legal framework for mental health, implementation remains constrained by chronic underfunding, workforce shortages, and centralised service delivery. Mental health receives less than one percent of the health budget, with services concentrated at referral facilities. This misalignment between policy commitments and system capacity limits access for rural populations, youth, refugees, and other vulnerable groups, perpetuating inequities in mental health outcomes

8. Mental Health Is a Cross-Cutting Development Issue, Not a Standalone Health Concern

Across the situation analysis, mental health consistently emerges as both a driver and outcome of broader development challenges including poverty, unemployment, education outcomes, gender inequality, migration, and social protection. Treating mental health as a narrow clinical issue risks overlooking its central role in human capital formation, productivity, social cohesion, and long-term development. Addressing mental health effectively therefore requires multisectoral action that integrates health, education, labour, urban development, social protection, and justice systems.

9. Uganda has a largely progressive mental health legal and policy framework, but weak implementation, financing, and enforcement, particularly at community and local government levels, continue to constrain effective service delivery.

CHAPTER 3: SOCIO-ECONOMIC DRIVERS OF MENTAL HEALTH

3.1 Introduction

This Chapter presents the background and overview of mental health as a socio-economic and population development concern. Mental health outcomes are deeply influenced by the broader socioeconomic contexts in which individuals and communities live. While biological and psychological factors play a role, evidence increasingly shows that the conditions of daily life, such as income levels, education, employment opportunities, living environments, and access to social support, are central determinants of mental well-being. These socioeconomic factors shape people's exposure to stressors, their capacity to cope, and their access to mental health services.

In contexts where poverty, inequality, and social exclusion are widespread, the risk of developing mental health conditions becomes significantly amplified. Understanding these drivers is therefore essential in designing policies and interventions that address not only clinical symptoms but also the underlying structural conditions that contribute to mental health disparities. This section examines how socioeconomic status, living and working conditions, community environments, and social support systems interact to influence mental health outcomes, particularly within the Ugandan context, characterized by a youthful population and persistent socioeconomic challenges.

3.2 Background and Overview

The link between socioeconomic conditions and mental health is well-documented, emphasizing that psychological well-being is shaped not only by individual characteristics but also by broader structural and environmental factors. Socioeconomic status, typically measured through income, education, occupation, and social standing, greatly influences access to essential resources such as healthcare, quality nutrition, safe housing, and opportunities for personal development (WHO, 2022). When these foundational needs are unmet or unstable, individuals and households experience chronic stress that can heighten vulnerability to mental health disorders. Research shows that financial insecurity, low educational attainment, and unemployment significantly increase the risk of depression, anxiety, and other stress-related conditions (Lund et al., 2018).

Globally, the burden of mental disorders is both widespread and unevenly distributed, with low- and middle-income countries bearing the heaviest impact. This disparity is closely tied to structural socioeconomic inequalities that limit access to preventive and treatment services (Patel et al., 2018). Individuals facing socioeconomic disadvantage are more likely to encounter adverse living conditions, social exclusion, and reduced community support all of which compound psychological distress. Mental illness can also reinforce economic vulnerability by impairing productivity, reducing employability, and increasing healthcare costs, thereby creating a cyclical relationship between poverty and mental ill-health (Allen et al., 2014).

In Uganda, these global patterns are intensified by a combination of demographic and economic dynamics. The country's predominantly youthful population, high levels of unemployment, persistent poverty, and limited mental health service coverage contribute to a growing prevalence of psychological distress, especially among young people (Kaggwa et al., 2022). Despite increased policy attention, mental health services remain underfunded and unevenly distributed, with rural and low-income households experiencing the greatest barriers to access. As such, social and economic conditions not merely individual factors play a decisive role in shaping mental health outcomes in Uganda. Understanding these socioeconomic drivers is therefore essential for developing responsive, equitable, and sustainable mental health interventions.

These dynamics carry significant implications in Uganda due to its exceptionally youthful population, with more than 78% of citizens under the age of 30. While this youth bulge presents an opportunity for accelerated development, it also poses risks if mental health needs are not addressed. Despite high prevalence rates of conditions such as depression, anxiety, PTSD, and substance abuse, over 90% of affected individuals do not receive treatment. The intersection of poverty, limited mental health services, and social pressures heightens vulnerability among young people. Uganda's potential demographic dividend therefore depends on ensuring youth receive adequate mental health support; without this, the large young population may become a source of social strain rather than economic productivity.

The rationale of this chapter is therefore to examine the socio-economic determinants of mental health in Uganda. Despite the high prevalence of mental disorders, mental health resources remain scarce and largely clinically focused, failing to address root socio-economic drivers such as poverty, food insecurity, unemployment, and gender inequality. For a country grappling with the legacies of conflict, HIV/AIDS, and climate shocks, focusing solely on biomedical approaches risks being ineffective. A comprehensive understanding of the social and economic underpinnings of mental health is therefore essential to design culturally appropriate, cost-effective, and sustainable interventions that can close the treatment gap and build national resilience. Identifies the relationship between socio-economic factors (poverty, unemployment, education, food insecurity, and asset wealth) and common mental disorders (depression, anxiety, PTSD) among Ugandan adults. Specifically, this chapter aims to explore the mediating and moderating roles of psychosocial factors (social support, stigma) in the link between socio-economic disadvantage and mental health and also examine how gender and geographic location (urban vs. rural, including post-conflict regions) shape the experience of socio-economic drivers of mental health, identifying unique vulnerabilities and resilience factors.



3.3 Methodology

3.3.1 Study Design

This chapter employs both quantitative and qualitative designs. The quantitative component involves the application of a Correlation Analysis and a Linear Regression Model on secondary data. The selected methodology ensured rigor, leveraging both empirical findings and a comprehensive review of existing evidence from scholarly and institutional sources. On the other hand, a systematic review was conducted to synthesize empirical evidence on the drivers of mental health in Uganda, with a specific focus on biological, psychological, socio-economic, and structural factors. The review aimed to consolidate findings across diverse populations and study designs to inform policy, programming, and future research.

3.3.2 Data Sources

Quantitative data was extracted from World Development Indicators, Global Burden of Disease, peer-reviewed journals, WHO and Ministry of Health reports, UN agencies. This includes data on variables such as GDP per-capita, DALYS, Average Years of Schooling, Urbanisation rate, and Unemployment. For qualitative data, a comprehensive search was undertaken across peer-reviewed journals, institutional repositories, and grey literature databases. Studies were identified through targeted searches in PubMed, Scopus, Web of Science, and African Journals Online (AJOL), using combinations of keywords such as “mental health,” “depression,” “anxiety,” “Uganda,” “determinants,” “risk factors,” “adolescents,” “refugees,” “HIV,” “IPV,” and “psychosocial.” Reference lists of included articles were manually screened to identify additional relevant studies. The search was restricted to studies conducted in Uganda or with Ugandan populations, published between 2010 and 2025.

3.3.3 Inclusion Criteria

Studies were included in the synthesis if they: i). Were conducted in Uganda or among Ugandan populations; ii). Examined mental health outcomes (e.g., depression, anxiety, PTSD, suicidality, substance use, behavioural disorders); iii). Reported empirical findings on determinants, correlates, or drivers of mental health; iv). Used recognized quantitative, qualitative, or mixed-methods designs; v). Were published in peer-reviewed journals or credible academic theses and reports. Both observational and intervention studies were included, which provided insight into underlying drivers or protective factors.

3.3.4 Exclusion Criteria

Studies were excluded if they: i). Focused exclusively on pharmacological or clinical efficacy without analysis of determinants; ii). Were purely theoretical or opinion-based without empirical data; iii). Were case reports with very limited generalisability; iv). Did not clearly report mental health outcomes or associated drivers.

3.3.5 Theoretical Framework

The theoretical framework guiding this study draws on four complementary perspectives to explain how socioeconomic conditions shape mental health outcomes in Uganda. The Social Determinants of Health Theory emphasizes that mental health is deeply rooted in structural and environmental factors such as poverty, education, housing, and access to care. These conditions influence exposure to stress and opportunities for healthy development, helping to explain why disadvantaged populations experience higher levels of psychological distress. The Psychological Stress Theory further clarifies how prolonged exposure to socioeconomic hardship creates physiological and emotional strain, increasing vulnerability to anxiety, depression, and substance abuse. Together, these theories demonstrate how social and economic inequalities become embodied as mental health challenges.

The Human Capital Theory and Life Course Perspective provide additional insight into the long-term developmental and economic implications of mental health. Human Capital Theory highlights mental well-being as a critical resource that enables individuals, particularly youth, to learn, work, and innovate therefore influencing national productivity and development. Meanwhile, the Life Course Perspective shows how early-life adversity and cumulative stress shape psychological outcomes over time, reinforcing intergenerational cycles of disadvantage. Collectively, these frameworks underscore the need for multi-level interventions that reduce socioeconomic stressors, expand access to education and mental health services, and support resilience from childhood through adulthood.

3.4 Discussion of Findings

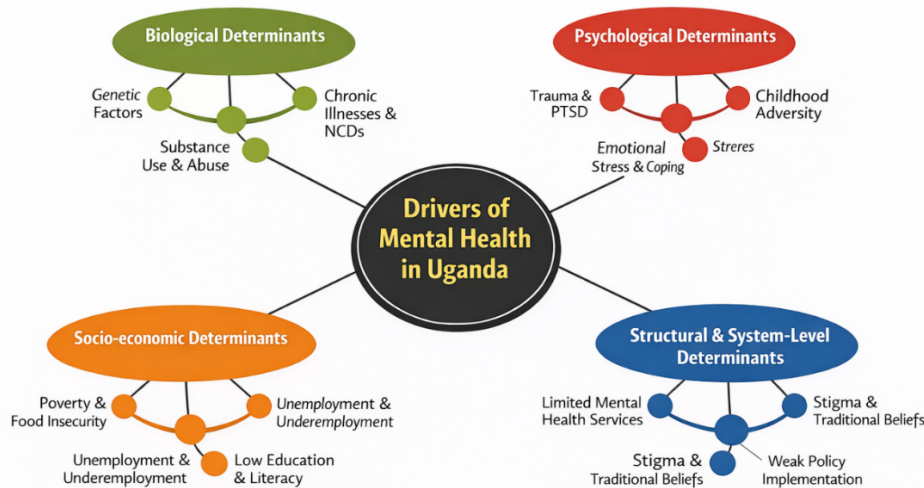
This section presents research findings from both the theoretical and empirical perspectives, as well as the quantitative results, as discussed below.

3.4.1 Findings from Theoretical and Empirical Literature

Mental health is shaped by a complex interacting set of factors that is broadly classified into three main pillars: biological, psychological, and social/environmental factors. This approach, often referred to as the biopsychosocial model, suggests that mental well-being is not determined by a single cause, but rather by a combination of genetic factors, individual experiences, and environmental influences, as illustrated in Figure 11.



Figure 11: Drivers of Mental Health in Uganda



Source: Author's own illustration

Mental health disorders in Uganda are shaped by a complex interaction of socioeconomic, psychological, and biological factors. This systematic review synthesizes evidence from peer-reviewed studies published between 2020 and 2026 that examine socioeconomic drivers of mental health outcomes in Uganda (Annex 6). A total of 16 papers fulfilled the inclusion criteria and were read for themes. Across three thematic domains, the literature consistently demonstrates that poverty, food insecurity, exposure to violence, gender inequality, displacement, substance use, environmental stressors, and limited mental health literacy are key determinants of depression, anxiety, post-traumatic stress disorder (PTSD), and broader psychological distress. These findings underscore the need for multisectoral, equity-focused mental health interventions that are embedded within social protection, education, gender, and humanitarian systems.

1. Psychological Determinants of Mental Health in Uganda

Psychological factors constitute the core drivers of Uganda's mental health burden. Evidence from 13 of 18 (72%) of high impact peer-reviewed studies in Uganda consistently identifies depression, anxiety, post-traumatic stress disorder (PTSD), suicidality, and substance use as the most prevalent conditions. Depression emerges as the dominant disorder, with prevalence estimates ranging from 6% in community-based surveys to over 40% among highly vulnerable populations, including mothers of malnourished children and adolescents in post-conflict regions. Anxiety disorders are similarly widespread across age groups, with particularly high prevalence among refugees and conflict-affected populations.

Across studies, poverty, unemployment, food insecurity, educational attainment, and inadequate housing were robust and recurring predictors of depression, anxiety, and psychological distress (Admassu et al., 2024; Hawkins et al., 2020; Ssebunnya et al., 2022;

Naisanga et al., 2022; Mugisha et al., 2025). Financial insecurity heightened chronic stress, eroded coping capacity, and restricted access to mental health services. Conversely, education consistently emerged as a protective factor, likely through improved health literacy, employment opportunities, and social mobility.

PTSD is highly concentrated in post-conflict districts, with prevalence estimates approaching 36%, strongly correlated with direct exposure to war-related trauma (Akena et al., 2023; Mayer, 2020). Refugee populations in Uganda report particularly high levels of PTSD, depression, and anxiety, driven by cumulative trauma exposure, prolonged displacement, poverty, ongoing insecurity, and weak social support systems (Bapolisi et al., 2020; Seruwagi et al., 2022). Duration of displacement and absence of protective safety structures further exacerbate mental health outcomes.

High psychological morbidity is also documented among survivors of intimate partner violence, childhood abuse, and conflict-related trauma (Thumann et al., 2016; Kisaakye et al., 2024; Anguzu et al., 2025). Women and adolescents are disproportionately affected, reflecting entrenched gender inequalities, power imbalances, and social vulnerability. Exposure to physical, sexual, emotional, and gender-based violence consistently ranks among the strongest predictors of depression, anxiety, and suicidality.

Although less frequently studied, suicidal ideation and behavior are reported in up to 10% of adults, particularly among people living with HIV (Tutlam et al., 2025). This highlights the intersection of psychological distress, chronic illness, and stigma.

Substance use, including alcohol and drugs, represents both a maladaptive coping strategy and an independent risk factor for mental illness. Prevalence estimates indicate substance use rates of approximately **19% among adolescents** and **15% among adults**. Strong associations were observed between substance use, risky sexual behavior, depression, anxiety, and psychological distress (Forry et al., 2019; Anyanwu et al., 2023; Batte et al., 2024). High comorbidity complicates diagnosis and treatment, contributing to poorer clinical outcomes.

Family dynamics play a critical role in shaping adolescent mental health. Caregiver depression, family history of mental illness, and non-relative caregiving arrangements are consistently associated with higher emotional and behavioral problems (Meyer et al., 2017; Ainamani et al., 2022; Bobholz et al., 2025). These findings underscore the intergenerational transmission of psychological distress and the importance of family-centered interventions. Protective psychological factors include family cohesion, social support, and school attendance, which buffer against depression and anxiety across multiple settings.

2. Socio-Economic Determinants

Socio-economic conditions represent the most consistent and cross-cutting determinants of mental health in Uganda. Poverty, unemployment, and food insecurity are repeatedly identified as strong predictors of depression, anxiety, and sustained psychological distress. Longitudinal studies in Kampala's urban informal settlements demonstrate that food insecurity increases the odds of persistent depression nearly tenfold, while unemployment and intimate partner violence further compound risk.

Among children and adolescents, orphanhood and domestic violence are strong predictors of depression and emotional distress. Gender disparities are evident across studies: females consistently show higher prevalence of depression and anxiety, while males are more likely to exhibit substance use and behavioral disorders (Rukundo et al., 2023; Mugisha et al., 2025). Adolescents, refugees, and residents of informal settlements experience layered vulnerabilities due to overlapping socio-economic disadvantages.

Refugee status and forced displacement intensify mental health risks, with elevated levels of PTSD, hopelessness, and stigma, particularly among refugee youth. Stigma, whether associated with mental illness, HIV, or sexual violence, emerges as a pervasive socio-economic determinant that undermines help-seeking behavior and worsens psychological outcomes (Favina et al., 2025; Seruwagi et al., 2022).

Household structure also influences mental health outcomes. Adolescents from larger families report higher rates of anxiety and disruptive behavior, likely reflecting resource constraints and reduced caregiver attention. In contrast, family cohesion and supportive parenting consistently serve as protective factors, reducing depression and behavioral challenges. Overall, socio-economic determinants demonstrate how structural inequality, household vulnerability, and social marginalization are translated into psychological morbidity.

3. Biological Determinants

Biological factors interact closely with psychological and socio-economic stressors to shape mental health outcomes in Uganda. Multiple studies document strong associations between mental disorders and chronic non-communicable diseases (NCDs), including hypertension, stroke, and obesity. For example, stroke survivors in Kampala exhibit a 31.5% prevalence of post-stroke depression, with physical disability emerging as a key predictor. Hypertension has also been linked to increased risk of major depressive disorder in population-based cohorts.

Chronic illness and public health crises significantly elevate mental health risk. HIV-positive status, warfarin use, and the COVID-19 pandemic are associated with higher levels of depression, anxiety, and suicidality (Clarke-Deelder et al., 2022; Rukundo et al., 2023; Mutinye et al., 2025). These biological stressors often interact with stigma, social isolation, and disruptions in health service delivery, amplifying psychological distress.

Maternal depression is a particularly salient biological and psychosocial determinant, with strong evidence linking maternal mental illness to adverse child outcomes, including malnutrition. Mothers of malnourished children consistently report significantly higher rates of depression than controls. Emerging biological concerns include attention-deficit/hyperactivity disorder (ADHD) and suicidality, with prevalence estimates of approximately 9% and 10%, respectively, among adults. Interestingly, higher body mass index (BMI) appears protective against generalized anxiety disorder in some cohorts, suggesting complex and context-specific biological interactions.

These findings underscore the bidirectional relationship between physical and mental health, wherein biological vulnerabilities exacerbate psychological distress, and mental illness, in turn, worsens physical health outcomes.

4. Structural Determinants

Structural determinants reflect the broader health system, policy environment, and societal context in which mental health is experienced in Uganda. Low mental health literacy, cultural stigma, and poor access to services remain major barriers to early detection and treatment (Akena et al., 2023; Ssebunnya et al., 2019). Many individuals seek care only when symptoms become severe, reinforcing cycles of disability, chronic illness, and poverty.

Uganda's mental health system is severely under-resourced, with less than 1% of the national health budget allocated to mental health and only 0.77 psychiatric beds per 100,000 population. Services are heavily concentrated in urban areas, leaving rural communities reliant on traditional and religious healers. This reliance reflects both cultural norms and systemic gaps in formal service provision rather than preference alone.

Climate vulnerability is an emerging structural determinant. Adolescents in disaster-prone regions such as Bududa and Manafwa report high levels of anxiety, depression, and disruptive behavior linked to recurrent landslides and floods. The COVID-19 pandemic further exposed structural weaknesses, with healthcare workers reporting extremely high levels of psychological distress due to financial strain, inadequate protective equipment, and limited institutional support.

Stigma operates at both community and institutional levels, embedded within health systems and social norms, perpetuating discrimination and obstructing access to care. Collectively, these structural determinants highlight the urgent need for system-level reforms, including increased financing, integration of mental health into primary healthcare, task-shifting approaches, stigma reduction, and climate-responsive mental health strategies.

Mental health in Uganda is shaped by interacting psychological, socio-economic, biological, and structural determinants, with poverty, trauma, chronic illness, and systemic underinvestment forming a reinforcing cycle of vulnerability. Effective responses therefore require multi-level interventions that address individual risk factors, strengthen families and communities, and reform health systems and social policies simultaneously.

3.4.2 Findings from Empirical Estimations – Ugandan Perspective

Despite the wide range of factors that drive mental health in Uganda, this study has strictly considered education, unemployment, income, and urbanization rate, mainly due to constraints faced in obtaining data on many others variables. Again, variables used in this study are macro-level variables and therefore might not bring out some of those micro-level details of mental illness which are presumed to be the main cause of mental illness among individuals and or households.

Correlation Results

Correlation results presented in Table 4 indicate a strong positive correlation (0.794) between mental illness and income implying that higher income levels are associated with higher levels of mental illness. However, it may not necessarily mean that income causes mental illness;

instead, higher-income regions may have better mental health diagnosis/reporting rates, access to services, or different stressors compared to lower income ones.

Table 3: Correlation Analysis Results

	Mental Illness	P-Value
Mental Illness	1	0.00
Income (GDP percapita)	0.793587**	0.00
Education (Average Years of Schooling)	0.845984**	0.00
Unemployment Rate	0.022738	0.91
Urbanisation Rate	-0.39736	0.05

Source: Authors' computation using data from the Global Burden of Disease and World Development Indicators

Correlation results further show a very strong positive correlation (0.846) indicating that higher education levels are associated with higher reported mental illness rates which may imply that better mental health awareness, more willingness to report, or more diagnosed cases in educated populations. However, this could also reflect stress associated with higher education attainment. The correlation between unemployment and mental illness was found to be at 0.023 implying almost no linear relationship. Unemployment rate here shows virtually no correlation with mental illness. In addition, correlation between urbanisation rate and mental illness was found to be -0.397 pointing to moderate negative correlation. Higher urbanisation rate is associated with lower mental illness. This could reflect migration patterns, younger populations in fast-growing cities, or under-reporting in rapidly urbanizing areas.

Regression Results

From the analysis, findings reveal that while mental health challenges are often clinically manifested at the individual level, they are deeply rooted in structural and socioeconomic conditions of Ugandans. Model results are presented in Table 5, demonstrating a high degree of explanatory power. The coefficient of determination (R^2) is 93.4%, indicating that variations in income, educational attainment, unemployment, and the urbanisation rate jointly account for a substantial proportion of the variation in mental illness. The F-statistic is 66.8 and is statistically significant at the 5% level, confirming the overall significance of the model. These results imply that the explanatory variables, when considered together, have strong joint explanatory power and that the model is stable and appropriate for inference.

Table 4: Multiple Linear Regression Results

Model	Unstandardized Coefficients		Standardized Coefficients	t	P-Value
	B	Std. Error	Beta		
Constant	11.246	0.267		42.197	0.000
Income (GDP Per Capita)	0.283	0.063	0.526	4.505	0.000
Education (Average Years of Schooling)	0.684	0.188	0.422	3.641	0.002
Unemployment	-0.098	0.09	-0.07	-1.098	0.286
Urbanisation Rate	-0.5	0.078	-0.416	-6.416	0.000

Source: Authors' computation using data from the Global Burden of Disease and World Development Indicators

1. **Education is a positive and significant determinant of mental illness in Uganda.** The estimated coefficient for education (0.684) suggests that a 1% increase in average years of education is associated with a 0.684% increase in reported mental illness, holding other variables constant. At face value, this positive association appears counterintuitive, given the well-established protective role of education in mental well-being. Literature provides several plausible explanations for this pattern in the Ugandan context.

- First, the education–expectation gap is particularly salient. As individuals advance through higher levels of education, expectations regarding employment, income security, and social mobility intensify. In an economy characterized by limited formal-sector absorption and rising graduate unemployment, unmet expectations can generate frustration, anxiety, and depressive symptoms. This burden is disproportionately borne by individuals from poorer households who face significant financial sacrifices to attain education, often through loans or household asset depletion, thereby amplifying psychological stress when returns to education are uncertain.
- Second, higher educational attainment is associated with greater cognitive awareness and mental health literacy, which may increase the likelihood of recognizing, reporting, and seeking help for mental health symptoms. In this sense, higher education may not increase the true incidence of mental illness, but rather its detection and disclosure. Thirdly, education may inadvertently increase social isolation and competitive pressure, especially in urban and academic environments where individual achievement is emphasized over communal support. These dynamics can erode traditional coping networks, particularly among young adults.

Conversely, a substantial body of literature affirms that education enhances problem-solving skills, self-efficacy, health literacy, and adaptive coping mechanisms, factors that typically protect against psychological distress (Marmot, 2018). In Uganda, education also strongly influences labour market access and long-term income trajectories, meaning that limited or interrupted schooling due to poverty, early marriage, disability, or conflict exposure is consistently associated with heightened mental health vulnerability in adulthood.

Importantly, the relationship between education and mental health is non-linear. Evidence suggests that individuals with mid-level qualifications but constrained employment prospects may experience worse mental health outcomes than both low- and high-education groups, largely due to labour market frustration and status inconsistency (Majcherek, Kowalski, & Lewandowska, 2022). Education also plays an indirect role in shaping stigma: higher educational attainment is generally associated with more positive attitudes toward mental illness and help-seeking (Braathu, Kabatooro, & Babirye, 2025), although cultural relevance remains critical for effective awareness interventions.



Overall, the literature indicates that expanding educational access alone is insufficient. Educational investments must be accompanied by improved quality, stronger alignment with labour market needs, and the integration of psychosocial support services within educational institutions.

2. **Income is a positive and significant determinant of mental health.** Income was found to have a positive and statistically significant coefficient (0.283), indicating that a 1% increase in income is associated with a 0.283% increase in reported mental illness. While counter to conventional assumptions, this finding aligns with several contextual realities highlighted in the literature. In Uganda, higher income is often associated with intense labour demands, multitasking, and extended working hours, particularly among self-employed individuals and professionals in the informal and semi-formal sectors. Higher-income earners frequently shoulder substantial social obligations, including extended family support, school fees, healthcare costs, and community contributions. These responsibilities, combined with pressure to maintain socioeconomic status, contribute to chronic stress and anxiety.

Additionally, higher income is linked to aspirational stress, including consumer debt, lifestyle inflation, job insecurity in formal employment, and the gradual erosion of traditional family support systems. Relative deprivation and income inequality further exacerbate psychological strain, as individuals compare themselves to higher-status peers despite material improvements. Similarly, the literature strongly confirms that income adequacy is a fundamental protective factor for mental health. Stable income ensures access to food security, safe housing, healthcare, and education which are the core determinants of psychological well-being. For the majority of Ugandan households reliant on volatile informal earnings, financial insecurity is a persistent source of stress, depression, and caregiver burden (WHO, 2020). Among children and adolescents, poverty is associated with emotional distress, impaired development, and reduced educational engagement, creating intergenerational cycles in which poor mental health undermines future productivity (ICHAD, 2019).

Access to mental health care in Uganda remains limited, centralized, and underfunded, making treatment contingent on the ability to afford transport, medication, or private services (Boardman & Ovuga, 2020). Consequently, income influences not only exposure to stress but also the likelihood of diagnosis and treatment. The literature therefore underscores the importance of social protection mechanisms, including cash transfers, livelihood support programs, and universal health coverage, as critical tools for alleviating poverty-related psychological distress.

- 3. Unemployment doesn't significantly determine mental health in Uganda.** The unemployment coefficient (-0.098) is negative and statistically insignificant, suggesting no reliable relationship between measured unemployment and mental illness in the model. This finding diverges from conventional evidence in high-income settings but is understandable within Uganda's labour market structure. Uganda's economy is dominated by the informal sector and subsistence agriculture, where the binary distinction between employment and unemployment is blurred. Many individuals classified as unemployed engage in informal income-generating activities, family labour, or seasonal work, which may buffer against the psychological distress typically associated with complete joblessness. Strong kinship networks and informal social support systems further mitigate the mental health impact of unemployment.

However, literature emphasizes that employment quality matters more than employment status. Precarious work characterized by low wages, job insecurity, long hours, and lack of legal protection is strongly associated with stress, anxiety, burnout, and substance misuse (MoH, 2022). Even among the employed, adverse psychosocial working conditions, high demands coupled with low autonomy, as described by Karasek's Job Demand–Control Model are linked to poor mental health outcomes, particularly in urban and industrial settings. Youth are especially vulnerable due to skill mismatches, labour market saturation, and limited upward mobility, which erode optimism and future expectations. These findings highlight the need for mental health strategies that extend beyond job creation to include decent work standards, youth employment programs, and workplace mental health interventions.

- 4. Urbanisation rate has a negative effect on mental health in Uganda.** Urbanisation rate exhibited a significant negative coefficient (-0.5), indicating that a 1% increase in urbanisation rate is associated with a 0.5% decrease in mental illness. This finding aligns with expectations in the Ugandan context, though it requires careful interpretation. For many Ugandans, particularly young and educated individuals, migration from rural to urban areas represents an escape from rural poverty, land fragmentation, limited services, and restrictive social norms. Urban centers offer perceived opportunities for employment, education, social mobility, and access to healthcare and recreational services. The initial phase of urban migration is often associated with optimism, improved autonomy, and enhanced social well-being.

This result may also reflect selection effects, as migrants tend to be healthier, more resilient, and more motivated than the general population, thereby skewing urban mental health indicators positively. Additionally, the model may capture short- to medium-term effects; the adverse mental health consequences of urban living, such as overcrowding, pollution, crime, high living costs, and social isolation, often emerge over time and may not be fully reflected in contemporaneous data. The literature therefore suggests that while urbanization can offer mental health advantages in early stages, unplanned and inequitable urban growth risks generating new psychosocial stressors. Integrating mental health considerations into urban planning, housing policy, and service delivery is essential to sustain these initial benefits.

3.5 Conclusions

Uganda's mental health outcomes are closely interlinked with the country's socio-economic factors. Study findings as well as patterns from national data sources, including the Uganda National Household Survey (UNHS) 2024, Uganda Demographic and Health Survey (UDHS) 2022 and the National Population and Housing Report (2024), indicate that income, unemployment, education, and urbanisation rate significantly influence mental well-being. The findings presented in this section above highlight the multifaceted pathways through which socio-economic factors shape mental health outcomes.

For instance, while higher income is often protective, for Uganda's case it is associated with increased mental illness. This reveals that financial growth solely may not improve mental well-being if it leads to aspirational stress, inequality, and inadequate social safety nets, underscoring the need for broader social protection measures alongside economic development. The relationship between education and mental health is complex and context-dependent, as higher education in Uganda is associated with increased mental illness, likely due to unmet expectations and labour market frustrations rather than a lack of cognitive benefits, indicating that improving educational access must be coupled with better labour market alignment, social and psychosocial support.

Finally, the urbanisation rate is associated with lower mental illness highlighting that migration to cities may initially improve well-being by offering opportunity and escape from rural hardships. However this positive effect may be temporary or skewed by selection bias, portraying a need for long-term urban planning to sustain these mental health gains or else the situation worsens for instance the Central region, with relatively higher income and education levels, and social amenities tend to record high incidences of mental health conditions like suicide / severe psychological distress compared to the Northern and Eastern regions, where historical conflict, poverty, and limited-service coverage persist. Access to mental health services largely remains highly centralized, with most facilities located in urban areas such as Cities leaving rural populations underserved and reliant on traditional healers or informal networks. Regional disparities reinforce inequalities in access to mental health services and exposure to risk factors.

CHAPTER 4: COST OF MENTAL HEALTH AND INACTION (HUMAN AND ECONOMIC BURDEN) IN UGANDA

4.1. Introduction

Mental disorders are a major and growing global challenge. Contextually, by 2019, approximately 12% of the global population suffered from mental disorders, *accounting for about 5% of the global Disability-Adjusted Life Years (DALYs)*. According to the 2019 Global Burden of Disease (GBD) study, mental disorders ranked among the top 10 global diseases in terms of prevalence. and the DALYs for mental disorders have risen from the 13th position in 1990 to the 7th in 2019, indicating a rapid increase in the global disease burden.

Global prevalence of depression and anxiety is rising rapidly, fueling roughly USD 1 trillion in lost economic output each year (GBD, 2019). However, investing in mental health can yield high returns; for every dollar spent on treating common mental illnesses like depression and anxiety, societies gain about *US\$4 in improved health and productivity* (Le Thi Minh Que & Vairavan, 2023). Quantifying the burden of mental illness in both health and financial terms is therefore crucial for understanding what is at stake and for making the case to act.

From a policy perspective, measuring the economic impact of mental health is vital for informed population and development planning. Mental illness imposes not only human suffering but also significant indirect costs through reduced productivity, higher unemployment, and strained social systems. In low-income countries like Uganda, the stakes are especially high. Uganda is ranked among the top six countries in Africa in terms of rates of *depressive disorders at 4.6%*, while 2.9% live with anxiety disorders (Omona et al., 2025)). In 2017, it was estimated that **35% of Ugandans experience a mental health condition**, with around 15% of the population requiring treatment (Molodynski, Cusack, & Nixon, 2017).

The 2025 World Bank report on “Financing Mental Health in Africa shows that Uganda’s mental disorder DALY rate (1,634 per 100k) exceeded the African average (1,416 per 100k). This implies that Mental health conditions in Uganda lead to a higher loss of healthy life compared to the African average, as shown by elevated DALY rates due to early death and long-term disability from mental disorders. However, investment is very low; mental health spending accounts for *less than 1% of the health budget*, below the WHO's recommended 5%. Such underinvestment, common in many countries, highlights the importance of economic analysis. Policymakers need clear evidence of the costs of inaction versus the benefits of intervention to prioritize mental health amid limited resources. By framing mental health as not only a medical issue but also a development and economic issue, an economic analysis can elevate its priority on the national agenda.

In this context, the study sought to comprehensively quantify the burden and costs of mental illness in Uganda through the year 2040, with a view to informing evidence-based policy. The study combines a cost-of-illness approach (summing direct medical costs and indirect

productivity losses) with burden-of-disease metrics (Quality-Adjusted Life Years (QALYs), and mortality) under multiple scenarios. The analysis will compare a “business-as-usual” trajectory where current trends and investments in prevention and care persist against scenarios in which mental health services are substantially scaled up.

This comparative scenario modeling highlights the cost of inaction versus the potential benefits of investment. Specifically, the introduction addresses the key questions in Text Box

Text box 2: Key Questions

- i. What is the projected human burden of mental disorders in Uganda over the next two decades (e.g., total DALYs and lives lost) if current trends continue unabated?
- ii. What are the economic costs associated with this burden under a business-as-usual scenario, including both direct healthcare expenditures and indirect losses in productivity?
- iii. How would scaled-up mental health interventions change these outcomes? In other words, to what extent could improvement in prevention and treatment reduce DALYs and avert economic losses by 2040?
- iv. What is the return on investment for strengthening mental health services, in other words, how do the costs of interventions compare to the avoided health burden and economic gains achieved?

4.2. Analytical Framework and Methodology

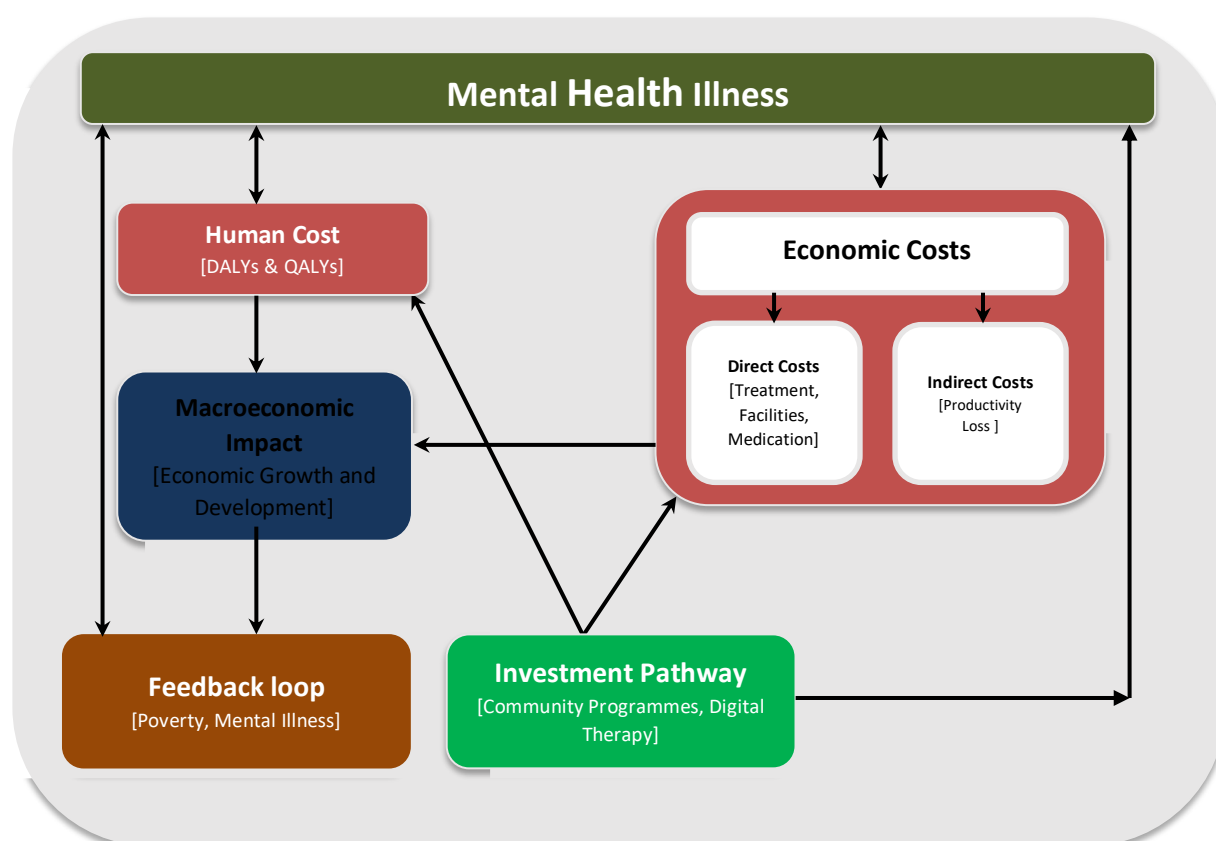
This analytical framework and methodology outline how the total cost of mental health (human and economic costs) and the cost of inaction has been quantified in Uganda through 2040. It covers all mental, neurological and substance-use disorders and proposes steps to project future trends. The methodology includes cost-of-illness analysis (direct and indirect costs) and burden-of-disease metrics (DALYs/QALYs, mortality).

4.2.1. Conceptual Framework

The analysis adopted a comprehensive cost-of-illness framework for mental disorders. As illustrated in global mental health economics studies, mental illnesses impose costs at multiple levels: direct costs (treatment, facility operations, medications), indirect costs (lost earnings due to disability, early retirement, caregiver time), and intangible costs (reduced quality of life). These costs interact with broader development: untreated mental illness leads to higher poverty and vice versa, forming a feedback loop that undermines national productivity.

The conceptual framework in Figure 12 illustrates how mental health disorders translate into human cost/burden (measured in DALYs, QALYs, morbidity, and mortality) and economic burden (direct, indirect, and intangible costs). These feed into broader macroeconomic impacts such as GDP growth and national productivity. A feedback loop highlights the cycle between poverty and poor mental health. Finally, investment pathways (e.g., community programs, digital therapy, prevention) intervene to reduce both human and economic burdens, breaking the cycle.

Figure 12: Conceptual Framework for Cost-of-Illness of Mental Health in Uganda



Source: Author's own illustration, based on study design (2025)

The conceptual model links mental health epidemiology to economic impact. On the health side, we will quantify disease burden via DALYs (combining years of life lost to premature mortality and years lived with disability) and QALYs (quality-adjusted life-years). These metrics capture the lost healthy life from mental disorders. On the economic side, we capture direct costs (healthcare expenditures) and indirect costs (productivity effects-presenteeism, and absenteeism). We will explicitly model feedback: for example, how rising mental health burdens translate into slower GDP growth, and how new investments (e.g., community programs or digital therapy) could break the cycle of disability and poverty. Finally, by contrasting a baseline pathway with an enhanced intervention scenario, we frame our analysis

as a value-of-investment exercise: every DALY averted, or output gained under investment is weighed against the costs of scaling up services.

4.2.2. Theoretical Framework

This study adopted the burden-of-disease theory (DALYs) and human capital approach, originally proposed by Weisbrod (1961) and later applied on studies in Mauritius, Kenya, Zambia, the East African Community (EAC), and the African region, to estimate the economic value of Disability-Adjusted Life Years (DALYs) lost due to health conditions. The DALYs uses time as a common metric taking into consideration both premature death and healthy years lost due to problems associated with living with a disease or health condition in this case mental health. DALYs are calculated as the sum of Years of Life Lost (YLL) due to premature mortality and Years Lost due to Disability (YLD). The YLL is the difference between death and average life expectancy (premature death). The approach values lost health in terms of forgone economic output, recognizing that health contributes directly to the creation of human capital across an individual's life cycle. At the modeling level, we will leverage epidemiological forecasting theories. Trends in mental disorder prevalence will be projected using time-series methods. Scenario analysis guides our intervention modeling: a counterfactual "status quo" versus "scale-up" scenario, enabling computation of the economic cost of inaction (the gap between them). This is analogous to investment case studies that quantify returns on health spending.

4.2.3. Empirical/Estimation Model

The empirical model translates the conceptual and theoretical frameworks into measurable evidence of how mental health conditions affect Uganda's population and economy. It estimates both the human burden measured through DALYs and the economic burden, which includes direct healthcare costs and productivity losses that occur when people miss work (absenteeism) or work below capacity (presenteeism). Using data from Uganda's national statistics and international sources such as the Global Burden of Disease (GBD) and the World Bank, the model projects the trend up to 2040.

The analysis first estimates how many years of healthy life Ugandans lose due to mental disorders and then assigns an economic value to those losses by linking them to average national income (GDP per capita). To ensure accuracy, it uses net GDP per capita, that is, total GDP per person minus health expenditure, because people derive value from their overall well-being and consumption, not from the cost of medical treatment itself. This approach, consistent with WHO guidance and Chisholm et al. (2016), ensures that the estimated economic losses from mental illness reflect forgone income, investment, and productivity.

In essence, every DALY lost due to mental disorders represents a direct loss in Uganda's economic potential. The model also estimates the cost of inaction by comparing two scenarios: (i) a "business-as-usual" path where current service trends, investments and gaps persist, and (ii) a "scale-up" scenarios where mental health interventions are expanded to avert 20%, 50%,

and 80% of the burden. The difference between these scenarios represents the cost of inaction, the economic and social losses Uganda will face if investments in mental health remain low, and the potential gains achievable through sustained investment and improved service coverage.

4.2.4. Study Population and Data Sources

The analysis covered the entire population of Uganda and was based on a population-level modelling approach using secondary data. No primary household or individual sampling for epidemiological purposes was undertaken. Instead, the study drew on nationally and regionally representative datasets to characterize the prevalence, incidence, and outcomes of mental health conditions.

Key data sources included Uganda's UBOS census data and epidemiological data obtained from the Global Burden of Disease (GBD) database. For productivity losses, data from the WHO, UNDP, and the UN Inter-Agency Task Force-Uganda MHIC report 2021 were used to quantify absenteeism and presenteeism. GDP per capita and health expenditure per capita data were obtained from the World Development Indicators (WDI) database.

4.3. Human Burden of Mental Health

Mental health conditions impose a heavy human burden in Uganda, affecting individuals' well-being as well as their families and communities. This burden can be understood mainly through the Disability-Adjusted Life Years (DALYs) and Quality-Adjusted Life Years (QALYs). For computational purposes and data constraints, the study adopts the DALYs approach. A Disability-Adjusted Life Year (DALY) measures the total number of years lost due to ill-health, disability, or premature death. DALYs are usually expressed per 100,000 population, but can also be estimated for the whole population.

In this study, we calculate the monetary value of DALYs lost in Uganda due to mental health based on the DALYs data are from the Institute for Health Metrics and Evaluation (IHME) Global Burden of Disease (GBD) Study 1990-2023 data³. DALY calculations for GBD are based on more than 120 000 cause-related data sources. Under the GBD study, DALYs of healthy life lost due to mental health are computed through summing of years of life lost (YLLs) and years lived with disability (YLDs). The disability being mental health disorders. Methodological details and sources of data used in the GBD study 2023 are from an article published by the GBD 2023 Diseases and Injuries Collaborators⁴.

4.3.1. Disability-Adjusted Life Year

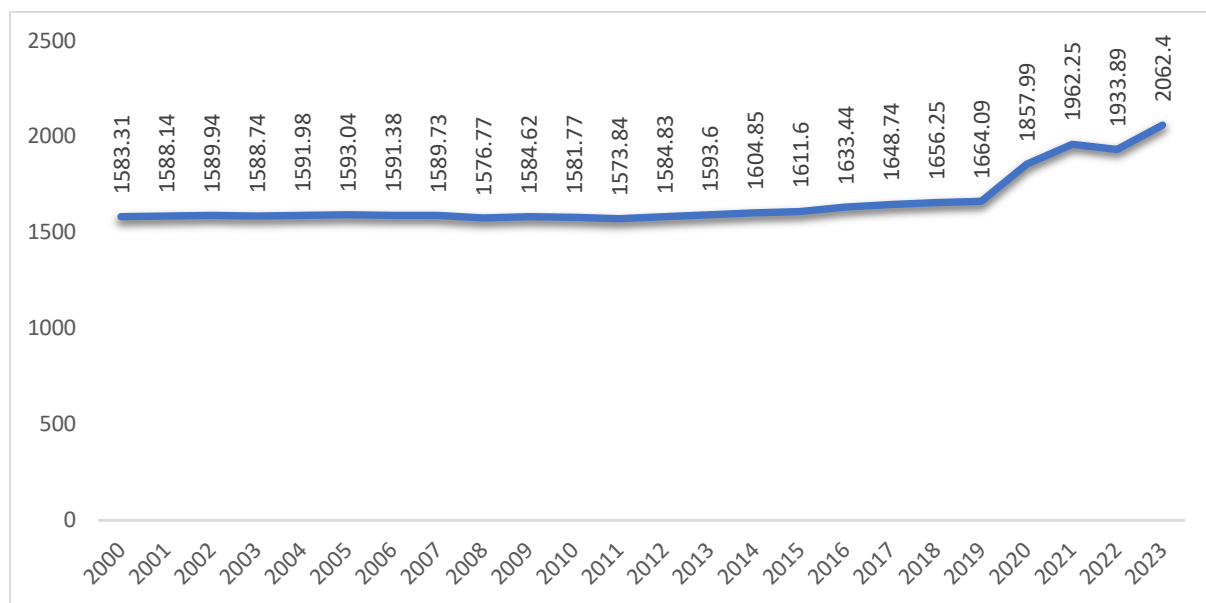
The burden of mental health disorders in Uganda steadily increased between 2000 and 2023, indicating a growing public health challenge. Between 2000 and 2023, Uganda experienced a gradual but sustained increase in the burden of mental health disorders as measured (Figure

³ Global Burden of Disease (GBD)-Institute for Health Metrics and Evaluation: <https://vizhub.healthdata.org/gbd-results/>

⁴ GBD2023 Diseases and Injuries Collaborators: Burden of 375 diseases and injuries, risk-attributable burden of 88 risk factors, and healthy life expectancy in 204 countries and territories, including 660 subnational locations, 1990–2023: a systematic analysis for the Global Burden of Disease Study 2023. *The Lancet*, 406(10513), 1873–1922: <https://www.thelancet.com/action/showPdf?pii=S0140-6736%2825%2901637-X>

13). In 2000, the DALY rate stood at approximately 1,583 per 100,000 population, remaining relatively stable through the mid-2000s, with only marginal year-to-year fluctuations. This period of stagnation suggests that the overall burden of mental disorders was persistent but not significantly escalating, likely reflecting under-detection and limited investment in mental health services during the early 2000s. From around 2013 onward, the data show a clear upward trajectory, with DALYs rising from roughly 1,601 per 100,000 to over 1,930 per 100,000 by 2023. The sharpest increases occurred between 2019 and 2021, a period associated with multiple socioeconomic stressors, including the COVID-19 pandemic, which exacerbated anxiety, depression, and other mental health conditions. This rise highlights the growing public-health significance of mental disorders in Uganda, reflecting both increasing incidence and improved recognition.

Figure 13: Disability-adjusted life years (DALYs) from Mental Health in Uganda -2000-2023



Source: Generated by authors using data from the Global Burden of Disease

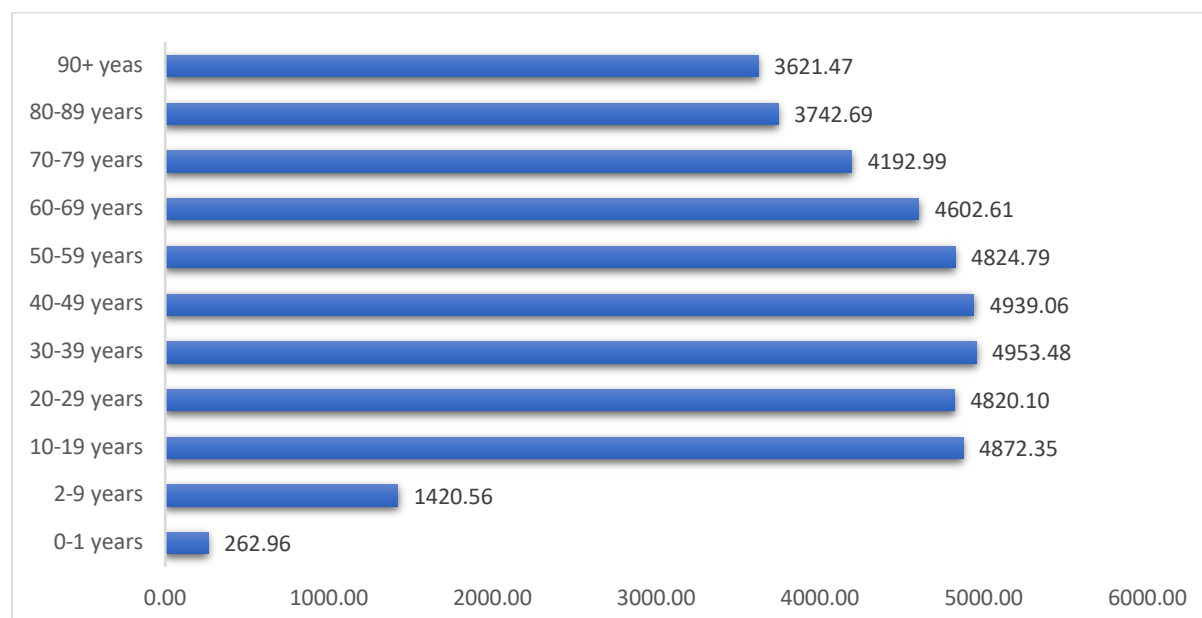
Mental health-related DALYs are highest among Ugandans aged 20-49 years, underscoring the heavy toll of mental disorders on the country's most productive population segment.

Figure 14 shows that as of 2023, the burden of mental health disorders varied significantly across age groups, with the highest DALYs observed among working-age adults. The 30-39 years and 40-49 years cohorts recorded the greatest burden, with DALY rates of 4,955.48 and 4,939.06 per 100,000 population, respectively. The burden remains elevated across the 20-59-year range, averaging over 4,800 DALYs per 100,000. DALY rates begin to decline in older age groups, falling to 4,192.99 among those aged 70-79 years, and further to 3,621.47 among individuals aged 90 years and above.

Conversely, younger age groups (below 20 years) exhibit markedly lower DALYs (1,420.56 for ages 2-9 years and 262.96 for ages 0-1 years), though rising rates among adolescents (10-19 years: 4,872.35) point to increasing mental health risks in youth, potentially linked to social

media exposure, academic stress, and family instability. Overall, this reflects the high prevalence of mental health challenges among individuals in their most economically active and socially demanding years. This pattern likely mirrors the cumulative effects of unemployment, underemployment, family pressures, and limited access to psychosocial support.

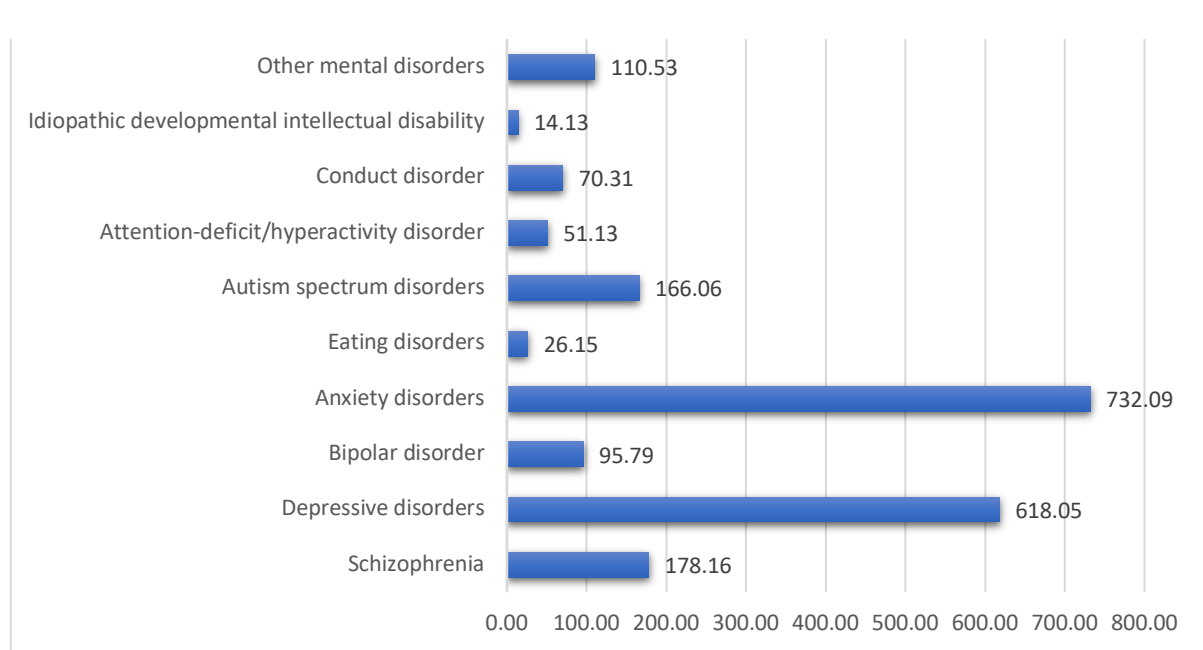
Figure 14: Disability-adjusted life years (DALYs) from Mental health by age groups in Uganda in 2023



Source: Generated by authors using data from the Global Burden of Disease

Anxiety and depressive disorders are the leading contributors to Uganda's mental health burden, jointly accounting for the majority of DALYs in 2023. Figure 15 shows that anxiety disorders registered the highest burden at 732.09 DALYs per 100,000 population, followed closely by depressive disorders at 618.05 DALYs per 100,000 in 2023. These two categories alone contribute a significant proportion of Uganda's total mental health burden, reflecting the widespread impact of chronic stress, economic uncertainty, trauma exposure, and limited access to psychosocial care. Schizophrenia and autism spectrum disorders also represent notable contributors, with DALYs of 178.16 and 166.06 per 100,000, respectively.

Other conditions such as bipolar disorder (95.79), conduct disorder (70.31), and attention-deficit/hyperactivity disorder (51.13) remain less prevalent but still contribute to the overall burden. Idiopathic developmental intellectual disability (14.13) and eating disorders (26.15) show relatively lower DALY values. Overall, Uganda's 2023 mental health profile reveals a pattern consistent with global trends where common mood and anxiety disorders constitute the bulk of the disease burden underscoring the need for community-level interventions, integrated counselling services, and scaled-up mental health funding within the primary health care system.

Figure 15: The DALYs in Uganda by mental health disorders in 2023

Source: Generated by authors using data from the Global Burden of Disease

4.3.2. Monetary value of DALYs

The monetary value of DALYs lost to mental health disorders in Uganda surged from UGX 283 billion in 2000 to UGX 2.96 trillion in 2023, signaling a rapidly growing economic burden linked to mental illness. Between 2000 and 2023, the economic cost associated with Disability-Adjusted Life Years (DALYs) lost due to mental health conditions in Uganda rose sharply, reflecting both the growing burden of mental disorders and the country's rising income levels (Table 5). In 2000, the total monetary value of DALYs lost was estimated at *US\$ 77.7 million (UGX 283.5 billion)*, but by 2023, this figure increased more than tenfold to approximately *US\$ 812.2 million (UGX 2.96 trillion)*.

The steady rise over time indicates that mental health-related productivity losses are becoming a major economic concern. The cumulative increase in the monetary value of DALYs underscores how untreated or poorly managed mental health conditions translate into escalating economic costs through lost productivity, reduced labour participation, and diminished quality of life. By 2023, the estimated UGX 2.96 trillion in lost economic value reflects not only the direct toll on individuals and families but also the broader macroeconomic implications for Uganda's human capital and national development goals.

Table 5: Monetary value of DALYs in Uganda 2000-2023 (UGX)

Year	No of DALYs	Monetary value (USD)	Monetary value (UGX)
2000	321416.76	77,660,718	283,461,620,700
2001	331742.96	72,990,086	266,413,813,900
2002	344532.07	77,946,936	284,506,316,400
2003	356003.07	84,191,166	307,297,755,900
2004	368250.63	100,800,000	367,920,000,000
2005	380456.26	115,900,000	423,035,000,000
2006	392258.26	123,500,000	450,775,000,000
2007	404273.55	147,700,000	539,105,000,000
2008	413381.6	178,900,000	652,985,000,000
2009	428185.86	323,100,000	1,179,315,000,000
2010	440401.32	339,800,000	1,240,270,000,000
2011	452079.4	352,800,000	1,287,720,000,000
2012	469032.87	347,200,000	1,267,280,000,000
2013	486526.95	372,400,000	1,359,260,000,000
2014	505376.62	427,100,000	1,558,915,000,000
2015	523796.05	431,000,000	1,573,150,000,000
2016	547426.02	390,700,000	1,426,055,000,000
2017	569635.44	418,000,000	1,525,700,000,000
2018	590049.83	448,000,000	1,635,200,000,000
2019	610671.48	481,700,000	1,758,205,000,000
2020	706397.17	565,900,000	2,065,535,000,000
2021	767997.56	638,800,000	2,331,620,000,000
2022	777679.56	714,600,000	2,608,290,000,000
2023	854814.56	812,200,000	2,964,530,000,000

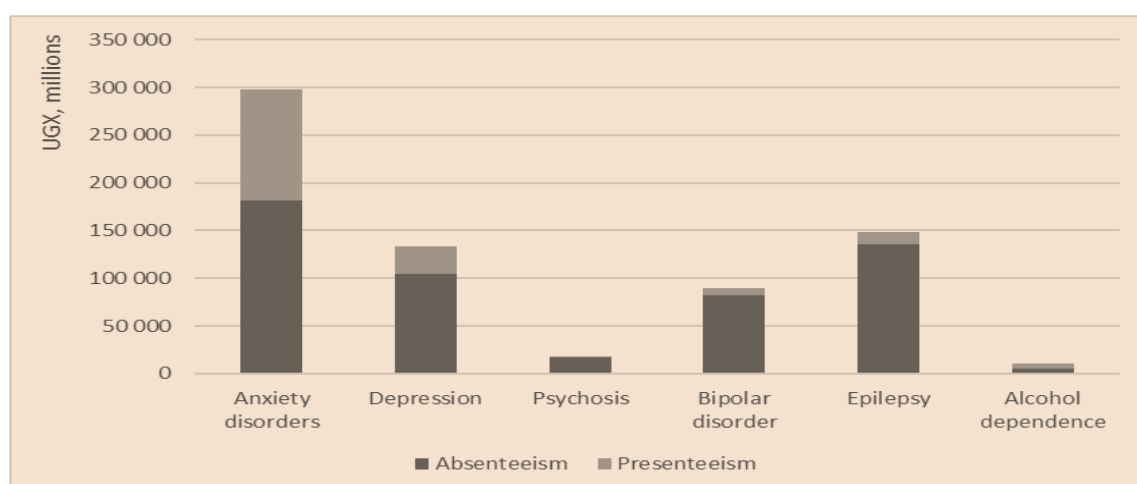
Source: Generated by authors using data from the Global Burden of Disease

4.4 Economic Burden of Mental Health

The economic burden of mental health is comprised of direct and indirect costs. Direct costs of mental health disorders refer to the expenditures on treatment, care, and related services. On the other hand, indirect costs constitute the larger share of the economic burden of mental illness. Indirect costs encompass the lost productivity from individuals not being able to work at full capacity (or at all) due to mental health issues, as well as the opportunity cost of caregiving. The total health expenditure for mental health in Uganda was 137.3 billion UGX (US\$37.6 million) in 2021.

Total health expenditures could not be disaggregated by mental health condition. The indirect economic losses caused by mental health conditions were estimated as the sum of losses due to absenteeism, presenteeism, and premature death. The total combined cost of absenteeism and presenteeism in Uganda is presented in Figure 16. The total number of working days absent was estimated to be 18.7 million for absenteeism and 6.0 million for presenteeism, which resulted in a total cost of 696.6 billion UGX (US\$191.0 million) in 2021. Absenteeism and presenteeism costs are highest for anxiety disorders.

Figure 16: Costs of absenteeism and presenteeism for mental health conditions (2021 UGX, millions)



Source: WHO, UNDP, and the UN Inter-Agency Task Force-Uganda MHIC report 2021.

Uganda loses far more from indirect costs than it spends on treating mental health conditions. Table 6 shows the total direct and indirect costs of mental health conditions in Uganda. The indirect economic losses are much higher than the direct losses. Total expenditure on health care for mental health conditions was 137.3 billion UGX (US\$37.6 million). In addition, the losses to the economy due to absenteeism and presenteeism amounted to 696.6 billion UGX (US\$191 million). The total direct and indirect (productivity) costs of mental health conditions on the Uganda economy in 2021 were 833.9 billion UGX (US\$228.6 million).

Table 6: Economic burden of mental health conditions in Uganda (2021 UGX, millions)

Cost	Total costs (2021 UGX, millions)	Total costs (2021 USD, millions)
Direct costs		
Health care expenditure	137,300	37.6
Total direct costs	137,300	37.6
Indirect costs		
Absenteeism	525,556	144.1
Presenteeism	171,075	46.9
Total indirect costs	696,631	191.0
Total	833,931	228.6

Source: World Health Organization, UNDP, and the United Nations Inter-Agency Task Force-Uganda MHIC report 2021

4.5 Cost of Inaction

The cost of inaction was computed by comparing two forward-looking scenarios using Uganda's projected mental health burden from 2024 to 2040. First, a Business-as-Usual (BAU) scenario was generated by projecting DALY rates forward based on historical trends. DALYs were modelled from a log-linear time trend and were then monetised using the human capital approach, where each DALY lost is valued at *net GDP per capita* (GDP per capita minus per-capita health expenditure) because people derive value from their overall well-being and consumption, not from the cost of medical treatment itself. Direct and indirect costs were also

projected by scaling the 2021 baseline expenditure on treatment, absenteeism, and presenteeism according to the growth in DALYs. Second, a set of scale-up scenarios was constructed by applying a linear reduction in disease burden (20%, 50%, and 80% DALY reduction by 2040). For each scale-up scenario, DALYs, direct costs, indirect costs, and the monetary value of lost productivity were recomputed by applying the reduction path to the BAU series. The cost of inaction is the difference between the BAU trajectory and each scale-up trajectory for every year. This gap represents the avoidable losses Uganda would incur if current mental health service gaps persist, quantified across human burden (DALYs), direct spending, indirect productivity losses, and their combined total economic cost.

4.5.1. Human Cost

Strengthening mental health interventions can significantly reduce Uganda's future economic losses associated with disability and premature mortality from mental disorders.

Table 7 shows the projected monetary value of DALYs lost to mental health disorders in Uganda from 2024 to 2040. Under the business-as-usual scenario, the economic cost is expected to rise sharply from US\$851 million in 2024 to US\$4.1 billion by 2040. However, scaling up mental health interventions could yield substantial savings of about US\$815 million under a 20% reduction scenario, US\$2.0 billion under a 50% reduction, and up to US\$3.3 billion under an 80% reduction by 2040. By 2040, the economic gains from reducing the burden of mental health conditions could be transformative, reinforcing the importance of integrating mental health into Uganda's broader human capital and development agenda.

Table 7: Projected Monetary Value of DALYs under Business-as-Usual and Scaled-Up Scenarios, 2024–2040

Year	Monetary value of DALYs under BAU (USD)	Monetary value of DALYs after Scale-Up Interventions-Reducing burden of disease (USD)			Saving (USD)		
		20 Percent	50 Percent	80 Percent	20 Percent	50 Percent	80 Percent
2024	851,400,000	851,400,000	851,400,000	851,400,000	-	-	-
2025	939,800,000	928,000,000	910,400,000	892,800,000	11,800,000	29,400,000	47,000,000
2026	1,037,000,000	1,011,000,000	972,400,000	933,500,000	26,000,000	64,600,000	103,500,000
2027	1,145,000,000	1,102,000,000	1,037,000,000	972,900,000	43,000,000	108,000,000	172,100,000
2028	1,263,000,000	1,200,000,000	1,105,000,000	1,010,000,000	63,000,000	158,000,000	253,000,000
2029	1,393,000,000	1,306,000,000	1,176,000,000	1,045,000,000	87,000,000	217,000,000	348,000,000
2030	1,537,000,000	1,422,000,000	1,249,000,000	1,076,000,000	115,000,000	288,000,000	461,000,000
2031	1,695,000,000	1,547,000,000	1,324,000,000	1,102,000,000	148,000,000	371,000,000	593,000,000
2032	1,869,000,000	1,682,000,000	1,402,000,000	1,122,000,000	187,000,000	467,000,000	747,000,000
2033	2,061,000,000	1,830,000,000	1,482,000,000	1,134,000,000	231,000,000	579,000,000	927,000,000
2034	2,273,000,000	1,989,000,000	1,563,000,000	1,136,000,000	284,000,000	710,000,000	1,137,000,000
2035	2,506,000,000	2,161,000,000	1,645,000,000	1,128,000,000	345,000,000	861,000,000	1,378,000,000
2036	2,763,000,000	2,348,000,000	1,727,000,000	1,105,000,000	415,000,000	1,036,000,000	1,658,000,000
2037	3,045,000,000	2,551,000,000	1,808,000,000	1,066,000,000	494,000,000	1,237,000,000	1,979,000,000
2038	3,357,000,000	2,769,000,000	1,888,000,000	1,007,000,000	588,000,000	1,469,000,000	2,350,000,000
2039	3,700,000,000	3,006,000,000	1,965,000,000	924,900,000	694,000,000	1,735,000,000	2,775,100,000
2040	4,077,000,000	3,262,000,000	2,039,000,000	815,500,000	815,000,000	2,038,000,000	3,261,500,000

Source: Authors' estimates based on GBD data, WBDI data, and cost-of-illness modelling.

4.5.2. Economic Cost

4.5.2.1. Direct Cos

Investing in scaled-up mental health interventions can generate substantial savings in health-care spending compared to maintaining the business-as-usual approach. The analysis in Table 8 highlights the projected direct costs of mental health disorders in Uganda under a “business-as-usual” (BAU) scenario and compares them with scenarios assuming scaled-up mental health interventions that progressively reduce the disease burden by 20%, 50%, and 80% between 2024 and 2040. Under the BAU path, where no major reforms or interventions are implemented, direct health-care costs are projected to increase steadily from USD 41.2 million in 2024 to USD 74.9 million by 2040. This continuous rise mirrors the expected growth in population, service utilization, and prevalence of mental health disorders. By contrast, scaling up interventions to reduce the burden of disease yields substantial cost savings. if the disease burden were reduced by 20%, Uganda could save approximately USD 534,624 in 2025, rising to USD 14.8 million by 2040. A 50% percent reduction scenario would save Uganda USD 37.1 million by 2040. An 80% reduction scenario would generate even larger savings up to USD 59.4 million per year by 2040. These findings underscore the fact that scaling up mental health care promises significant returns in avoided health-care costs.

Table 8: Projected Direct Costs and Potential Savings from Scaled-Up Mental Health Interventions in Uganda, 2024-2040

Year	Direct costs under BAU (USD)	Direct costs after Scale-Up Interventions- Reducing burden of disease (USD)			Savings (USD)		
		20 Percent	50 Percent	80 Percent	20 Percent	50 Percent	80 Percent
2024	41,223,945	41,223,945	41,223,945	41,223,945	-	-	-
2025	42,769,920	42,235,296	41,433,360	40,631,424	534,624	1,336,560	2,138,496
2026	44,373,871	43,264,524	41,600,504	39,936,484	1,109,347	2,773,367	4,437,387
2027	46,037,973	44,311,549	41,721,913	39,132,277	1,726,424	4,316,060	6,905,696
2028	47,764,482	45,376,258	41,793,922	38,211,586	2,388,224	5,970,560	9,552,896
2029	49,555,739	46,458,505	41,812,655	37,166,804	3,097,234	7,743,084	12,388,935
2030	51,414,171	47,558,108	41,774,014	35,989,919	3,856,063	9,640,157	15,424,252
2031	53,342,297	48,674,846	41,673,670	34,672,493	4,667,451	11,668,627	18,669,804
2032	55,342,732	49,808,459	41,507,049	33,205,639	5,534,273	13,835,683	22,137,093
2033	57,418,187	50,958,641	41,269,322	31,580,003	6,459,546	16,148,865	25,838,184
2034	59,571,475	52,125,041	40,955,389	29,785,738	7,446,434	18,616,086	29,785,737
2035	61,805,516	53,307,257	40,559,870	27,812,482	8,498,259	21,245,646	33,993,034
2036	64,123,337	54,504,837	40,077,086	25,649,335	9,618,500	24,046,251	38,474,002
2037	66,528,081	55,717,268	39,501,048	23,284,828	10,810,813	27,027,033	43,243,253
2038	69,023,007	56,943,981	38,825,442	20,706,902	12,079,026	30,197,565	48,316,105
2039	71,611,498	58,184,342	38,043,608	17,902,874	13,427,156	33,567,890	53,708,624
2040	74,297,062	59,437,649	37,148,531	14,859,412	14,859,413	37,148,531	59,437,650

Source: Authors' estimates based on GBD data and cost-of-illness modelling

4.5.2.2. Indirect Costs

Scaling up mental health interventions could greatly cut productivity losses from absenteeism, saving Uganda millions in annual economic costs by 2040. Table 9 presents the projected costs of absenteeism associated with mental health disorders from 2024 to 2040.

Under the business-as-usual (BAU) scenario, absenteeism costs are expected to rise steadily from US\$158 million in 2024 to US\$284.7 million by 2040, reflecting the growing prevalence of untreated mental illness. However, implementing scaled-up interventions could lead to substantial savings up to US\$ 56.9 million under an 20% reduction scenario, US\$ 142.4 million under a 50% reduction, and US\$ 227.7 million under 80% reduction by 2040. These results show that investing in preventive and early-response mental health programs would significantly reduce productivity losses due to absenteeism.

Table 9: Projected Absenteeism Costs and Potential Savings from Scaled-Up Mental Health Interventions, 2024–2040

Year	Absenteeism costs under Business as usual (USD)	Absenteeism costs after Scale-Up Interventions-Reducing burden of disease (USD)			Saving (USD)		
		20 Percent	50 Percent	80 Percent	20 Percent	50 Percent	80 Percent
2024	158,000,000	158,000,000	158,000,000	158,000,000	-	-	-
2025	163,900,000	161,900,000	158,800,000	155,700,000	2,000,000	5,100,000	8,200,000
2026	170,100,000	165,800,000	159,400,000	153,100,000	4,300,000	10,700,000	17,000,000
2027	176,400,000	169,800,000	159,900,000	150,000,000	6,600,000	16,500,000	26,400,000
2028	183,100,000	173,900,000	160,200,000	146,400,000	9,200,000	22,900,000	36,700,000
2029	189,900,000	178,000,000	160,200,000	142,400,000	11,900,000	29,700,000	47,500,000
2030	197,000,000	182,300,000	160,100,000	137,900,000	14,700,000	36,900,000	59,100,000
2031	204,400,000	186,500,000	159,700,000	132,900,000	17,900,000	44,700,000	71,500,000
2032	212,100,000	190,900,000	159,100,000	127,300,000	21,200,000	53,000,000	84,800,000
2033	220,100,000	195,300,000	158,200,000	121,000,000	24,800,000	61,900,000	99,100,000
2034	228,300,000	199,800,000	157,000,000	114,200,000	28,500,000	71,300,000	114,100,000
2035	236,900,000	204,300,000	155,400,000	106,600,000	32,600,000	81,500,000	130,300,000
2036	245,700,000	208,900,000	153,600,000	98,299,712	36,800,000	92,100,000	147,400,288
2037	255,000,000	213,500,000	151,400,000	89,237,866	41,500,000	103,600,000	165,762,134
2038	264,500,000	218,200,000	148,800,000	79,358,101	46,300,000	115,700,000	185,141,899
2039	274,400,000	223,000,000	145,800,000	68,611,814	51,400,000	128,600,000	205,788,186
2040	284,700,000	227,800,000	142,400,000	56,947,907	56,900,000	142,300,000	227,752,093

Source: Authors' estimates based on WHO, UNDP, UN Inter-Agency Task Force data and cost-of-illness modelling

Improving mental health services can also greatly enhance workforce productivity and yield substantial economic savings by reducing presenteeism losses. Table 10 presents the projected presenteeism costs (losses in productivity due to reduced performance while at work) associated with mental health disorders in Uganda from 2024 to 2040. Under the business-as-usual (BAU) scenario, presenteeism costs are projected to rise from US\$51.4 million in 2024 to US\$92.7 million in 2040, reflecting the increasing burden of untreated mental illness in the workforce. However, if scaled-up mental health interventions are implemented, Uganda could achieve significant savings. By 2040, potential savings amount to about US\$18.5 million under a 20% reduction scenario, US\$46.3 million under a 50% reduction, and US\$74.1 million under an 80% reduction in the burden of disease. These results emphasize that investing in mental health care not only improves well-being but also enhances workplace productivity.

Table 10: Projected Presenteeism Costs and Potential Savings from Scaled-Up Mental Health Interventions, 2024–2040

Year	Presenteeism costs under Business as usual (USD)	Presenteeism costs after Scale-Up Interventions-Reducing burden of disease (USD)			Saving (USD)		
		20 Percent	50 Percent	80 Percent	20 Percent	50 Percent	80 Percent
2024	51,420,293	51,420,293	51,420,293	51,420,293	-	-	-
2025	53,348,650	52,681,792	51,681,504	50,681,217	666,858	1,667,146	2,667,433
2026	55,349,323	53,965,590	51,889,990	49,814,390	1,383,733	3,459,333	5,534,933
2027	57,425,025	55,271,586	52,041,429	48,811,271	2,153,439	5,383,596	8,613,754
2028	59,578,570	56,599,641	52,131,248	47,662,856	2,978,929	7,447,322	11,915,714
2029	61,812,876	57,949,571	52,154,614	46,359,657	3,863,305	9,658,262	15,453,219
2030	64,130,974	59,321,150	52,106,416	44,891,681	4,809,824	12,024,558	19,239,293
2031	66,536,004	60,714,103	51,981,253	43,248,402	5,821,901	14,554,751	23,287,602
2032	69,031,227	62,128,104	51,773,420	41,418,736	6,903,123	17,257,807	27,612,491
2033	71,620,026	63,562,773	51,476,894	39,391,014	8,057,253	20,143,132	32,229,012
2034	74,305,909	65,017,671	51,085,313	37,152,955	9,288,238	23,220,596	37,152,954
2035	77,092,519	66,492,297	50,591,965	34,691,633	10,600,222	26,500,554	42,400,886
2036	79,983,631	67,986,086	49,989,769	31,993,452	11,997,545	29,993,862	47,990,179
2037	82,983,165	69,498,401	49,271,254	29,044,108	13,484,764	33,711,911	53,939,057
2038	86,095,187	71,028,529	48,428,543	25,828,556	15,066,658	37,666,644	60,266,631
2039	89,323,916	72,575,682	47,453,330	22,330,979	16,748,234	41,870,586	66,992,937
2040	92,673,728	74,138,983	46,336,864	18,534,746	18,534,745	46,336,864	74,138,982

Source: Authors' estimates based on WHO, UNDP, UN Inter-Agency Task Force data and cost-of-illness modelling

Overall, strengthening mental health interventions can greatly reduce productivity-related losses, yielding significant long-term savings in Uganda's economy. Table 11 presents estimate of Uganda's indirect economic losses arising from mental health disorders between 2024 and 2040, capturing both absenteeism (lost workdays) and presenteeism (reduced productivity while at work). Under the *business-as-usual (BAU)* scenario, indirect costs are expected to rise from *US\$209.4 million in 2024 to US\$377.4 million by 2040*, driven by the increasing prevalence and economic impact of mental health conditions.

In contrast, scaling up interventions to reduce the disease burden could result in substantial economic savings. By 2040, potential savings are estimated at *US\$75.5 million* under a *20% reduction scenario*, *US\$188.7 million* under a *50% reduction*, and *US\$301.9 million* under an *80% reduction*. These findings highlight that expanding access to effective mental health care could significantly reduce productivity losses and improve economic performance over time.

Table 11: Projected Indirect Costs (Absenteeism and Presenteeism) and Potential Savings from Scaled-Up Mental Health Interventions, 2024–2040

Year	Indirect costs under Business as usual (USD)	Indirect costs after Scale-Up Interventions- Reducing burden of disease (USD)			Saving (USD)		
		20 Percent	50 Percent	80 Percent	20 Percent	50 Percent	80 Percent
2024	209,400,000	209,400,000	209,400,000	209,400,000	-	-	-
2025	217,300,000	214,500,000	210,500,000	206,400,000	2,800,000	6,800,000	10,900,000
2026	225,400,000	219,800,000	211,300,000	202,900,000	5,600,000	14,100,000	22,500,000
2027	233,900,000	225,100,000	211,900,000	198,800,000	8,800,000	22,000,000	35,100,000
2028	242,600,000	230,500,000	212,300,000	194,100,000	12,100,000	30,300,000	48,500,000
2029	251,700,000	236,000,000	212,400,000	188,800,000	15,700,000	39,300,000	62,900,000
2030	261,200,000	241,600,000	212,200,000	182,800,000	19,600,000	49,000,000	78,400,000
2031	271,000,000	247,300,000	211,700,000	176,100,000	23,700,000	59,300,000	94,900,000
2032	281,100,000	253,000,000	210,800,000	168,700,000	28,100,000	70,300,000	112,400,000
2033	291,700,000	258,900,000	209,600,000	160,400,000	32,800,000	82,100,000	131,300,000
2034	302,600,000	264,800,000	208,000,000	151,300,000	37,800,000	94,600,000	151,300,000
2035	314,000,000	270,800,000	206,000,000	141,300,000	43,200,000	108,000,000	172,700,000
2036	325,700,000	276,900,000	203,600,000	130,300,000	48,800,000	122,100,000	195,400,000
2037	337,900,000	283,000,000	200,700,000	118,300,000	54,900,000	137,200,000	219,600,000
2038	350,600,000	289,300,000	197,200,000	105,200,000	61,300,000	153,400,000	245,400,000
2039	363,800,000	295,600,000	193,300,000	90,942,793	68,200,000	170,500,000	272,857,207
2040	377,400,000	301,900,000	188,700,000	75,482,653	75,500,000	188,700,000	301,917,347

Source: Authors' estimates based on WHO, UNDP, UN Inter-Agency Task Force data and cost-of-illness modelling

4.5.3. Total Cost of Inaction and potential Savings from scale-up

Investing in mental health interventions offers strong economic returns, significantly reducing Uganda's projected long-term costs of inaction. Table 12 presents and aggregation of human cost, and economic cost (direct and indirect) of mental health disorders in Uganda under the *business-as-usual (BAU)* scenario and compares it with potential outcomes under scaled-up intervention scenarios. The total cost encompassing human cost, direct health expenditures and indirect productivity losses is projected to rise from *US\$ 1.1 billion in 2024* to *US\$ 4.5 billion by 2040* if no additional measures are implemented. Strengthening mental health interventions could, however, generate substantial economic benefits. By 2040, Uganda could save approximately *US\$ 906 million* under a *20% reduction in disease burden*, *US\$ 2.26 billion* under a *50% reduction*, and up to *US\$ 3.62 billion* with an *80% reduction*. These projections reaffirm that investing in mental health care yields significant long-term savings and contributes to broader economic resilience and productivity growth.

Table 12: Projected Total Cost of Inaction and Potential Savings from Scaled-Up Mental Health Interventions in Uganda, 2024–2040

Year	Total costs under Business as usual (USD)	Total costs after Scale-Up Interventions- Reducing burden of disease (USD)			Total Saving (USD)		
		20 Percent	50 Percent	80 Percent	20 Percent	50 Percent	80 Percent
2024	1,102,000,000	1,102,000,000	1,102,000,000	1,102,000,000	-	-	-
2025	1,200,000,000	1,185,000,000	1,162,000,000	1,140,000,000	15,000,000	38,000,000	60,000,000
2026	1,307,000,000	1,274,000,000	1,225,000,000	1,176,000,000	33,000,000	82,000,000	131,000,000
2027	1,424,000,000	1,371,000,000	1,291,000,000	1,211,000,000	53,000,000	133,000,000	213,000,000
2028	1,553,000,000	1,476,000,000	1,359,000,000	1,243,000,000	77,000,000	194,000,000	310,000,000
2029	1,695,000,000	1,589,000,000	1,430,000,000	1,271,000,000	106,000,000	265,000,000	424,000,000
2030	1,849,000,000	1,711,000,000	1,503,000,000	1,295,000,000	138,000,000	346,000,000	554,000,000
2031	2,019,000,000	1,843,000,000	1,578,000,000	1,313,000,000	176,000,000	441,000,000	706,000,000
2032	2,206,000,000	1,985,000,000	1,654,000,000	1,324,000,000	221,000,000	552,000,000	882,000,000
2033	2,411,000,000	2,139,000,000	1,733,000,000	1,326,000,000	272,000,000	678,000,000	1,085,000,000
2034	2,635,000,000	2,306,000,000	1,812,000,000	1,318,000,000	329,000,000	823,000,000	1,317,000,000
2035	2,882,000,000	2,486,000,000	1,891,000,000	1,297,000,000	396,000,000	991,000,000	1,585,000,000
2036	3,153,000,000	2,680,000,000	1,970,000,000	1,261,000,000	473,000,000	1,183,000,000	1,892,000,000
2037	3,450,000,000	2,889,000,000	2,048,000,000	1,207,000,000	561,000,000	1,402,000,000	2,243,000,000
2038	3,776,000,000	3,116,000,000	2,124,000,000	1,133,000,000	660,000,000	1,652,000,000	2,643,000,000
2039	4,135,000,000	3,360,000,000	2,197,000,000	1,034,000,000	775,000,000	1,938,000,000	3,101,000,000
2040	4,529,000,000	3,623,000,000	2,265,000,000	905,800,000	906,000,000	2,264,000,000	3,623,200,000

Source: Authors' estimates based on the cost-of-illness modelling.

4.6. Mental Health Intervention Investment Scenarios

Scenario-based modelling was undertaken to estimate the scale-up required across individual interventions to achieve reductions of 20%, 50%, and 80% in Uganda's *Total Cost of Mental health*. The analysis used baseline clinical and population-based interventions values drawn from the 2022 Ministry of Health, WHO, and UN Interagency Task Force on NCDs report on the prevention and management of mental health conditions in Uganda. The model jointly considers multiple evidence-based clinical interventions across six priority conditions (depression, anxiety, psychosis, bipolar disorder, epilepsy, and alcohol use disorder) reflecting differences in treatment intensity, feasible coverage, and expected impact. In addition, population-based interventions were explicitly incorporated, including a nationwide regulatory ban on highly hazardous pesticides to prevent suicide and the universal delivery of school-based social and emotional learning (SEL) programmes to reduce the incidence of depression, anxiety, and suicide among adolescents. The model quantifies how much expansion in each clinical and population-level intervention would be required to avert the *Total Cost of Mental Health* under each scenario.

Table 13 breakdowns of treatment-based intervention coverage levels required to achieve reductions of 20%, 50%, and 80% in Uganda's total mental health burden. The projections span from 2025 to 2040 and cover a comprehensive suite of clinical and population-based interventions targeting mental, neurological, and substance use disorders (Annex 1). Under the 20% reduction scenario, intervention coverage begins at modest levels but sets the foundation for future scale-up. For anxiety and depression, basic psychosocial treatment for mild depression is projected to reach 7.1% coverage by 2025. Interventions for psychosis, bipolar disorder, and epilepsy also show incremental increases from their respective baselines. Alcohol use and dependence interventions, such as identification and brief interventions, are initiated at lower coverage levels, while withdrawal management and relapse prevention are introduced gradually. Population-based strategies, including the consideration of a nationwide ban on highly hazardous pesticides to address suicide rates and the rollout of school-based social and emotional learning (SEL) programmes, begin with targeted reach. Resource requirements in this scenario include 99 psychiatrists by 2025 and a health funding allocation of 1.3% for mental health.

Table 13: Treatment-Based Interventions considered for reduction of the Total Cost of Mental Health

INTERVENTIONS		Baseline	20%		50%		80%	
			2025	2040	2025	2040	2025	2040
Funding areas	Funding (percentage of total health budget)	1%	1.85%	2.5%	3.2%	4.7%	4.96%	7.6%
Human Resources	No. of Psychiatrists	70	99	216	146	458	205	761
Anxiety Disorders	Basic psychosocial treatment (mild cases)	5.0%	7.5%	17.6%	11.5%	38.4%	16.6%	64.5%
	Basic psychosocial treatment and anti-depressant medication (moderate-severe)	5.0%	7.1%	15.6%	10.5%	33.1%	14.8%	55.0%
	Intensive psychosocial treatment and anti-depressant medication (moderate-severe)	1.0%	4.0%	16.5%	9.0%	42.0%	15.1%	70.0%
Depression	Basic psychosocial treatment for mild depression	5.0%	7.1%	15.6%	10.5%	33.1%	14.8%	55.0%
	Basic psychosocial treatment and anti-depressant medication of first episode	5.0%	6.7%	13.7%	9.5%	28.0%	13.0%	45.9%
	Intensive psychosocial treatment and anti-depressant medication of first episode	5.0%	7.5%	17.9%	11.7%	39.1%	16.9%	65.8%
	Intensive psychosocial treatment and anti-depressant medication of recurrent cases	5.0%	6.6%	13.1%	9.2%	26.5%	12.5%	43.3%
	Intensive psychosocial treatment and anti-	1.0%	3.3%	12.9%	7.2%	32.6%	12.0%	57.3%

	depressant medication of recurrent cases on a maintenance basis							
Psychosis	Basic psychosocial support and anti-psychotic medication	20%	22.0%	30.4%	25.4%	47.5%	29.6%	69.0%
	Intensive psychosocial support and anti-psychotic medication	5.0%	7.8%	19.6%	12.5%	43.6%	18.1%	70.0%
Bipolar Disorder	Basic psychosocial treatment, advice, and follow-up for bipolar disorder, plus mood-stabilizing medication	20%	20.6%	23.2%	21.7%	28.5%	23.0%	35.2%
	Intensive psychosocial intervention for bipolar disorder, plus mood-stabilizing medication	5.0%	5.8%	8.9%	7.0%	15.4%	8.6%	23.5%
Epilepsy	Basic psychosocial support, advice, and follow-up, plus anti-epileptic medication	40%	41.0%	45.1%	42.6%	53.5%	44.7%	64.1%
Alcohol use/dependence	Identification and assessment of new cases of alcohol use/dependence	5.0%	7.0%	15.2%	10.3%	32.1%	14.4%	53.3%
	Brief interventions and follow-up for alcohol use/dependence	5.0%	7.7%	18.9%	12.2%	41.9%	17.8%	70.7%
	Management of alcohol withdrawal	5.0%	8.3%	21.8%	13.7%	49.6%	20.1%	80.0%
	Relapse prevention medication for alcohol use/dependence	1.0%	3.1%	11.8%	6.6%	29.5%	10.9%	51.9%
Preventive Health Interventions	Nation-wide regulatory ban of highly hazardous pesticides to prevent suicide	30%	32.7%	43.9%	37.2%	66.8%	42.8%	95.5%
	Universal school-based social and emotional learning (SEL) interventions to prevent depression/anxiety and suicide in adolescents aged 12-17 years	5.0%	8.0%	20.2%	12.9%	45.4%	19.1%	77.0%

Source: Author's modeling using values from MOH

Note: Mental health care extends well beyond facility-based, medication-focused treatment and requires sustained community, psychosocial, and social support interventions to achieve, prevention, recovery and wellbeing.

The 50% reduction scenario involves significantly scaling up both clinical and population-level interventions. Coverage for basic psychosocial treatment for mild depression increases substantially, alongside broader implementation of intensive psychosocial treatments and medication-assisted interventions. Services for psychosis, bipolar disorder, and epilepsy are

expanded, with coverage percentages rising markedly. Alcohol use interventions, particularly withdrawal management, see notable growth in reach, while relapse prevention becomes more widespread. Population-based measures, such as the pesticide ban and universal school-based SEL programmes, extend to a larger portion of the population. Mental health resources are ramped up, with the number of psychiatrists and funding allocation rising accordingly to meet the demands of enhanced coverage.

The 80% reduction scenario represents the most ambitious scale-up of interventions, aiming for transformative change in Uganda's mental health landscape. Coverage for basic psychosocial treatment for mild depression is projected to reach 55.0% by 2040, with intensive treatments and medication available to the majority of those in need. Interventions for psychosis, bipolar disorder, and epilepsy are delivered at high coverage levels, reflecting a comprehensive approach. Alcohol use and dependence services, such as withdrawal management, are scaled up dramatically, reaching 80.0% coverage by 2040. Population-wide initiatives including the nationwide pesticide ban and universal SEL programmes are fully implemented, ensuring maximum reach. The resources required are substantial, with the number of psychiatrists rising to 761 by 2040 and mental health funding allocation increasing proportionally to support the expanded intervention coverage.

4.7. Conclusions

The substantial and increasing economic and human cost of mental health disorders in Uganda is revealed by this analysis, highlighting the necessity of incorporating mental health into national development and health planning. The results show that mental illness is a serious economic problem that affects long-term growth, household welfare, and productivity in addition to being a medical concern. The findings have a significant impact on Uganda's capacity to meet its Vision 2040 and NDP IV human capital development goals. Some of the analysis's main conclusions are listed below:

- i. **Growing Human Burden:** DALYs increased from 1,583 per 100,000 in 2000 to 1,930 per 100,000 in 2023, indicating a steady increase in the burden of mental health disorders in Uganda. Growing prevalence, restricted access to care, and an increase in psychosocial stressors are all contributing factors to this upward trend. Uganda will experience declining health outcomes and productivity losses in the absence of increased investment in prevention and treatment, undermining advancements in economic growth, labor participation, and education.
- ii. **Age Groups and Disorders Most Affected:** Anxiety and depressive disorders continue to be the main causes, with working-age adults (20–49 years old) bearing the greatest burden. This suggests that conditions are common among people who are at the peak of their economic activity. In other words, Uganda's labor force is disproportionately affected by mental illness, which lowers worker productivity and limits economic competitiveness. This underscores the need for workplace mental health programs and youth-focused interventions.
- iii. **Growing Economic Losses:** Due to both an increase in the prevalence of disease and a n increase in income, the monetary value of DALYs lost increased from UGX 283 billion

in 2000 to UGX 2.96 trillion in 2023. This suggests that the financial stability and productivity of the country are at risk due to the rapidly growing economic impact of mental illnesses. To shield Uganda's expanding economy from unseen productivity drains, proactive mental health investment is essential.

- iv. Direct vs Indirect Costs:** In 2021, direct health spending on mental health was UGX 137.3 billion (US\$ 37.6 million), while indirect productivity losses reached UGX 696.6 billion (US\$ 191 million). Indirect costs are thus five times higher than direct costs. Uganda's limited mental health spending leads to greater productivity losses. Investing in accessible, preventive, and community-based mental health care would yield cost savings by reducing absenteeism and presenteeism.
- v. Cost of Inaction:** If current trends continue, total costs of mental health disorders are projected to increase from US\$ 1.1 billion in 2024 to US\$ 4.5 billion by 2040 under the business-as-usual (BAU) scenario. Maintaining the status quo will result in escalating losses equivalent to several percentage points of GDP, limiting Uganda's ability to meet health and development targets under NDP IV.
- vi. Potential Savings from Scale-Up:** Strengthening interventions could save US\$ 906 million (20% reduction), US\$ 2.26 billion (50%), and US\$ 3.62 billion (80%) by 2040 through reduced DALYs, absenteeism, and healthcare costs. Therefore, scaling up evidence-based mental health interventions offers one of the highest returns on investment in public health, freeing resources for other national priorities and improving population wellbeing.

In conclusion, mental health disorders impose a heavy and growing human and economic burden on Uganda, undermining productivity, household welfare, and national development. The cost of inaction is substantial and will continue to rise if current service gaps persist. Conversely, strategic investments in prevention, community-based services, and early treatment offer significant health and fiscal dividends. Scaling up mental health interventions is therefore not only a public-health imperative but also a sound economic policy essential for safeguarding Uganda's human capital, reducing poverty, and achieving sustainable growth under the Fourth National Development Plan (NDP IV).

CHAPTER 5: SYSTEM READINESS AND HUMAN RESOURCE ANALYSIS

5.1 Introduction

Uganda's population is characterized by a significant proportion of young individuals, with children and adolescents representing over 50% of the total demographic. This youthful demographic bears a substantial mental health burden, as approximately 16% of new cases in mental health outpatient facilities are from this group (Kigozi et al., 2010). However, pervasive mental health issues also affect adult populations, where conditions such as depression, anxiety, and post-traumatic stress disorder are prevalent, largely exacerbated by socio-political challenges, including the aftereffects of conflict and refugee crises (Akol et al., 2015; Ssebunnya et al., 2009). Despite this pressing need for comprehensive mental health services, both child and adolescent mental health, as well as adult mental health, have historically received inadequate attention, often overshadowed by stigma surrounding mental health disorders in Uganda (Hays & Costello, 2022).

The infrastructure for mental health services in Uganda remains critically underfunded and fragmented, with a mere 1% of the national health budget allocated to mental health care (Kigozi et al., 2010). Such underfunding limits the system's readiness to respond effectively to mental health needs and results in significant gaps in accessibility and service quality. Reports indicate that there are only 0.6 mental health specialists per 100,000 people in the public sector, underscoring dire shortages in trained professionals available to meet both child and adult mental health needs, exacerbated by the higher population growth rate and rural inequalities (Nsereko et al., 2011). The challenges of recruitment, training, and retention of skilled mental health personnel are profound, necessitating robust strategies such as task-shifting, where responsibilities are delegated to trained lay health workers, and increased investment in mental health education across all levels of healthcare (Wakida et al., 2018).

Human resource dynamics play a critical role in the mental health landscape. The distribution of healthcare workers shows significant regional disparities, with northern Uganda particularly affected following years of conflict that devastated healthcare infrastructure (Mugisha et al. 2016). In contrast, southern regions may exhibit better service availability and health outcomes. The integration of mental health services into primary healthcare remains suboptimal, partly due to a lack of comprehensive training and adequate resources for general healthcare providers (Wakida et al., 2019). There is an urgent need for improved supervision and monitoring systems, particularly for community health workers, which have previously been identified as critical barriers to effective mental health service delivery (Wakida et al. (2019). Further, the experiences and perspectives of caregivers often bearing significant burdens without adequate support highlight the importance of involving various stakeholders, including traditional healers and community leaders, to create a more holistic approach to mental health care (Verity et al., 2021).

Quality of services and infrastructure readiness must consider not only the physical facilities but also the mental health care delivery framework in Uganda. Efforts to actively involve

community perceptions in mental health planning have shown promise; stakeholder engagement is essential for creating culturally relevant interventions that resonate with the local population (Shah et al., 2017). The integration of technology, such as e-learning platforms for training and awareness campaigns, offers promising avenues to enhance workforce capacity and service delivery (McMullen et al., 2024).

Uganda's mental health system faces formidable challenges linked to infrastructural deficits and workforce inadequacies that hinder effective service delivery for both children and adults. Addressing these challenges mandates a multifaceted approach that combines policy reform, funding increases, and innovative workforce strategies to comprehensively integrate mental health services into the healthcare system. Such reforms are essential for enabling a resilient health system that can adequately respond to the mental health needs of Uganda's diverse population. This paper, therefore, analyses system readiness and human resources for mental health services in Uganda.

5.2 Overview of Uganda's health infrastructure and workforce context

The health infrastructure of Uganda has undergone significant transformations in recent years, particularly in the realm of mental health services. Faced with challenges such as a high burden of mental health disorders and a shortage of trained professionals, the government has initiated various policies aimed at improving access and quality of care.

5.2.1 Structure of Health System

Uganda's mental health system is integrated into a broader public health system that operates on a decentralized model. The service delivery framework includes three primary tiers: community health centres (HC IIs, HCIIIs, and HCIVs); general hospitals; and regional and national referral hospitals. Community health centres serve as the first point of contact for individuals seeking mental health services, providing basic healthcare, including counselling and referral pathways to higher-level facilities when necessary (Kigozi et al., 2010). General, regional and national referral hospitals offer more specialized services and are better equipped to manage complex mental health conditions such as schizophrenia, epilepsy and bipolar disorder.

Despite the effort to decentralize mental health services, significant disparities exist in service availability and quality across different regions. Rural areas often suffer from a lack of essential resources, leading to a heavy reliance on urban facilities, which are more accessible and better staffed. Consequently, individuals in rural regions frequently bypass local facilities to seek help at distant urban centres, exacerbating the burden on these already overstretched services (Zakumumpa et al., 2017). Moreover, the allocation of funding specifically for mental health remain low, with only a small percentage of the health budget dedicated to mental health initiatives (Kigozi et al., 2010).

Mental health problems have various causes, which include drug and substance abuse, psychological or physical abuse, trauma, neglect, social isolation, loneliness, discrimination, stigma, bereavement, severe or long-term stress and prolonged illness.

This therefore requires a multi-sectoral government approach to implement targeted interventions towards the categories of people that are particularly vulnerable to mental ill-health. Notably these comprise of homeless people, ethnic minority groups, prisoners or offenders, people with learning disabilities, refugees and displaced persons, persons living with HIV and AIDS (PLWHA), victims of perpetual gender-based violence, and abused children etc.

Government of Uganda provides for mental health services through its MDAs integrating preventive and promotive targeted specific or sensitive Mental health interventions.

Services for persons with mental ill-health are scanty and mostly comprise of institutionalized medical care provided at Government national and regional referral hospitals. The Ministry of Health provides for Mental health care through the Uganda National Essential Health Care Package (UNEHCP), updated in 2024. The UNEHCP includes mental health as one of its core components, committing the government to providing essential, cost-effective, and universally accessible mental health services. The package's implementation strategy requires the decentralization and integration of basic to complex mental healthcare management into primary healthcare (PHC); (Health Centre III, IV), and tertiary healthcare (RRH and NRH) settings ensuring services are closer to communities.

However, while the Ministry of Health (MoH) takes the lead in clinical management, the Ministry of Gender, Labour and Social Development (MGLSD) focuses on the social determinants of mental health. It implements several interlinked frameworks to provide for preventive mental health by addressing vulnerability, poverty, and social exclusion. A number of these frameworks are in force. Specifically.

- i. National Strategic Programme Plan for Orphans and Vulnerable Children (NSPPI). This Operational Framework for Social Care and Support System operationalize the National Social Protection Policy to prevent mental distress caused by socio-economic shocks. It as well mandates a case management approach at the community and parish levels to identify at-risk individuals and connect them to social supports, while recognizing the family as the "first line of care" and providing caregiver training.
- ii. Child and Adolescent Mental Health (CAMH) Policy Guidelines though a multisectoral approach. These guidelines to drive mental health promotion specifically for young people. It prioritises prevention of disorders, setting directions to prevent mental, neurological, and substance use (MNS) disorders among children. Secondly it emphasises social protection integration linking social protection services with mental health promotion to reduce the impact of violence and abuse on a child's development.
- iii. National Strategic Programme Plan for Orphans and Vulnerable Children (NSPPI). Herein, the ministry implements preventive mental health through the Psychosocial Support (PSS) core program area of the NSPPI.

- iv. Community-Based Frameworks & Communication Strategies such as the Positive Mindset Strategy which is a comprehensive dedicated communication strategy to promote positive mindsets and cultural values among youth, aiming to counter "moral decay" and cultural degeneration that contribute to mental health precursors and risk factors. Furthermore, the GBV Prevention Guidelines address Gender-Based Violence (GBV), a major driver of mental health disorders like depression and PTSD. The MGLSD manages these frameworks to provide psychosocial support to survivors and prevent future occurrences.

5.2.2. Standards for Mental Health management

The standards for mental health (MH) management in Uganda remain heavily skewed to clinical management rather than prevention. Primarily, this is anchored by the Butabika National Referral Hospital and Regional Referral Hospitals that face human and financial resource shortages. While the Uganda Clinical Guidelines 2023, mandate the integration of mental health into primary care. Actual implementation of these guidelines at community hospitals (Health Centre III and IV levels) is inconsistent due to a persistent shortage of specialized human resources trained in psychiatry recruited. Consequently, case management often defaults to a "medical/treatment" approach focused on pharmaceutical methods rather than holistic psychosocial rehabilitation.

A critical challenge to these standards is the increase in establishment of unregulated service providers, particularly in the informal private sector and faith-based "healing centres." Due to the scarcity and high cost of formal psychiatric care, many families resort to unlicensed practitioners who operate outside the oversight of the Uganda Medical and Dental Practitioners Council (UMDPC). These unregulated institutions (entities, cultural/community approaches) and methods are often reported to employ harmful human health practices, including physical restraint and the denial of evidence-based medication, under the guise of spiritual or traditional intervention. This creates a treatment eco-system where the most vulnerable citizens are exposed to human rights abuses in the private sector while the public sector remains underfunded to provide practical, accessible alternatives.

Licensing and auditing private mental health providers, including traditional and faith-based practitioners through the UMDPC would ensure integration and operationalization of the Mental Health Act. Furthermore, the government should shift from a "health facility-treatment" model to a "community-based rehabilitation" framework by scaling empowering community health workers and Village Health Teams (VHTs) to conduct early screening and follow-up on care.

5.2.3 Workforce for mental health

1. Introduction and Context

Uganda's demand for mental health services has risen sharply in recent years, placing growing pressure on an already constrained health system. Reported mental health cases increased from 494,326 in 2021 to 843,295 in 2024, representing a 70.6 percent increase over four years. This trend reflects a combination of rising disease burden, improved detection, and

increased care-seeking behaviour. The scale and distribution of this demand require a workforce planning approach that mirrors how services are delivered in practice, across Government, Private Not-for-Profit (PNFP), and Private-for-Profit (PFP) facilities and across primary, secondary, and tertiary levels of care. Accordingly, this analysis assesses mental health workforce demand and supply by translating approved staffing norms into establishment requirements across the full national service delivery platform. Rather than relying on abstract provider-to-population ratios, the approach grounds workforce needs in facility-level service organizations, enabling direct linkage to recruitment, deployment, budgeting, supervision, and training policies.

2. Manpower Demand Analysis

Mental health workforce demand is estimated by translating approved staffing norms into establishment requirements across the national service delivery platform. Rather than relying on abstract population ratios, this approach grounds demand in the number and type of facilities where mental health services are expected to be delivered, making the results directly relevant to deployment, supervision, budgeting, and recruitment decisions. The analysis covers Health Centre III (HC III), Health Centre IV (HC IV), and General Hospitals across Government, PNFP, and PFP ownership categories, recognising the central role of non-state providers in service delivery. Regional Referral Hospitals (RRHs) and National Referral Hospitals (NRHs) are retained within the Government platform, reflecting their specialized and tertiary care mandate. Approved establishment norms are applied consistently by facility level, with specific adaptations for referral hospitals where standardized national norms are not uniformly defined. Table 14 presents the approved mental health establishments for the combined platform. It provides a system-wide estimate of the number of mental health posts required to operationalise minimum service standards across all levels of care.

Table 14: Approved Mental Health Establishments for the Combined Platform (Government, PNFP, and PFP)

Level	Government	PNFP	PFP	Total Facilities	Establishment Norm	Approved Posts
General Hospitals	57	82	68	207	18 per hospital	3,726
HC III	1,471	363	260	2,094	2 per facility	4,188
HC IV	207	41	35	283	5 per facility	1,415
RRHs (16)	16	–	–	16	Aggregated	69
NRH – Butabika	1	–	–	1	Facility structure	109
Other NRHs (7)	7	–	–	7	22 per NRH (proxy)	154
Total						9,661

Source: Author synthesis of MOH & MoPS data

The establishment profile highlights that mental health service delivery is overwhelmingly anchored at lower-level and general hospital platforms. Health Centre III, Health Centre IV, and General Hospitals together account for 96.6 percent of all approved mental health posts, underscoring the importance of these levels for access, early detection, continuity of care, and follow-up. While referral hospitals account for a smaller share of posts, their role in clinical governance, specialist care, supervision, and training remains indispensable. A critical structural constraint emerges from this demand profile: the national pool of psychiatrists, estimated at about 70, cannot be spread thinly across the entire platform. This makes it neither

feasible nor efficient to base service expansion on psychiatrist deployment alone. Instead, psychiatrists must be positioned strategically within referral networks to provide leadership, manage complex cases, and supervise task-shared care delivered by mid- and lower-level cadres.

3. Manpower Supply Analysis

a. Indicative Staffing Levels and System Capacity

Reliable, comprehensive staffing-in-post data for mental health services are not consistently available across all facility levels and ownership categories. To support planning in the absence of complete data, an indicative filled-versus-gap analysis was undertaken using conservative and transparent assumptions on staffing levels by facility type. These estimates are intended to act as a planning tool rather than a definitive staffing audit and are designed to be updated as routine HRH data systems improve. Table 15 summarizes estimated filled posts and gaps across the combined service delivery platform.

Table 15: Indicative Filled Posts and Gaps for the Combined Platform (Planning Estimate)

Facility Level	Approved Posts	Estimated Filled	Estimated Gap
HC III	4,188	1,466	2,722
HC IV	1,415	566	849
General Hospitals	3,726	2,070	1,656
RRHs	69	46	23
NRH – Butabika	109	30	79
Other NRHs	154	112	42
Total	9,661	4,290	5,371

Source: Author synthesis of MOH & MoPS data

The distribution of gaps shows that shortages are most acute at district and community-facing levels, where mental health services should ideally be initiated and sustained. Staffing deficits at HC III and HC IV undermine early detection, follow-up, adherence support, and psychosocial interventions, often resulting in delayed care and unnecessary referrals to higher-level facilities. These patterns point to a system that is structurally misaligned with the locus of demand.

b. Training Capacity, Enrolment, and Graduate Output

Uganda's ability to close mental health workforce gaps is constrained by limited national training capacity and diminishing interest of trainees, particularly for specialised and mid-level clinical cadres. Training provision is heavily skewed toward lower-level nursing programmes, while advanced clinical and specialist pathways remain narrow. Table 16 summarises national training capacity for selected mental health programmes.

Table 16: National Training Capacity for Selected Mid-level Mental Health Programmes

Programme	Annual Capacity			
	2022	2023	2024	2025
Certificate in Mental Health Nursing	150	148	200	200
Diploma in Mental Health Nursing	100	60	130	280
Diploma in Mental Health Nursing (Extension)	70	100	120	150
Diploma in Clinical Psychiatry	50	-	-	-
Higher Diploma in Clinical Psychiatry	50	-	-	-

Source: Ministry of Education and Sports

While nursing programmes generate the bulk of mental health graduates, enrolment and graduation data show persistently low output for key clinical programmes. In particular, the Diploma in Psychiatric Clinical Medicine remains extremely constrained, producing only one to three graduates annually in recent years. This limits the availability of cadres critical for diagnosis, treatment initiation, and case management at HC IV and hospital levels. Projected graduation trends for 2026–2030 reinforces this pattern. Output from Certificate and Diploma-level mental health nursing programmes increases modestly, while growth in psychiatric clinical and specialist training remains slow. As a result, supply growth is unlikely to keep pace with demand without deliberate expansion of training capacity, faculty, clinical placement infrastructure and incentives to attract more trainees.

Table 17: Annual Graduation rates in Mental health Programs

Programme	2018	2019	2020	2021	2022	2023*	2024*	2025
Certificate in Mental Health Nursing (CMHN)	79	84	169	163	110	146	141	135
Diploma in Mental Health Nursing (DMHN)	49	61	29	71	51	38	36	10
Diploma in Psychiatric Clinical Medicine (Mental Health)	2	2	2	1	2	2	1	
Diploma in Clinical Psychiatry	34	50	51	49	53	59	58	

Source: MoH-Analysis of the Health Labour Market of Uganda 202

4.Manpower Gap for Mental Health Professionals

a. Cadre-Level Workforce Gaps

Linking establishment demand with indicative supply estimates reveals the scale and composition of Uganda’s mental health workforce gap. Table 18 presents estimated gaps by cadre across the combined platform.

Table 18: Estimated Mental Health Workforce Gaps by Cadre

Cadre	Demand	Estimated Filled	Estimated Gap
Psychiatrists	356	70	286
Clinical Psychologists	97	23	74
Psychiatric Clinical Officers	1,026	384	642
Psychiatric Nursing Officers	1,051	402	649
Assistant Nursing Officers (Psychiatry)	3,690	1,346	2,344
Enrolled Nurses & MH Assistants	4,203	1,496	2,707
Counsellors	38	8	30
Social Workers	223	86	137
Medical Officers (Non-specialist)	10	5	5
Other Allied Health	2	0	2

Source: Author synthesis of MOH & MoPS data

The gap profile shows that the largest deficits lie in frontline and mid-level cadres, particularly enrolled nurses, assistant nursing officers, psychiatric nursing officers, and psychiatric clinical officers. These cadres are the backbone of service delivery at HC III, HC IV, and General Hospitals and determine whether care can be provided consistently at scale. Specialist shortages, while smaller in absolute numbers, remain highly consequential for leadership, supervision, and complex case management.

b. Phasing the Workforce Gap, 2026–2030

Given training lead times, fiscal constraints, and absorption capacity, closing the full workforce gap immediately is neither realistic nor sustainable. Instead, the estimated gap of 6,876 posts is phased over the period 2026–2030, prioritising rapid expansion of deployable cadres while scaling specialist cadres more conservatively. Table 19 presents the projected annual absorption and residual gaps by cadre.

Table 19: Projected Mental Health Workforce Gap Reduction by Cadre (2026–2030)

Cadre	Total Gap	Planned Additions (2026–2030)	Residual Gap in 2030
Psychiatrists	286	35	251
Clinical Psychologists	74	34	40
All Other Cadres	6,516	6,516	0
Total	6,876	6,585	291

Source: Author synthesis of MOH & MoPS data

The projections indicate that, if current scale-up plans are implemented, Uganda can largely close gaps in frontline mental health staffing by 2030. Mid- and lower-level cadres are expected to reach full coverage, driven by higher training output and the relative feasibility of expansion. However, specialist cadres remain a binding constraint. Even after planned additions, substantial shortages of psychiatrists and clinical psychologists persist, reflecting long training pipelines and limited intake capacity.

F. Conclusion and Policy Implications

This analysis shows that Uganda’s mental health workforce challenge is large but strategically manageable. Most service delivery gains over the next five years can be achieved by scaling up frontline and mid-level cadres and strengthening supervision, referral, and task-sharing arrangements. At the same time, persistent shortages of specialists highlight the need for a longer-term strategy focused on expanding advanced training capacity, improving retention, and leveraging regional and network-based models of specialist support. Aligning workforce planning with service delivery realities, rather than relying solely on specialist benchmarks, provides a credible pathway for expanding access, improving quality, and responding effectively to Uganda’s rapidly growing mental health needs.

5.2.4 Challenges and Opportunities in Service Delivery

The effective delivery of mental health services in Uganda is hindered by multiple challenges. Chief among them are resource constraints, which include the insufficient availability of medications, essential equipment, and trained personnel, primarily resulting in high levels of unmet need (Palmer et al., 2024). Moreover, heavy workloads and inadequate supervision of the existing workforce further compromise the quality and continuity of care (Mukasa et al., 2019).

Nonetheless, there are opportunities to address these challenges. Advocacy for increased funding to mental health services and the establishment of supportive policies is crucial for building a sustainable mental health system that can cater for the needs of the population. Furthermore, enhancing intersectoral collaboration, including partnerships with traditional

healers, will provide a holistic approach to mental health care that respects cultural practices while promoting effective treatment (Akol et al., 2018). Efforts to train and integrate community health workers into the mental health sphere can extend service reach and promote public awareness.

Uganda's health infrastructure and workforce context for mental health services present a complex array of challenges and opportunities. The decentralization of mental health services, while promising, requires further investments and policy support to ensure equitable access and quality care. By focusing on the training of healthcare personnel, addressing societal stigma, and leveraging innovative solutions, Uganda can work toward a more resilient and effective mental health system that meets the diverse needs of its population.

To establish the level of system readiness, a study was undertaken of various health facilities as detailed below.

5.3 Objectives

- i) To assess System Readiness Using WHO-AIMS and HRH Frameworks
- ii) To map disparities in infrastructure and workforce distribution relative to population needs, highlighting underserved regions and systemic bottlenecks.
- iii) To explore Stakeholder Engagement and Cultural Responsiveness.
- iv) To generate Evidence-Based Recommendations for System Strengthening

5.4 Study Design and Methodology

5.4.1 Study Design

The study adopted a mixed-methods design, combining quantitative surveys and qualitative interviews to assess system readiness and human resource dynamics for mental health in Uganda. This approach enabled a comprehensive analysis of both measurable resource availability and service utilization, as well as the experiences and perceptions of stakeholders within the mental health system (Ssempiira et al., 2018; Iversen et al., 2021).

The quantitative component involved cross-sectional surveys across health facilities at multiple levels Community Health Centres (HC III), Health Centre IVs, General Hospitals, and Regional/National Referral Hospitals spanning Uganda's regions (Northern, Eastern, Central, Western, and South West). The surveys assessed critical factors such as workforce composition, facility readiness, access to and utilization of mental health services, and barriers faced by health workers in service delivery (Aber et al., 2023; Katende et al., 2015).

The qualitative component employed semi-structured interviews and focus group discussions with health managers, community health workers, and mental health specialists. These interactions explored perceived barriers and facilitators to effective service delivery, complementing quantitative findings with contextual insights (Wipfli et al., 2023; Akol et al., 2015). By synthesizing both approaches, the study provided a holistic understanding of Uganda’s mental health infrastructure and workforce challenges (Kiberu et al., 2019).

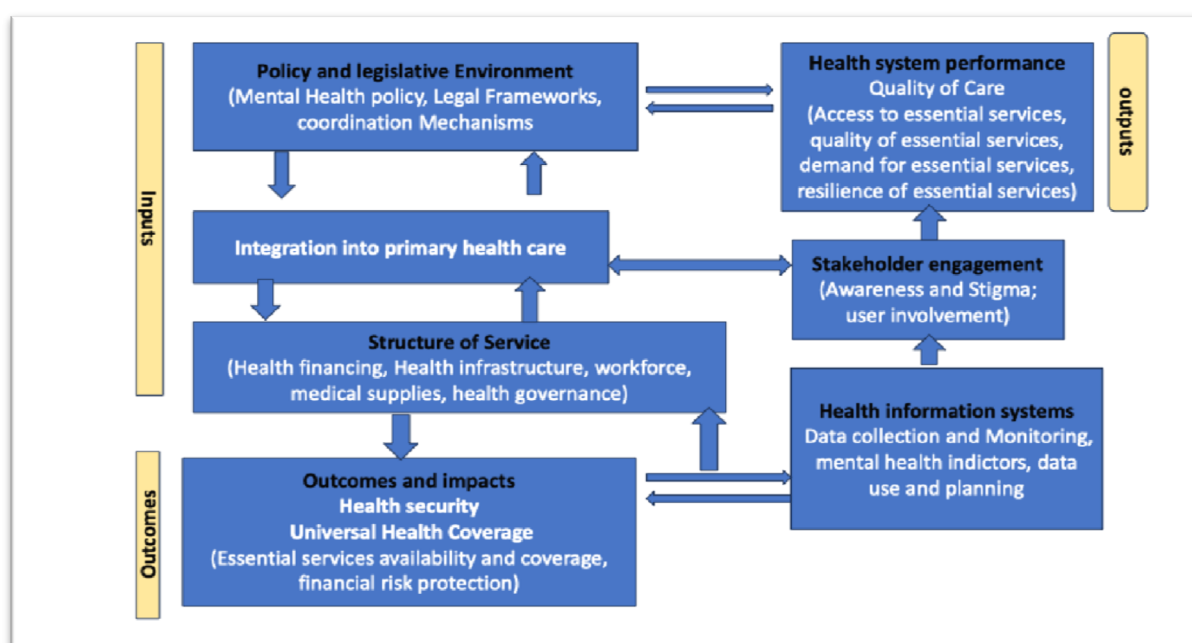
5.4.2 Sampling Framework, Sample Size, and Data Collection

A stratified random sampling method was used to select health facilities across levels and regions, ensuring diversity and reducing selection bias. Facilities were stratified by type (community health centres, general hospitals, referral hospitals) and geography (Northern, Eastern, Central, Western Uganda), with random selection within each stratum. The sample size was calculated using Cochran’s formula for finite populations, targeting approximately 125 facilities to achieve a 95% confidence level and 5% margin of error. Adjustments were made based on the total number of operational health facilities to ensure proportional representation across tiers.

Data collection followed a phased approach. Quantitative surveys captured information on infrastructure readiness, workforce composition, and service delivery practices using validated instruments aligned with the Global Burden of Disease methodologies (Cheng et al., 2025). Qualitative interviews and focus group discussions with facility staff and policymakers explored contextual factors, service delivery barriers, and human resource dynamics. Data were triangulated with secondary sources, including national health statistics and Ministry of Health reports, to validate findings. This mixed-methods strategy provided a robust, multi-dimensional understanding of mental health service readiness and workforce challenges in Uganda (Effatpanah et al., 2024; Umar et al., 2023).

5.4.3 Conceptual Framework

This conceptual framework assesses Uganda’s mental health system readiness using World Health Organisation’s Assessment Instruments for Mental Health Services (WHO-AIMS), integrating infrastructure and workforce determinants. WHO defines a mental health system as *“all structures and activities whose primary purpose is to promote, maintain, or restore mental health and prevent mental disorders”*. Therefore, by applying the WHO Health Systems Readiness and Human Resources for Health (HRH) Capacity frameworks, it offers a structured, multidisciplinary approach to evaluate service delivery, resource allocation, and workforce distribution. It emphasizes the need for robust systems, policy effectiveness, and culturally responsive stakeholder engagement for integrated mental health services.

Figure 17: Conceptual Framework

Source: Author's design based on literature

Integrated mental health services illustrate a potential path forward as in Figure 17, asserting the necessity for a comprehensive approach in service delivery. Accessibility remains a critical challenge, confronted by inadequate mental health facilities and geographical barriers that must be addressed to ensure that care reaches all populations, particularly vulnerable groups (Akol et al., 2017). Quality of care indicators are paramount for assessing the effectiveness of mental health services. Implementing evidence-based practices requires regular monitoring and assessment of organizational readiness, which is crucial for fostering a culture of continuous improvement in care standards (Brown et al., 2010).

5.5 Results and Findings

5.5.1 Participating Facilities

Data was collected from a total of 125 health facilities across Uganda, as presented in Table 20. Participating facilities included health centre IIIs, Health centre IVs, General hospitals, regional referral hospitals and national referral hospitals drawn from public and private domains.

Table 20: Facilities Participating in the Study by Type

Facility type	Total	%	Government/ Public	%	Private For Profit	%	Private Not for Profit	%
General Hospital	13	10.4	3	2.4	6.0	4.8	4.0	3.2
HCIII	73	58.4	56	44.8	11.0	8.8	6.0	4.8
HCIV	27	21.6	21	16.8	4.0	3.2	2.0	1.6
Regional Referral	12	9.6	12	9.6	0.0	0.0	0.0	0.0
	125	100.0	92	73.6	21.0	16.8	12.0	9.6

Source: Primary data

In this study, 58.4% of health facilities were HCIII, 21.6% HCIV, while 10.4% were general hospitals. Regional referral hospitals constituted a smaller proportion (9.6%) of the overall count, showcasing the hierarchical nature of the healthcare system in Uganda.

5.5.2 Existence of a functional Mental Health Unit

The survey revealed that only 36.8% (46 facilities) reported having a functional mental health unit, while 63.2% (79 facilities) of the facilities reported no functional mental health unit. This highlights a critical gap in the infrastructure necessary for providing specialized mental health care. According to WHO standards, a functional mental health unit should comprise a well-staffed facility equipped to provide comprehensive, rights-based mental health services. The lack of dedicated mental health units undermines effective service delivery, as ongoing support and treatment require a consistently structured approach.

5.5.3 Coverage Radius

From the study, the observed coverage radius of facilities indicates that geographic access to mental health services is still a significant structural barrier. According to the field assessment, 73 health facilities (58.4%) served clients who had to travel more than 15 kilometers to access the service as shown in Table 21.

Only twelve percent (12.0%) of facilities (15 facilities) serve a population within a radius of 0-5 km, 10.4% (13 facilities) 6-10 km while 19.2% (24 facilities) and 11-15 km to access mental health services. This shows that less than 25% of facilities served clients within 10 km highlighting a clear mismatch between community need and service location.

Table 21: Core Functional aspects of Mental health Units

Variable	Frequency	Percentage
Functional Mental health Unit		
Yes	46	36.8
No	79	63.2
Coverage (Radius)		
0-5km	15	12
6-10km	13	10.4
11-15km	24	19.2
More than 15km	73	58.4
Type of care provided		
Inpatient and outpatient services	36	28.8
outpatient services	89	71.2
Rehabilitation services		
Yes	7	5.6
No	118	94.4
Availability of a medicine's procurement plan		
No	38	30.4
Drug stockout in the last 12 months		
Yes	72	57.6
No	53	42.4
Availability of essential medicines and commodities for mental health		
Yes	95	76
No	30	24

Infrastructure		
External environment and service delivery areas fenced		
No	29	23.2
Yes	86	68.8
Partially	10	8
Reliable and safe water supply		
Yes	90	72
NO	35	28
Reliable power supply		
Yes	103	82.4
No	22	17.6
Referrals		
Presence of a community referral pathway		
Yes	106	84.8
No	19	15.2
Functional Patient Referral pathway		
Yes	109	87.2
No	16	12.8
Functional Mental Health Emergency Unit		
Yes	20	16
No	105	84
Qualified mental health worker present at all times		
Yes	50	40
No	75	60
Existence of a capacity building plan for Mental health		
Yes	47	37.6
No	78	62.4
Standard operating procedures for managing mental health conditions		
Yes	53	42.4
No	72	57.6

Source: Primary data

5.5.4 Services Provided

The range and depth of mental health services offered across surveyed facilities reveal a system that remains narrowly treatment-oriented and weak on continuity of care. While majority of facilities (71.2%) provide only outpatient services and 28.8% offer a combination of inpatient and outpatient care, service provision is heavily skewed toward acute and episodic treatment rather than comprehensive mental health management.

Most notably, rehabilitation services are almost absent, with only 5.6% (7 facilities) reporting the availability of rehabilitation. This represents a critical gap in the mental health care continuum. Rehabilitation services are essential for recovery, reintegration, and long-term functioning, particularly for individuals with severe, chronic, or recurrent mental health conditions.

The near absence of rehabilitation capacity implies that patients are treated for symptoms but left without structured support for recovery, relapse prevention, and social reintegration. This service configuration reinforces a cycle of repeated illness, frequent relapse, and avoidable readmissions, placing sustained pressure on families, communities, and the health system.

Overall, the current service mix reflects a system that prioritizes short-term clinical contact over long-term mental health outcomes.

5.5.5 Medicines Procurement Plans and Drug Stockouts

The availability of plans for the procurement of medications paints a conflicting picture of systemic vulnerability and readiness. A significant 30.4% (38 facilities) function without any official plan, despite the fact that 69.6% (87 facilities) reported having a procurement plan for mental health medications. This implies that facilities are vulnerable to predictable disruptions in the availability of medications due to the lack of organized planning.

Drug shortages continue to be a serious issue. In the past year, stockouts of necessary psychotropic medications occurred in more than half of the facilities (57.6%), jeopardizing clinical efficacy and treatment continuity. The high frequency of recent stockouts suggests that availability is irregular rather than consistent, even though 76.0% of facilities reported having essential medicines available.

5.5.6 Infrastructure and Utilities

Infrastructure assessment reveals significant deficiencies, particularly regarding reliable utility services. At least 68.8% have fenced their environments, which is important for patient safety and treatment compliance. Furthermore, while 82.4% reported a reliable power supply, only 72% had a dependable water supply. These infrastructural elements are essential for creating therapeutic environments conducive to recovery.

5.5.7 Community and Patient Referral Pathways

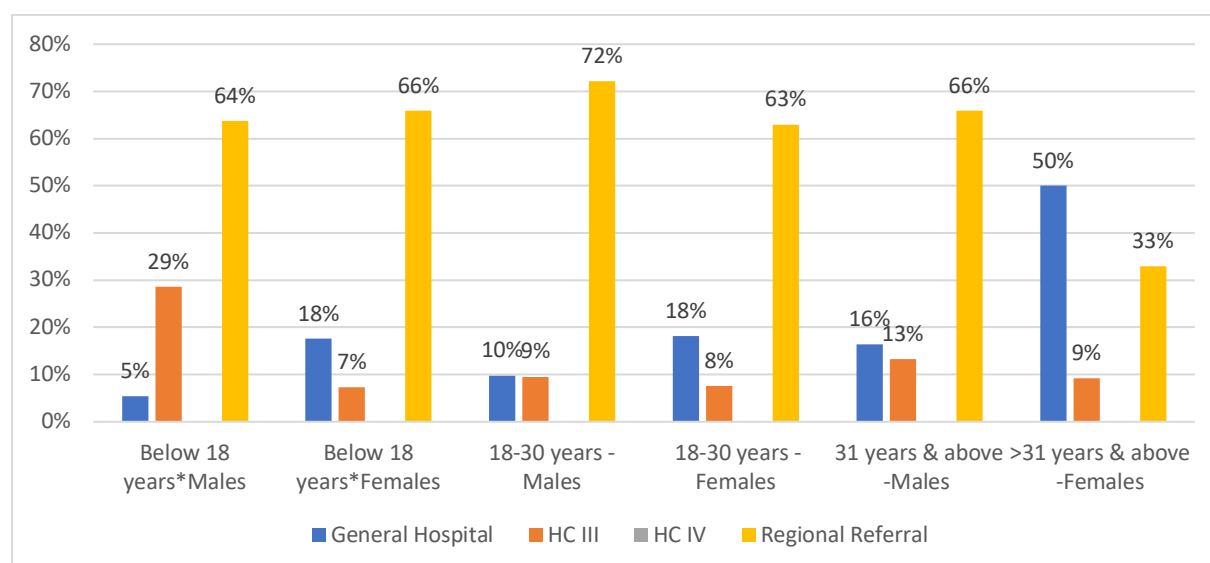
Approximately 85% (106 facilities) had community referral pathway. This indicates some functionality in linking patients to necessary services within the broader community. Additionally, 87.2% (109 facilities) of the facilities reported the existence of a functional patient referral pathway, which is fundamental for ensuring that individuals receive continuous care and follow-up after initial treatment.

5.5.8 Functional Emergency Units

The findings revealed that only 16.0% (20 facilities) had functional emergency units. This situation poses significant risks to patients experiencing acute mental health crisis, as timely and appropriate emergency interventions are critical to preventing adverse outcomes.

5.5.9. Service Utilization

Figure 18: Service utilization by facility and age group



Source: Primary data

Figure 18 shows that mental health service utilisation is strongly concentrated at Regional Referral Hospitals for all age groups. The highest utilisation in regional referral hospitals is among males aged 18-30 years (72%), followed by children below 18 years at 64% for males and 66% for females. Use of General Hospitals remains modest, ranging from 5% among boys below 18 years to a peak of 50% among females aged 31 years and above, the only group where mid-level facilities dominate. HC III utilisation is generally low, varying between 7-29%, with the highest use observed among boys below 18 years (29%). HC IV utilisation is minimal across all categories, rarely exceeding 10%, indicating limited use of intermediate referral services. Overall, these percentages highlight a system heavily skewed toward higher-level facilities, reflecting gaps in primary and secondary service delivery.

Table 22: Percent of Mental health disorders by sex and age group in 2024

Mental Health Disorder	Sex	Below 18 years	18 -30 years	31 years and above
Drug abuse	male	71.4	89.2	74.4
	female	28.6	10.8	25.6
	Total	100.0	100.0	100.0
Alcohol abuse	male	36.1	79.5	59.9
	female	63.9	20.5	40.1
	Total	100.0	100.0	100.0
Schizophrenia, schizotypal and delusional disorders	male	45.8	64.5	59.4
	female	54.2	35.5	40.6
	Total	100.0	100.0	100.0
Mood (affective) disorder	male	37.9	55.2	48.9
	female	62.1	44.8	51.1
	Total	100.0	100.0	100.0
Neurotic, stress-related and somatoform disorders	male	55.1	82.7	74.0
	female	44.9	17.3	26.0
	Total	100.0	100.0	100.0
	male	64.8	68.6	68.9

Disorders of adult personality and behaviour	female	35.2	31.4	31.1
	Total	100.0	100.0	100.0
Other (epilepsy, organic mental disorders, mental retardation, behavioral and emotional disorder with onset usually occurring in childhood)	male	52.3	54.3	55.6
	female	47.7	45.7	44.4
	Total	100.0	100.0	100.0

Source: Primary data

Table 22 shows clear and consistent sex and age differentials in mental health disorders. Substance-related conditions are overwhelmingly male-dominated, particularly among young adults aged 18–30 years, where drug abuse (89.2%) and alcohol abuse (79.5%) are highest among males, highlighting a critical vulnerability in this age group. In contrast, females below 18 years bear a larger share of alcohol abuse (63.9%), mood (affective) disorders (62.1%), and schizophrenia-related disorders (54.2%), suggesting early-onset mental health risks among girls. Across most disorder categories, males dominate in adulthood, especially for neurotic and stress-related disorders, where male prevalence reaches 82.7% in ages 18–30 and 74.0% among those 31 years and above. Disorders of adult personality and behaviour consistently affect more males across all age groups at about 65–69%. The findings point to early emotional and affective vulnerability among females, contrasted with high-risk behavioral and substance-related disorders among males, particularly in young adulthood, underscoring the need for age- and sex-responsive mental health prevention and care strategies.

5.6 Discussion of results

5.6.1 Health Facility Landscape and Coverage

Uganda's health system is structured within a decentralized framework that includes various levels of service delivery, such as Health Centre IIIs, Health Centre IVs, General Hospitals, and Regional Referral Hospitals. The available data indicate a predominance of public facilities providing mental health services, while private for-profit and private not-for-profit institutions also play a contribution role. Many of these facilities serve populations that are located more than 15 kilometers away, creating serious access barriers, particularly for rural and underserved communities.

Such spatial inequities undermine the aspirations outlined in Uganda's Vision 2040 regarding universal health coverage (UHC) as well as the National Development Plan IV (NDP IV), which emphasizes inclusive service delivery. To bridge these access gaps, there is a need for strategic investments in transportation infrastructure, mobile outreach initiatives, and community-based mental health platforms. Geographic expansion must be coupled with capacity building at lower-level facilities to ensure that mental health services are effectively utilized.

5.6.2 Functionality of Mental Health Units

The presence of functional mental health units across Uganda's health facilities remains limited. Outpatient services are more commonly reported, while inpatient capacity tends to be

under-resourced. The irregularity of mental health clinic days and the weak integration of mental health services into routine operations reflect a systemic challenge, wherein mental health is often perceived as peripheral rather than a core component of primary healthcare. NDP IV advocates for an integrated, people-centred health system that institutionalizes mental health within facility structures. These efforts must be supported by clear operational guidelines, dedicated budgets, and performance monitoring mechanisms. Effective institutionalization will require a change in perception regarding mental health among both providers and policymakers, aligning more closely with the objectives of Vision 2040.

5.6.3 Service Availability and Integration

The data reveal a diverse but uneven provision of mental health services. While facilities may report offering psychosocial support, patient identification, referrals, integrated counselling, and community outreach, critical services such as suicide prevention, rehabilitation, and substance abuse treatment are often inconsistently available. Access to these services frequently relies on informal channels like walk-ins and family referrals, with structured referral pathways being largely absent. This lack of standardization hinders continuity of care and limits the potential for early intervention. To align with NDP IV's health system strengthening agenda, Uganda requires a comprehensive minimum service package for mental health applicable at all facility levels, including preventive, promotive, curative, and rehabilitative services, along with clear protocols for referral, follow-up, and community engagement strategies.

5.6.4 Infrastructure Utilities and Human Resources for Mental Health

Infrastructure deficits are among the most pressing challenges identified in this dataset. Many facilities lack essential elements such as reliable water and power supplies, adequate administrative and mental health ward space. The absence of operational emergency units or functional referral systems leaves mental health bed capacity minimal, which compromises the ability to manage acute cases or provide quality inpatient care. Addressing these deficiencies requires a multi-faceted approach that incorporates capital investments in facility upgrades and the integration of mental health infrastructure into national health facility standards.

The capacity of human resources for mental health in Uganda is critically low. Few facilities reports having specialized mental health personnel such as psychiatrists, psychologists, and psychiatric nurses. A majority of facilities rely on general health workers who usually have limited mental health training. This further aggravated by the limited staffing levels for mental health professionals compared to the stipulated and approved staff structure Table 18 in the annex of the report. The qualifications within the workforce are often skewed towards those with lower academic credentials. Moreover, many mental health practitioners work part-time or hold multiple roles, which can impair service quality, limit scalability, and contribute to staff burnout. Aligning with NDP IV's focus on human capital development, Uganda must invest in mental health training pathways, provide incentives for rural deployment, and integrate mental health competencies into both pre-service and in-service training curriculums.

5.6.5 Mental Health Medicines, Commodities, referrals and emergency services

The availability of essential psychotropic medications in Uganda is inconsistent and unreliable. Facilities often report shortages of essential medications, which obstruct access and significantly limit the capacity for treatment. Additionally, mental health medications can be expensive for individuals to obtain on their own, as illustrated in Table 21. Strengthening procurement systems and ensuring regular supply through effective management of both centralized and decentralized channels are urgent actions required to meet NDP IV's health commodity security objectives.

The functionality of mental health referral systems is inadequate, particularly in rural regions where emergency services are scarce, leading to fragmented care. This situation compromises the overall health system's effectiveness as patients face delays in accessing necessary care. To address these shortcomings, Uganda must develop standard operational protocols for mental health referrals and enhance emergency response systems, ensuring that mental health is recognized as a critical component of the overall healthcare framework (Mugisha et al., 2019). Training community health workers to identify early signs of mental illness and facilitate prompt referrals is essential for improving mental healthcare access.

5.6.6 Financing and Budgeting

The financial landscape for mental health care in Uganda is characterized by fragmentation and lack of a standalone budget for mental health. Only 1.6% of facilities report having dedicated budgets for mental health, often resulting in inadequate funding, visibility and accountability (Petersen et al., 2019). The current scenario sees mental health allocations absorbed into broader health expenditure, reflecting systemic neglect that undermines effective strategic planning (Kigozi et al., 2010). NDP IV advocates for results-based financing and improved resource mobilization for health, calling for the institutionalization of mental health budgeting at both national and district levels and linking these budgets to service delivery outcomes while exploring innovative funding mechanisms.

5.6.7 Service Utilization and Data Systems

Although some data on mental health service utilization is available, it often lacks consistency and completeness. Limited age-disaggregated data suggests higher service utilization among adults, with insufficient services targeted at paediatric populations (Abayneh et al., 2017). Inconsistencies in data collection and reporting impede the potential for informed planning and accountability. Strengthening the Health Management Information System (HMIS) to include comprehensive mental health indicators is crucial for effective data-driven decision-making (Brown et al., 2022). This effort aligns with NDP IV's emphasis on establishing robust monitoring and evaluation frameworks to enhance service delivery.

5.7 Conclusion

Evidence indicates that without deliberate policy interventions addressing infrastructure deficits, workforce shortages, service integration gaps, and financing constraints, Uganda's mental health system will continue to underperform. Implementing these recommendations will strengthen system readiness, improve access and quality of care, and align national mental health services with the aspirations of UHC and the Sustainable Development Goals (SDGs).



CHAPTER 6: CONCLUSIONS AND RECOMMENDATIONS

The findings of SUPRE 2025 demonstrate that mental health in Uganda has reached a critical inflection point. Rising prevalence of mental illness, escalating economic costs, deepening socio-economic drivers, and persistent system readiness gaps collectively reveal a widening mismatch between mental health needs and the country's capacity to respond. Mental illness is no longer a peripheral health concern but a structural constraint on human capital development, productivity, social cohesion, and Uganda's ability to harness its demographic potential under NDP IV and the Ten-Fold Growth Strategy.

The analysis shows that mental health risks are concentrated along the life course and are strongly shaped by poverty, unemployment, education stress, urbanization, gender-based violence, and substance abuse, while service delivery remains constrained by workforce shortages, weak community-level response, limited integration into routine care, and over-reliance on specialized facilities. At the same time, the cost-of-inaction evidence confirms that failure to act will result in mounting productivity losses and long-term economic harm that far exceed the cost of strategic investment in prevention and early care.

Taken together, these findings point to the urgent need for a paradigm shift from a facility-centred, specialist-dependent model to an integrated, community-anchored, and system-wide response that embeds mental health across primary health care, social services, education, labour, and community systems. The recommendations that follow therefore focus on a limited set of high-impact, feasible actions that directly address the structural weaknesses identified in the analysis and offer a pathway for strengthening mental health as a foundation for Uganda's human capital development and sustainable socio-economic transformation.

1. Address the socio-economic drivers of mental illness through integrated social and economic interventions, while strengthening the evidence base to inform design and implementation.

Preliminary analysis in SUPRE 2025 indicates that a range of socio-economic factors, including poverty, unemployment, education-related stress, food insecurity, substance abuse, and household instability, are significantly associated with mental health outcomes. However, the depth of analysis was constrained by limited facility-based data and the absence of detailed case-level information capturing a wider set of socio-economic risk factors. There is therefore a need for follow-up studies and implementation-focused research, using more comprehensive facility data and qualitative case studies, to better understand pathways of risk and inform the targeting, sequencing, and effectiveness of interventions.

Notwithstanding these limitations, Government, through the Ministry of Gender, Labour and Social Development working with the Ministry of Finance, the Ministry of Education and Sports, the Ministry of Local Government, and Local Governments, should embed psychosocial support, life-skills development, and clear referral pathways within livelihood, skills development, and youth employment programmes, particularly for unemployed youth, NEET populations, and households experiencing chronic poverty and food insecurity. The education system should strengthen school-based mental health promotion, guidance and counselling, and early identification of psychosocial distress linked to academic

pressure, family instability, and adolescent development. At community level, substance-use prevention and harm-reduction initiatives should be scaled up through Local Governments, Village Health Teams, and civil society actors, supported by regulation of alcohol and drug availability and sustained community awareness efforts.

2. Develop nationwide community based mental health care systems to improve access to and availability of mental health services country wide

Government, through the Ministry of Health in collaboration with Local Governments, should position Village Health Teams (VHTs) and Community Health Extension Workers (CHEWs) as the backbone of community-based mental health care through targeted financing, skills development, and integration into the formal health system. This will enable early identification of individuals at risk of mental disorders, referral to appropriate health facilities, and guided follow-up at household level. Community mental health screening, referral tracking, and follow-up should be formally embedded in CHEW/VHT guidelines, reporting tools, and the routine health information system, allowing parish- and sub-county-level monitoring of emerging mental health trends and timely response. This task-sharing approach should be linked to and supported by health facilities through mentorship, referral feedback, technical supervision and integration with primary health care services to ensure continuity of care.

3. Strengthen integration of mental health in health care at all levels by institutionalizing a minimum, integrated mental health service package across both public and private health facilities and ensuring facility readiness for such care.

In order to ensure health facility readiness, the Ministry of Health, working with District Local Governments, referral hospitals, and private health sector regulators, should ensure availability of a) integrated mental health screening, b) uninterrupted availability of essential mental health medicines and supplies, c) basic treatment, and referral, as well as d) standard operating procedures for care and treatment; emergency and crisis-response; and number and cadre of trained staff. Capacitating private facilities will expand access points for early identification, care and referral, particularly in urban and peri-urban areas where they are often the first point of contact. Integration of mental health into routine care will reduce stigma by normalizing help-seeking and avoid isolation of services. Compliance should be reinforced through routine supervision, licensing and accreditation processes, performance reviews, and quality improvement mechanisms, supported by targeted capacity-building for facilities that fall below minimum standards. Health training institutions should ensure mental health is embedded in health training curricular to ensure preservice training for health cadre

4. Strengthen mental health human resources through deliberate and targeted training and recruitment and decentralized competency building

To address critical mental health workforce shortages, Referral Hospitals and districts should be mandated to recruit mental health professional including counsellors, psychologists and social workers into the health system as per the existing revised staffing structures by health facility level. A specialist-led model to support a tiered, competency-based task-sharing approach that strengthens the capacity of non-specialist health workers

to provide mental health care while redefining the role(s) of mental health specialists needs to be implemented. Psychiatrists should be repositioned as regional level mentors and supervisors, providing outreach clinics, tele-mentoring, and rotational support rather than remaining concentrated at central facilities.

As per the mental Health act (2018) Ministry of public service should create positions for and Ministry of Health and Local Governments should recruit district mental health focal persons to coordinate mental health programs and services in all districts.

The Ministry of Health, working with the ministry of Education and Sports, training institutions, Local Governments, and professional councils, should adopt a tiered mental health workforce strategy that prioritizes; a) training in relevant knowledge and skills in mental health care for all health worker training institutions, b) in-service training courses that target non specialist health workforce aimed at task-sharing and decentralized service delivery. Given the severe shortage of specialists, mental health screening, basic treatment, and follow-up should be systematically integrated into the roles of all health workers including general medical officers, clinical officers, nurses, midwives, and community health workers as guided by the minimum integrated mental health service package. To reverse low uptake in mental health specialist training programmes, Government should introduce targeted incentives such as bonded scholarships.

5. Reposition Butabika Hospital as a national centre of excellence, training, and referral support rather than a congested long-stay facility

Government, through the Ministry of Health and the Health Service Commission, should prioritise targeted investment and processes to reposition Butabika National Referral Mental Hospital to become a National Institute of Mental Health whose role will include: providing highly specialized mental health care; training of specialist and non-specialist health workers; designing and implementation of mental health related research and technical support and mentoring to regional and district health facilities.

Government, through the Ministry of Health and the Health Service Commission should prioritize targeted investment to address critical staffing gaps, infrastructure deficits, and service quality constraints, while deliberately reducing avoidable excessive patient admissions through strengthening the referral systems.

Immediate actions include filling approved specialist and nursing positions to enhance the level of specialist care and improve unsafe staff-to-patient ratios, stabilizing funding for essential medicines and equipment maintenance, and completing critical infrastructure projects such as secure perimeter works and modern records systems.

6. Close policy, legal, and regulatory gaps to strengthen mental health governance, patient safety, and accountability across all service providers

Government should urgently operationalise the Mental Health (Amendment) Act through clear subsidiary regulations, enforceable standards, and inspection mechanisms that apply to all mental health service providers, including public facilities, private medical centres, faith-based institutions, and traditional and informal care providers. Regulatory

frameworks should explicitly prohibit the operation of unlicensed detention, remand, correction, or isolation centres masquerading as treatment facilities, and strengthen protections against abuse, unlawful confinement, inhumane treatment, and violations of patient rights.

The Ministry of Health, working with Local Governments, regulatory councils, and law enforcement agencies, should establish mandatory licensing, registration, routine inspection, and sanctions for non-compliance, alongside clear referral and reporting obligations for private and informal providers. Mental health responsibilities should be embedded within binding sectoral regulatory frameworks

7. Establish and regulate a national digital mental health back-borne as complementary entry points for early assessment, guidance, and referral

Government, through the Ministry of Health and relevant regulatory bodies, should integrate accredited digital mental health platforms into the formal mental health system as complementary access points for early screening, self-referral, guidance, and crisis support. Well-governed digital platforms can reduce stigma-related barriers to help-seeking, particularly among adolescents, youth, and urban populations, while extending reach beyond facility-based services. Clear standards should be established for governance, clinical oversight, data protection, interoperability with national health information systems, and referral linkages to public and private health facilities, ensuring quality, safety, and accountability.

8. Leverage private-sector and diaspora-led initiatives to expand specialised mental health and addiction care

Government should harness accredited private-sector and diaspora-led mental health initiatives to expand specialised capacity in areas such as substance-use treatment, rehabilitation, counselling, and stepped care, through structured public-private partnerships. Regulatory facilitation, contractual arrangements, and targeted fiscal incentives should be aligned with national mental health priorities to complement public services, reduce pressure on referral hospitals, and improve access to specialised care where public capacity remains limited.

9. Strengthen mental health data systems to support evidence-based policy, planning, and investment

Mental health indicators should be systematically integrated into routine health, education, and social protection information systems to enable timely surveillance, service planning, and accountability. Government should prioritise standardised reporting, facility and community-level data capture, and interoperability across systems, while investing in periodic surveys and research to address critical data gaps on prevalence, socio-economic determinants, service utilisation, and outcomes.



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Annex

Annex 1: Interventions considered for 20 Percent reduction of the mental health burden

Intervention coverage for mental, neurological, and substance use disorders		2025	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035	2036	2037	2038	2039	2040
Anxiety disorders	Basic psychosocial treatment (mild cases)	7.5%	8.1%	8.8%	9.5%	10.2%	10.9%	11.7%	12.4%	13.1%	13.8%	14.5%	15.1%	15.8%	16.4%	17.0%	17.6%
	Basic psychosocial treatment and anti-depressant medication (moderate-severe)	7.1%	7.6%	8.2%	8.8%	9.4%	10.0%	10.6%	11.2%	11.8%	12.4%	13.0%	13.5%	14.1%	14.6%	15.1%	15.6%
	Intensive psychosocial treatment and anti-depressant medication (moderate-severe)	4.0%	4.8%	5.7%	6.6%	7.4%	8.3%	9.2%	10.1%	10.9%	11.8%	12.6%	13.5%	14.3%	15.0%	15.8%	16.5%
	Basic psychosocial treatment for mild depression	7.1%	7.6%	8.2%	8.8%	9.4%	10.0%	10.6%	11.2%	11.8%	12.4%	13.0%	13.5%	14.1%	14.6%	15.1%	15.6%
Depression	Basic psychosocial treatment and anti-depressant medication of first episode	6.7%	7.2%	7.6%	8.1%	8.6%	9.1%	9.6%	10.1%	10.6%	11.0%	11.5%	12.0%	12.4%	12.9%	13.3%	13.7%
	Intensive psychosocial treatment and anti-depressant medication of first episode	7.5%	8.2%	8.9%	9.6%	10.3%	11.1%	11.8%	12.5%	13.3%	14.0%	14.7%	15.4%	16.0%	16.7%	17.3%	17.9%
	Intensive psychosocial treatment and anti-depressant medication of recurrent cases	6.6%	7.0%	7.5%	7.9%	8.4%	8.8%	9.3%	9.7%	10.2%	10.7%	11.1%	11.5%	11.9%	12.3%	12.7%	13.1%
	Intensive psychosocial treatment and anti-depressant medication of recurrent cases on a maintenance basis	3.3%	4.0%	4.6%	5.3%	5.9%	6.6%	7.3%	8.0%	8.7%	9.3%	10.0%	10.6%	11.2%	11.8%	12.4%	12.9%
Psychosis	Basic psychosocial support and anti-psychotic medication	22.0%	22.6%	23.1%	23.7%	24.3%	24.9%	25.5%	26.1%	26.7%	27.2%	27.8%	28.4%	28.9%	29.4%	29.9%	30.4%
	Intensive psychosocial support and anti-psychotic medication	7.8%	8.6%	9.4%	10.2%	11.1%	11.9%	12.7%	13.5%	14.4%	15.2%	16.0%	16.7%	17.5%	18.2%	18.9%	19.6%
Bipolar disorder	Basic psychosocial treatment, advice, and follow-up for bipolar disorder, plus mood-stabilizing medication	20.6%	20.8%	21.0%	21.2%	21.3%	21.5%	21.7%	21.9%	22.1%	22.2%	22.4%	22.6%	22.8%	22.9%	23.1%	23.2%
	Intensive psychosocial intervention for bipolar disorder, plus mood-stabilizing medication	5.8%	6.0%	6.2%	6.4%	6.6%	6.8%	7.1%	7.3%	7.5%	7.7%	7.9%	8.2%	8.4%	8.5%	8.7%	8.9%
Epilepsy	Basic psychosocial support, advice, and follow-up, plus anti-epileptic medication	41.0%	41.3%	41.5%	41.8%	42.1%	42.4%	42.7%	43.0%	43.3%	43.6%	43.8%	44.1%	44.4%	44.6%	44.9%	45.1%
Alcohol use/dependence	Identification and assessment of new cases of alcohol use/dependence	7.0%	7.5%	8.1%	8.7%	9.2%	9.8%	10.4%	11.0%	11.6%	12.1%	12.7%	13.2%	13.8%	14.3%	14.8%	15.2%
	Brief interventions and follow-up for alcohol use/dependence	7.7%	8.5%	9.2%	10.0%	10.8%	11.6%	12.4%	13.1%	13.9%	14.7%	15.5%	16.2%	16.9%	17.6%	18.3%	18.9%
	Management of alcohol withdrawal	8.3%	9.2%	10.1%	11.0%	12.0%	12.9%	13.9%	14.9%	15.8%	16.7%	17.7%	18.5%	19.4%	20.3%	21.1%	21.8%

	Relapse prevention medication for alcohol use/dependence	3.1%	3.7%	4.3%	4.9%	5.5%	6.1%	6.7%	7.3%	7.9%	8.5%	9.1%	9.7%	10.2%	10.8%	11.3%	11.8%
Population-based mental health interventions	Nation-wide regulatory ban of highly hazardous pesticides to prevent suicide	32.7%	33.4%	34.2%	35.0%	35.8%	36.5%	37.3%	38.1%	38.9%	39.7%	40.4%	41.2%	41.9%	42.6%	43.2%	43.9%
	Universal school-based social and emotional learning (SEL) interventions to prevent depression/anxiety and suicide in adolescents aged 12-17 years	8.0%	8.8%	9.6%	10.5%	11.3%	12.2%	13.1%	13.9%	14.8%	15.6%	16.5%	17.3%	18.1%	18.8%	19.5%	20.2%
Psychiatrists		99	106	114	122	131	139	147	156	164	172	180	188	195	203	210	216
Funding (1% of total health budget)		1.3%	1.3%	1.4%	1.5%	1.6%	1.7%	1.7%	1.8%	1.9%	2.0%	2.0%	2.1%	2.2%	2.3%	2.3%	2.4%

Annex 2: Interventions considered for 50 Percent reduction of the mental health burden

Intervention coverage for mental, neurological, and substance use disorders		2025	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035	2036	2037	2038	2039	2040
Anxiety disorders	Basic psychosocial treatment (mild cases)	11.5%	13.3%	15.1%	17.0%	18.9%	20.8%	22.7%	24.5%	26.4%	28.3%	30.1%	31.9%	33.6%	35.3%	36.9%	38.4%
	Basic psychosocial treatment and anti-depressant medication (moderate-severe)	10.5%	12.0%	13.5%	15.1%	16.6%	18.2%	19.8%	21.4%	23.0%	24.6%	26.1%	27.6%	29.0%	30.4%	31.8%	33.1%
	Intensive psychosocial treatment and anti-depressant medication (moderate-severe)	9.0%	11.2%	13.4%	15.7%	18.0%	20.3%	22.7%	25.0%	27.3%	29.6%	31.8%	34.0%	36.1%	38.2%	40.1%	42.0%
	Basic psychosocial treatment for mild depression	10.5%	12.0%	13.5%	15.1%	16.6%	18.2%	19.8%	21.4%	23.0%	24.6%	26.1%	27.6%	29.0%	30.4%	31.8%	33.1%
Depression	Basic psychosocial treatment and anti-depressant medication of first episode	9.5%	10.7%	12.0%	13.2%	14.5%	15.8%	17.1%	18.4%	19.7%	21.0%	22.3%	23.5%	24.7%	25.8%	26.9%	28.0%
	Intensive psychosocial treatment and anti-depressant medication of first episode	11.7%	13.5%	15.3%	17.2%	19.2%	21.1%	23.0%	25.0%	26.9%	28.8%	30.7%	32.5%	34.2%	35.9%	37.6%	39.1%
	Intensive psychosocial treatment and anti-depressant medication of recurrent cases	9.2%	10.3%	11.5%	12.7%	13.9%	15.1%	16.3%	17.6%	18.8%	20.0%	21.1%	22.3%	23.4%	24.5%	25.5%	26.5%
	Intensive psychosocial treatment and anti-depressant medication of recurrent cases on a maintenance basis	7.2%	8.9%	10.6%	12.3%	14.1%	15.9%	17.7%	19.5%	21.3%	23.0%	24.8%	26.4%	28.1%	29.6%	31.2%	32.6%
Psychosis	Basic psychosocial support and anti-psychotic medication	25.4%	26.8%	28.3%	29.9%	31.4%	33.0%	34.5%	36.1%	37.6%	39.2%	40.7%	42.1%	43.6%	44.9%	46.2%	47.5%
	Intensive psychosocial support and anti-psychotic medication	12.5%	14.6%	16.7%	18.9%	21.0%	23.2%	25.4%	27.6%	29.8%	31.9%	34.0%	36.1%	38.1%	40.0%	41.9%	43.6%

	Basic psychosocial treatment, advice, and follow-up for bipolar disorder, plus mood-stabilizing medication	21.7%	22.1%	22.6%	23.1%	23.5%	24.0%	24.5%	25.0%	25.5%	25.9%	26.4%	26.9%	27.3%	27.7%	28.1%	28.5%
Bipolar disorder	Intensive psychosocial intervention for bipolar disorder, plus mood-stabilizing medication	7.0%	7.6%	8.1%	8.7%	9.3%	9.9%	10.5%	11.1%	11.7%	12.2%	12.8%	13.3%	13.9%	14.4%	14.9%	15.4%
Epilepsy	Basic psychosocial support, advice, and follow-up, plus anti-epileptic medication	42.6%	43.4%	44.1%	44.8%	45.6%	46.4%	47.1%	47.9%	48.7%	49.4%	50.2%	50.9%	51.6%	52.3%	52.9%	53.5%
	Identification and assessment of new cases of alcohol use/dependence	10.3%	11.7%	13.2%	14.7%	16.2%	17.8%	19.3%	20.9%	22.4%	23.9%	25.4%	26.8%	28.2%	29.6%	30.9%	32.1%
	Brief interventions and follow-up for alcohol use/dependence	12.2%	14.2%	16.2%	18.2%	20.3%	22.4%	24.5%	26.6%	28.6%	30.7%	32.7%	34.7%	36.6%	38.4%	40.2%	41.9%
Alcohol use/dependence	Management of alcohol withdrawal	13.7%	16.1%	18.5%	21.0%	23.5%	26.0%	28.6%	31.1%	33.6%	36.1%	38.5%	40.9%	43.2%	45.4%	47.5%	49.6%
	Relapse prevention medication for alcohol use/dependence	6.6%	8.1%	9.6%	11.2%	12.8%	14.5%	16.1%	17.7%	19.3%	20.9%	22.5%	24.0%	25.4%	26.9%	28.2%	29.5%
Population-based mental health interventions	Nation-wide regulatory ban of highly hazardous pesticides to prevent suicide	37.2%	39.1%	41.1%	43.2%	45.2%	47.3%	49.4%	51.5%	53.6%	55.6%	57.6%	59.6%	61.5%	63.3%	65.1%	66.8%
	Universal school-based social and emotional learning (SEL) interventions to prevent depression/anxiety and suicide in adolescents aged 12-17 years	12.9%	15.0%	17.2%	19.5%	21.8%	24.0%	26.3%	28.6%	30.9%	33.2%	35.4%	37.5%	39.6%	41.6%	43.5%	45.4%
Psychiatrists		146	166	188	209	231	253	275	297	319	340	362	382	402	422	440	458
Funding (1% of total health budget)		1.7%	1.9%	2.1%	2.3%	2.5%	2.7%	2.9%	3.2%	3.4%	3.6%	3.8%	4.0%	4.1%	4.3%	4.5%	4.7%

Annex 3: Interventions considered for 80 Percent reduction of the mental health burden

Intervention coverage for mental, neurological, and substance use disorders		2025	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035	2036	2037	2038	2039	2040
Anxiety disorders	Basic psychosocial treatment (mild cases)	16.6%	19.8%	23.0%	26.3%	29.7%	33.1%	36.5%	39.8%	43.2%	46.5%	49.7%	52.9%	56.0%	58.9%	61.8%	64.5%
	Basic psychosocial treatment and anti-depressant medication (moderate-severe)	14.8%	17.4%	20.2%	22.9%	25.8%	28.6%	31.4%	34.3%	37.1%	39.9%	42.6%	45.3%	47.8%	50.3%	52.7%	55.0%
	Intensive psychosocial treatment and anti-depressant medication (moderate-severe)	15.1%	18.9%	22.9%	26.9%	31.0%	35.1%	39.2%	43.3%	47.3%	51.3%	55.3%	59.1%	62.9%	66.5%	69.9%	70.0%
	Basic psychosocial treatment for mild depression	14.8%	17.4%	20.2%	22.9%	25.8%	28.6%	31.4%	34.3%	37.1%	39.9%	42.6%	45.3%	47.8%	50.3%	52.7%	55.0%
Depression	Basic psychosocial treatment and anti-depressant medication of first episode	13.0%	15.2%	17.4%	19.7%	22.0%	24.3%	26.6%	29.0%	31.3%	33.5%	35.8%	38.0%	40.1%	42.1%	44.1%	45.9%
	Intensive psychosocial treatment and anti-depressant medication of first episode	16.9%	20.1%	23.4%	26.8%	30.2%	33.7%	37.1%	40.6%	44.0%	47.4%	50.7%	54.0%	57.1%	60.1%	63.0%	65.8%
	Intensive psychosocial treatment and anti-depressant medication of recurrent cases	12.5%	14.5%	16.6%	18.7%	20.9%	23.0%	25.2%	27.4%	29.5%	31.7%	33.8%	35.8%	37.8%	39.7%	41.5%	43.3%
	Intensive psychosocial treatment and anti-depressant medication of recurrent cases on a maintenance basis	12.0%	15.0%	18.1%	21.2%	24.4%	27.6%	30.8%	34.0%	37.1%	40.3%	43.3%	46.3%	49.2%	52.0%	54.7%	57.3%
Psychosis	Basic psychosocial support and anti-psychotic medication	29.6%	32.2%	34.8%	37.6%	40.3%	43.1%	45.9%	48.7%	51.4%	54.2%	56.8%	59.4%	62.0%	64.4%	66.8%	69.0%
	Intensive psychosocial support and anti-psychotic medication	18.1%	21.7%	25.4%	29.1%	32.9%	36.7%	40.5%	44.3%	48.1%	51.9%	55.5%	59.1%	62.6%	65.9%	69.1%	70.0%
	Basic psychosocial treatment, advice, and follow-up for bipolar disorder, plus mood-stabilizing medication	23.0%	23.8%	24.6%	25.4%	26.3%	27.2%	28.0%	28.9%	29.8%	30.6%	31.4%	32.2%	33.0%	33.8%	34.5%	35.2%
Bipolar disorder	Intensive psychosocial intervention for bipolar disorder, plus mood-stabilizing medication	8.6%	9.6%	10.6%	11.6%	12.7%	13.7%	14.8%	15.8%	16.9%	17.9%	18.9%	19.9%	20.8%	21.8%	22.6%	23.5%
Epilepsy	Basic psychosocial support, advice, and follow-up, plus anti-epileptic medication	44.7%	46.0%	47.3%	48.6%	50.0%	51.4%	52.7%	54.1%	55.5%	56.8%	58.1%	59.4%	60.6%	61.8%	63.0%	64.1%
	Identification and assessment of new cases of alcohol use/dependence	14.4%	17.0%	19.6%	22.3%	25.0%	27.8%	30.5%	33.3%	36.0%	38.7%	41.3%	43.9%	46.4%	48.8%	51.1%	53.3%
	Brief interventions and follow-up for alcohol use/dependence	17.8%	21.3%	24.9%	28.5%	32.2%	36.0%	39.7%	43.4%	47.1%	50.8%	54.3%	57.8%	61.2%	64.5%	67.7%	70.7%
Alcohol use/dependence	Management of alcohol withdrawal	20.1%	24.2%	28.4%	32.6%	37.0%	41.4%	45.7%	50.1%	54.5%	58.8%	63.0%	67.1%	71.0%	74.9%	78.6%	80.0%
	Relapse prevention medication for alcohol use/dependence	10.9%	13.6%	16.4%	19.2%	22.0%	25.0%	27.9%	30.8%	33.6%	36.5%	39.2%	41.9%	44.6%	47.1%	49.5%	51.9%

Population-based mental health interventions	Nation-wide regulatory ban of highly hazardous pesticides to prevent suicide	42.8%	46.3%	49.8%	53.5%	37.0%	60.9%	64.6%	68.3%	72.0%	75.7%	79.2%	82.7%	86.1%	89.4%	92.5%	95.5%
	Universal school-based social and emotional learning (SEL) interventions to prevent depression/anxiety and suicide in adolescents aged 12-17 years	19.1%	22.9%	26.8%	30.8%	37.0%	38.9%	43.0%	47.1%	51.2%	55.2%	59.1%	62.9%	66.6%	70.2%	73.7%	77.0%
Psychiatrists		205	242	279	318	36.98%	396	435	475	514	552	590	626	662	696	730	761
Funding (1% of total health budget)		2.3%	2.6%	3.0%	3.3%	37.0%	4.1%	4.5%	4.8%	5.2%	5.6%	5.9%	6.3%	6.6%	6.9%	7.3%	7.6%

Annex 4: Approved Costed Staff establishments for Mental health in Uganda Health system (NRH, RRRH, GH, HC IV and III)

Butabika National Mental Referral Hospital							
No.	Job Title/Position	Salary Scale	Approved Est.	Filled	Vacant	Monthly Salary	Annual Salary
1	Senior Consultant (General Psychiatry)	U1SE	5	1	4	13,113,953	786,837,180
2	Senior Consultant (Forensic Psychiatry)	U1SE	1	0	1	13,113,953	157,367,436
3	Senior Consultant (Addiction Psychiatry)	U1SE	1	0	1	13,113,953	157,367,436
4	Senior Consultant (Child & Adolescent Psychiatry)	U1SE	1	0	1	13,113,953	157,367,436
5	Consultant (General Psychiatry)	U1SE	5	3	2	12,321,555	739,293,300
6	Consultant (Forensic Psychiatry)	U1SE	1	0	1	12,321,555	147,858,660
7	Consultant (Addiction Psychiatry)	U1SE	1	0	1	12,321,555	147,858,660
8	Consultant (Child & Adolescent Psychiatry)	U1SE	1	1	0	12,321,555	147,858,660
9	Associate Consultant (General Psychiatry)	U2 (Med-1)	10	3	7	6,071,555	728,586,600
10	Associate Consultant (Forensic Psychiatry)	U2 (Med-1)	2	0	2	6,071,555	145,717,320
11	Associate Consultant (Addiction Psychiatry)	U2 (Med-1)	2	0	2	6,071,555	145,717,320
12	Associate Consultant (Child & Adolescent Psychiatry)	U2 (Med-1)	2	0	2	6,071,555	145,717,320
13	Medical Officer	U4 (Med-1)	10	5	5	5,408,476	649,017,120
14	Principal Clinical Psychologist	U2 (Med-2)	4	1	3	4,658,476	223,606,848
15	Senior Clinical Psychologist	U3 (Med-2)	8	0	8	4,658,476	447,213,696
16	Clinical Psychologist	U4 (Med-2)	14	4	10	4,408,476	740,623,968
17	Principal Psychiatric Clinical Officer	U3 (Med-2)	6	4	2	4,658,476	335,410,272

18	Senior Psychiatric Clinical Officer	U4 (Med-2)	8	5	3	4,408,476	423,213,696
19	Senior Clinical Officer (EEG)	U4 (Med-2)	2	0	2	4,408,476	105,803,424
20	Senior Clinical Officer (ECG)	U4 (Med-2)	2	0	2	4,408,476	105,803,424
21	Psychiatric Clinical Officer	U5 (Med)	15	2	13	2,608,476	469,525,680
22	Principal Nutritionist	U2 (Med-2)	1	0	1	4,921,555	59,058,660
23	Senior Nutritionist	U3 (Med-2)	1	0	1	4,658,476	55,901,712
24	Principal Counsellor	U2	1	0	1		-
25	Senior Counsellor	U3	2	1	1		-
26	Counsellor	U4	3	0	3	723,868	26,059,248
	Sub total		109	30	79		
Mulago National Referral Hospital							
No.	Job Title/Position	Salary Scale	Approved Est.	Filled	Vacant	Monthly Salary	Annual Salary
1	Senior Consultants	U1SE	1			13,113,953	157,367,436
2	Consultant	U1SE	1			12,321,555	147,858,660
3	MoSG	U2 Med-1	1			6,071,555	72,858,660
4	Principal Psychiatric Clinical Officer	U3 Med-2	1			4,658,476	55,901,712
5	Senior Nursing Officer	U3 Med-2	1			4,658,476	55,901,712
6	Nursing Officer	U4 Med-2	4			4,408,476	211,606,848
7	Clinical Psychologist	U5 Med	1			2,608,476	31,301,712
8	Psychiatric Clinical Officer	U5 Med	2			2,608,476	62,603,424
9	Assistant Nursing Officer	U5 Med	6			2,608,476	187,810,272
10	Mental Health Assistant	U8	4			596,000	28,608,000
	Sub total		22				
Regional Referral Hospitals (16 Regional Referrals countrywide)							
No.	Job Title/Position	Salary Scale	Approved Est.	Filled	Vacant	Monthly Salary	Annual Salary
1	Senior Consultant	U1SE	1			13,113,953	157,367,436
2	Consultant (Psychiatry)	U1SE	2			12,321,555	295,717,320

3	MoSG (Psychiatry)		U2 (Med-1)	3			6,071,555	218,575,980
4	Principal Clinical Psychologist		U2 (Med-2)	1			4,921,555	59,058,660
5	Principal Psych Clinical Officer		U3 (Med-2)	1			4,658,476	55,901,712
6	Senior Clinical Psychologist		U3 (Med-2)	1			4,408,476	52,901,712
7	Sen Psych Clinical Officer		U4 (Med-2)	4			4,408,476	211,606,848
8	Clinical Psychologist		U4 (Med-2)	2			4,408,476	105,803,424
9	Psychiatric Clinical Worker		U5 (Med-2)	2			933,461	22,403,064
10	Social Worker (Psychiatry)		U4L	1			723,868	8,686,416
11	Principal Nursing Officer (Psychiatry)		U2 (Med-2)	1			4,921,555	59,058,660
12	Senior Nursing Officer (Psychiatry)		U3 (Med-2)	2			4,658,476	111,803,424
13	Clinical Officer - Child and Adolescent Mental Health		U5 (Med-2)	2			2,608,476	62,603,424
14	Nursing Officer (Psychiatry)		U4 (Med-2)	4			4,408,476	211,606,848
15	Assistant Nursing Officer (Psychiatry)		U5 (Med-2)	10			2,608,476	313,017,120
16	Senior Enrolled Psych Nurse		U6 (Med-2)	10			1,558,476	187,017,120
17	Enrolled Nurse (Psychiatry)		U7 (Med-2)	20			1,421,634	341,192,160
18	Consellor		U4	2			723,868	17,372,832
	Sub total			69				
General Hospitals								
No.	Job Title/Position		Salary Scale	Approved Est.	Filled	Vacant	Monthly Salary	Annual Salary
1	Senior Psychiatric Officer		U4 (Med-2)	1			4,408,476	52,901,712
2	Social Worker Psychiatry		U4L	1			723,869	8,686,428
3	Nursing Officer Psych		U4 (Med-2)	3			4,408,476	158,705,136
4	Assistant Nursing Officer Psych		U5 (Med)	4			2,608,476	125,206,848
5	Clinical Officer		U5 (Med)	4			2,608,476	125,206,848
6	Senior Enrolled Psych Nurse		U6 (Med)	1			1,558,476	18,701,712
7	Enrolled Psych Nurse		U7 (Med)	4			1,421,634	68,238,432
	Sub total			18				

HC IV							
No.	Job Title/Position	Salary Scale	Approved Est.	Filled	Vacant	Monthly Salary	Annual Salary
1	Psychiatric Nursing Officer	U5(Med)	1			2,608,476	31,301,712
2	Assistant Nursing Officer (Psychiatry)	U5(Med)	2			2,608,476	62,603,424
3	Enrolled Nurse (Psychiatry)	U7(Med)	2			1,421,634	34,119,216
	Sub total		5				
HC III							
No.	Job Title/Position	Salary Scale	Approved Est.	Filled	Vacant	Monthly Salary	Annual Salary
1	Assistant Nursing Officer (Psychiatry)	U5(Med)	1			2,608,476	31,301,712
2	Enrolled Nurse (Psychiatry)	U7(Med)	1			1,421,634	17,059,608
	Sub total		2				

Source: Public Service

Annex 5: Cost price of Essential mental health commodities and supplies to health facilities in Uganda

SN	DESCRIPTION	DRUG CLASS	UNIT	PRICE
1	Amitriptyline 25mg tablet	Anxiolytics	1,000	22,600
2	Benzhexol 2mg tablet	Muscle relaxant	500	32,650
3	Carbamazepine 200mg tablet	Antiepileptic	1,000	84,800
4	Clonazepam 2mg tablets	Benzodiazepine	30	19,700
5	Fluoxetine 20mg cap	Selective serotonin reuptake inhibitor	100	4,600
6	Haloperidol tablets 5mg	Antipsychotic	100	28,950
7	Phenobarbital 30mg tablet	Barbiturate	1,000	38,200
8	Phenytoin sodium 100mg tablet	Anticonvulsants	1,000	91,700
9	Sodium valproate 500mg tabs	Anticonvulsants	100	44,750
10	Haloperidol 5mg/1ml injection	Antipsychotic	5	21,850
11	Haloperidol decanoate 50mg/ml depot injection	Antipsychotic	1	6,550
12	Phenobarbital 200mg/2ml injection	Barbiturate	10	60,800
13	Phenytoin sodium 50mg/ml injection 2-5ml	Anticonvulsants	1	12,150

Source: NMS

Annex 6: List of Studies Included in the Systematic Review

Study (Author, Year)	Population, Country	Category/ Theme	Outcome	Key Finding/ Key Driver of Mental Health
Abbo et al., 2013	Children 3–19, N.E Uganda	Psychological	Anxiety disorders	Prevalence of anxiety disorder at 26.6%; Social phobia (1.1.4%). The main predictor being Food Insecurity (p<0.05)
Admassu et al., 2024	Youth 16–24, Kampala slums	Psychological	Depression trajectories	Key drivers of Depression includes: Food insecurity (9.76), Intra partner violence (IPV at 2.7), unemployment predict sustained depression, estimated at 95% CI, P<0.001.
Akena et al., 2023	Adults ≥18, Central Uganda	Psychological	MDD, GAD, PTSD, SUD	Depression 6.8%; Post traumatic stress disorder (PTSD) 32%; stigma & literacy associated
Ashaba et al., 2015	Mothers, Mbarara	Health/ Psychological	Maternal depression, child malnutrition	Maternal Depression prevalence 42% in cases vs 12% controls; linked to child malnutrition
Batte et al., 2024	Adolescents 13–18, Manafwa Watershed	Psychological	Studied Mental health challenges (Anxiety, Depression, Anger, Disruptive Behaviour, self-concept)	Depression Prevalence 65%; Anxiety 44%; Disruptive behaviour 31%; Substance abuse 19%; Family >5 children 47.6% P<0.001; 95% CI.
Bulamba et al., 2024	Adults ≥35, Wakiso	Health, Psychological	Depression, anxiety, NCDs	Depression 18.9%; anxiety 11.4%; comorbidity with hypertension: 95% CI, P<0.001
Byansi et al., 2023	Adolescent girls, SW Uganda	Socioeconomic	Mental Health Service utilization	High vs low attendance patterns; only 34% completed 16 times required access; family support influences attendance and access to mental health services: 95% CI, P<0.005
Filiatreau et al., 2023	Adolescent girls, Masaka	Psychological	Psychosocial Wellbeing: Hopelessness, self-esteem	Reduced hopelessness, improved self-esteem/self-concept hence improved mental health.
Kalungi et al., 2025	Cross-sectional survey (Uganda Genome Resource cohort)	Health	Prevalence of MDD, GAD, PTSD, ADHD, suicidality, alcohol abuse	Associations with BMI, hypertension, smoking, marriage
Kinyanda et al., 2013	Children 3–19, NE Uganda	Socioeconomic	Depression	Domestic violence & orphan hood predictors: Prevalence: IPV 43.7%. Determinants: War-related torture (OR=1.58, p=0.01) and poverty (p<0.001) were strongly associated with IPV and subsequent mental illness.

Kinyanda et al., 2016	Adults, HIV+	Psychological	MDD, suicidality	Stigma increases risk; social support protective
Mugisha et al., 2015	Adults, Northern Uganda	Health, Socioeconomic	MDD prevalence	24.7% prevalence; food insecurity, HIV+, widowhood predictors
Mugisha et al., 2025	Adolescents 14–17, Gulu & Masaka	Socioeconomic, Psychological	Depression, anxiety	34% depression; food insecurity, abuse, neglect predictors
Nabayinda et al., 2023	Children 8–13, Southern Uganda	Psychological	Depression	Family cohesion protective
Ssewamala et al., 2023	Children 8–13, Masaka	Multiple Family Groups (MFG)	Multiple Family Group, ODD, depression, self-concept	Reduced ODD/depression; improved self-concept
Tutlam et al., 2025	Refugee youth 13–30	Health, Psychological	HIV-Focused, Depression, PTSD	Stigma, unemployment, orphan hood predictors.
Ziegel, 2025	Adolescents/adults, Wakiso & Hoima	Psychological, Environmental	Anxiety, alcohol use	Anxiety ~2%; hazardous alcohol use 5–18%; religion protective
Meyer et al., 2020	Refugee settlements (Kiryandongo, Adjumani)	Psychological/ Environmental	Adolescent depression, anxiety	Anxiety prevalence: 50% (Kiryandongo), 12.7% (Adjumani); Depression: 43.6% vs 31%



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